

Vanuatu 2023

 **MICS**

Multiple Indicator
Cluster Survey



Snapshot of Key Findings

July 2024

Vanuatu 2023



Snapshot of Key Findings

July 2024



Pacific
Community
Communauté
du Pacifique



unicef 
for every child

The Vanuatu Multiple Indicator Cluster Survey (MICS) was carried out in 2023 by Vanuatu Bureau of Statistics (VBoS) in collaboration with the Vanuatu Ministry of Health and other government ministries as part of the Global MICS Programme. Technical support was provided by the United Nations Children's Fund (UNICEF), United Nations Population Fund (UNFPA) and Pacific Community (SPC) with funding of Government of Vanuatu, Government of New Zealand and financial support of UNICEF and UNFPA.

The Global MICS Programme was developed by UNICEF in the 1990s as an international multi-purpose household survey programme to support countries in collecting internationally comparable data on a wide range of indicators on the situation of children and women. MICS surveys measure key indicators that allow countries to generate data for use in policies, programmes, and national development plans, and to monitor progress towards the Sustainable Development Goals (SDGs) and other internationally agreed upon commitments.

The objective of this publication is to facilitate the timely dissemination and use of key findings from the Vanuatu MICS 2023. This publication contains key findings from the survey, and the comprehensive findings and results are available in the Survey Findings Report.

For more information on the Global MICS Programme, please go to mics.unicef.org.

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Sample & Survey Characteristics

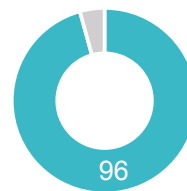


Response Rates

Household

Household	Number
Sampled	5,112
Occupied	4,522
Interviewed	4,327

Response Rates



Survey Implementation

Implementing agency:
Vanuatu Bureau of Statistics

Sampling frame:
2022 Vanuatu Agricultural Census (VNAC).
Vanuatu 2020 National Population and Housing Census.

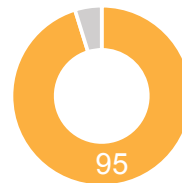
Interviewer training:
June to July 2023.

Fieldwork:
July to October 2023.

Questionnaires:
Household
Women age 15-49
Men age 15-49
Children under 5
Children age 5-17

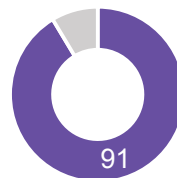
Women age 15-49

Eligible for interview	3,583
Interviewed	3,412



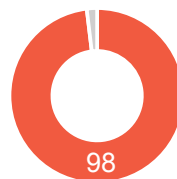
Men age 15-49

Eligible for interview	1,520
Interviewed	1,389



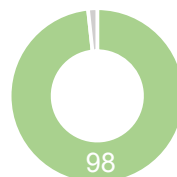
Children under 5

Eligible for interview	2,082
Mothers/Caretakers interviewed	2,043



Children age 5-17

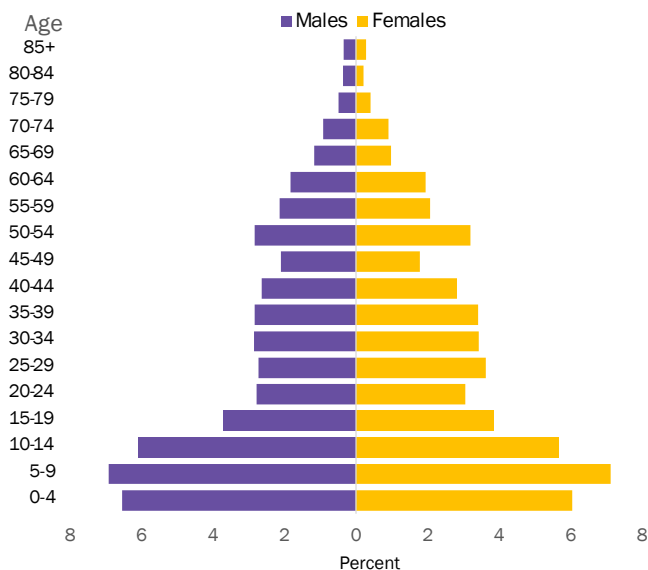
Eligible for interview	2,508
Mothers/Caretakers interviewed	2,466



Population Characteristics

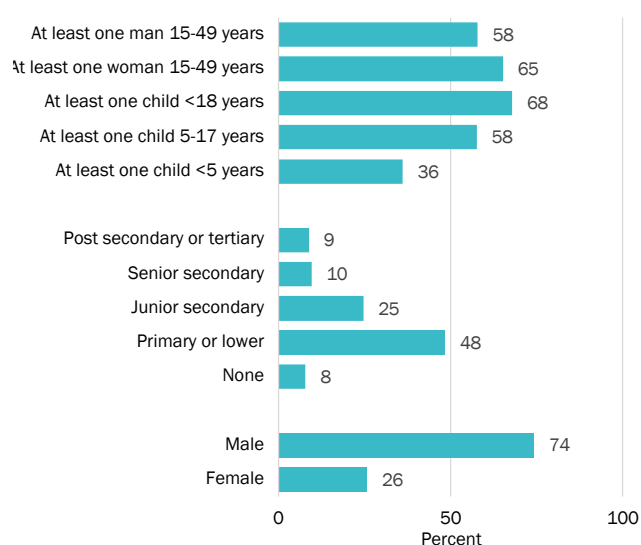


Household Population Age & Sex Distribution



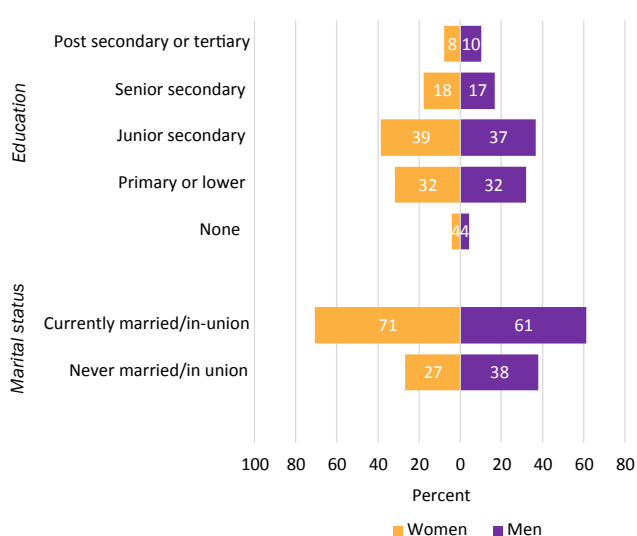
Percent distribution of household population by age group and sex.

Household Composition & Characteristics of Head of household



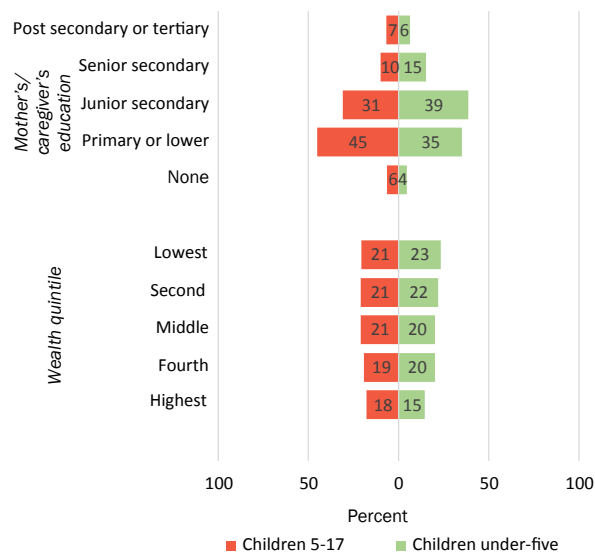
Percent of households by selected characteristics.

Women & Men's Profile



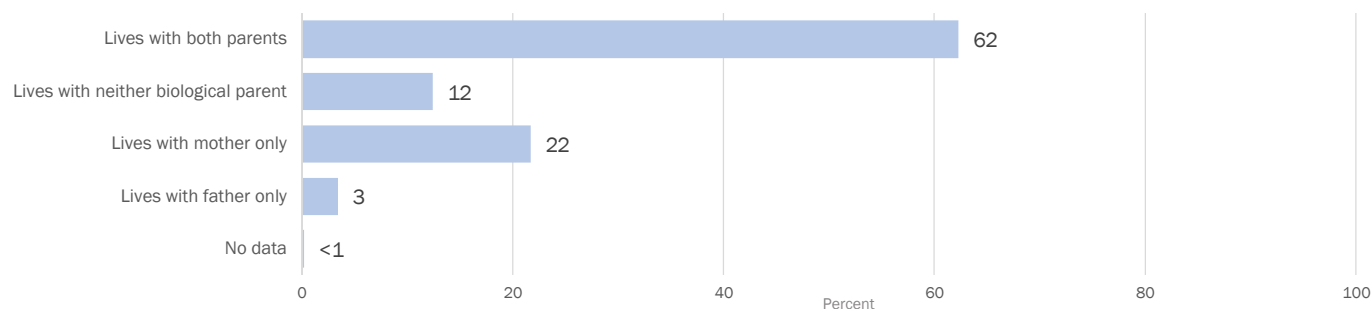
Percent distribution of women and men age 15-49 by background characteristics.

Children's Profile



Percent distribution of children age 5-17 and under-five by background characteristics.

Children's (0-17 years) Living Arrangements



Percent distribution of children age 0-17 years according to living arrangements.

Provincial Distribution of Population (percent)

Province	Household population	Women 15-49	Men 15-49	Children under 5	Children 5-17
National	100.0	100.0	100.0	100.0	100.0
Torba	2.9	2.6	2.7	2.6	2.8
Sanma	19.5	19.6	20.5	20.0	19.2
Penama	13.1	11.3	11.1	14.5	15.1
Malampa	13.3	12.2	11.5	11.5	14.0
Shefa	35.9	40.3	41.1	31.8	32.3
Tafea	15.3	14.0	13.1	19.7	16.6

Key Messages

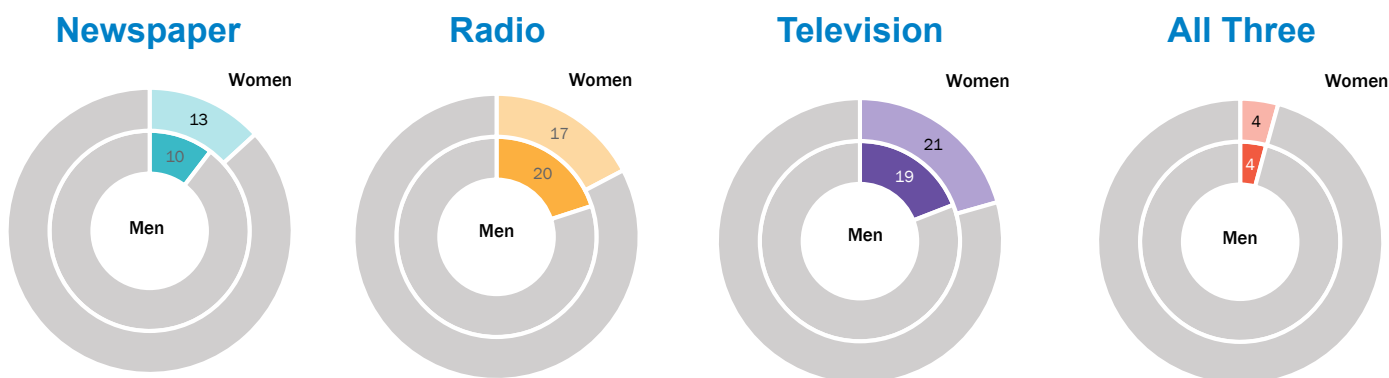
- For the Vanuatu MICS 2023, 5,112 households were sampled, from which 4,327 were interviewed. This represents a response rate of 97 percent.
- The majority of household heads are male (74 percent) but a sizable percentage of households are headed by women.
- 22 percent of the children in Vanuatu are living with only their mother.



Mass Media, Communications & Internet



Exposure to Mass Media



Percentage of women & men age 15-49 years who are exposed to specific mass media (newspaper, radio, television) on a weekly basis and percentage of women & men age 15-49 who are exposed to all three on a weekly basis.

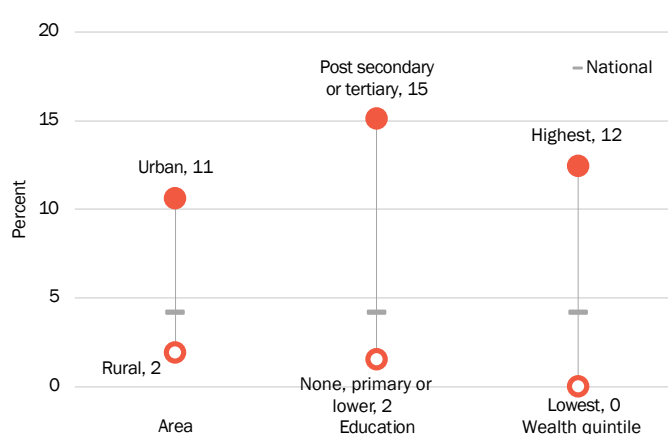
Inequalities in Exposure to Mass Media

Women Exposed to Newspaper, Radio & Television Weekly



Percentage of women age 15-49 years who are exposed to newspaper, radio, and television on a weekly basis.

Men Exposed to Newspaper, Radio & Television Weekly



Percentage of men age 15-49 years who are exposed to newspaper, radio, and television on a weekly basis.

Household Ownership of Information & Communication Technology (ICT) Equipment & Internet at Home

Province	Radio	Television	Telephone-Fixed line	Telephone-Mobile	Computer	Internet at Home
National	21.2	19.2	0.9	83.3	18.4	59.6
Torba	6.1	1.7	0.0	57.2	2.5	31.8
Sanma	21.7	12.5	1.3	83.8	12.7	55.5
Penama	14.3	1.6	0.1	78.7	7.6	36.8
Malampa	15.9	6.5	0.4	78.4	13.2	62.8
Shefa	30.3	41.0	1.4	93.1	31.7	75.4
Tafea	14.0	8.3	0.4	74.4	12.7	50.2

Percentage of households which own a radio, television-fixed line, telephone- mobile, computer and that have access to the internet at home.

Inequalities in Household Ownership of ICT Equipment & Internet at Home

Household Ownership of a Radio



Percentage of households with a radio at home.

Household Ownership of a Computer



Percentage of households with a computer at home.

Household Ownership of a Mobile Telephone



Percentage of households with mobile telephone.

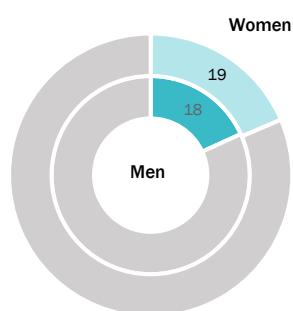
Households with Internet



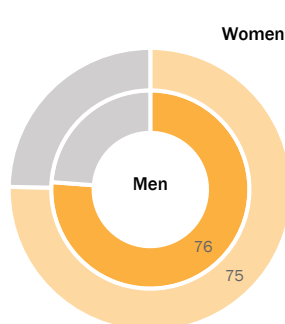
Percentage of households with access to the internet at home.

Use of Information & Communication Technology

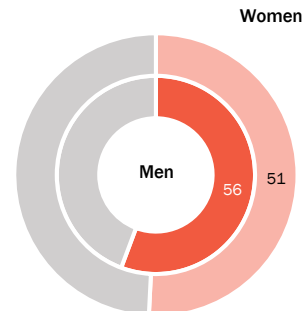
Computer Use



Mobile Phone Use



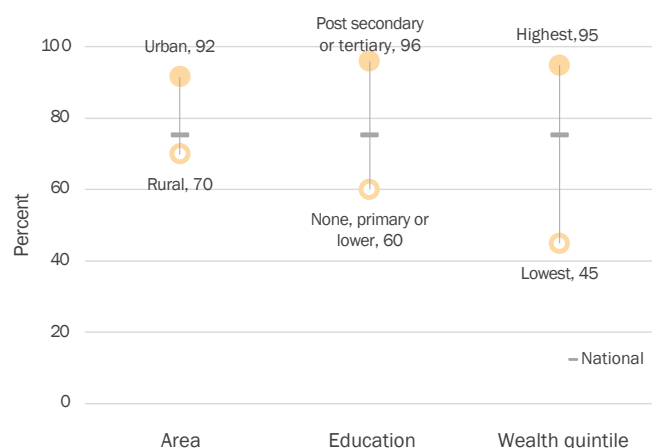
Internet Use: SDG17.8.1



Percentage of women and men age 15-49 years who during the last 3 months used a computer, used a mobile phone and used the internet.

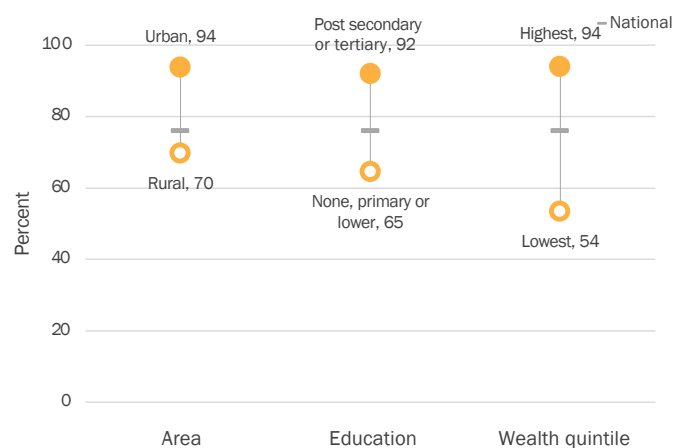
Disparities in Use of Information & Communication Technology

Disparities in Mobile Phone Use among Women



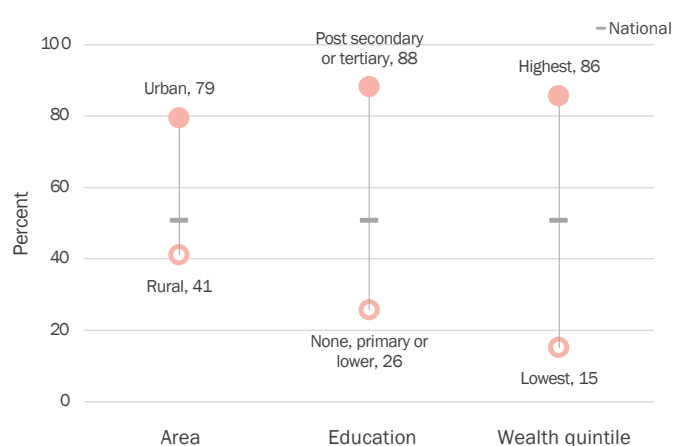
Percentage of women age 15-49 years who during the last 3 months used a mobile phone.

Disparities in Mobile Phone Use among Men



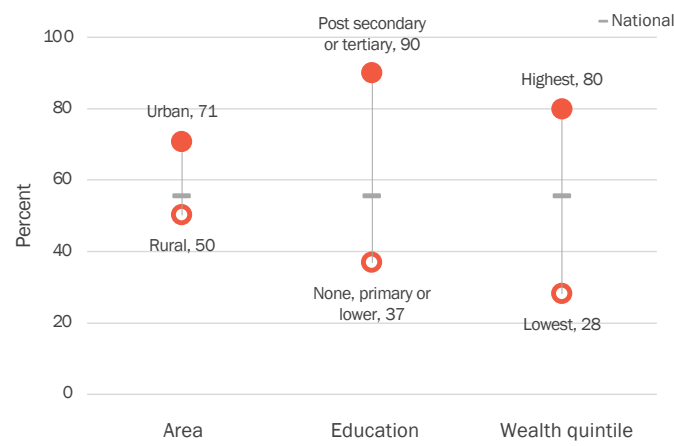
Percentage of men age 15-49 years who during the last 3 months used a mobile phone.

Disparities in Internet Use among Women: SDG17.8.1



Percentage of women age 15-49 years who used the internet in the last 3 months.

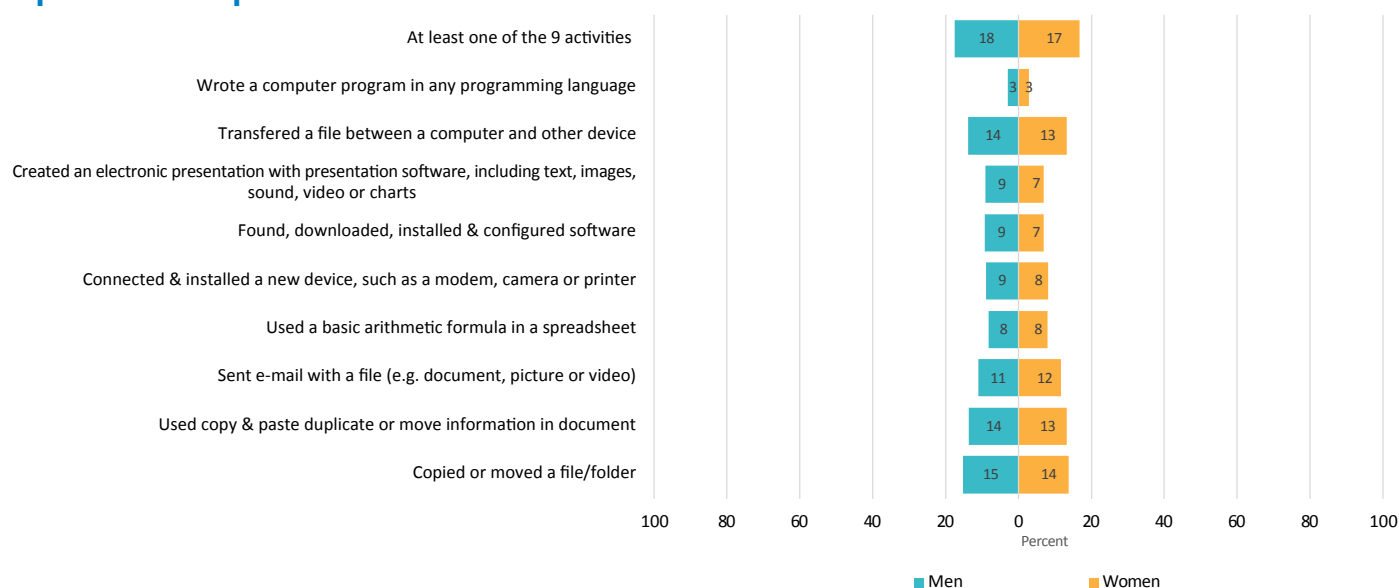
Disparities in Internet Use among Men: SDG17.8.1



Percentage of men age 15-49 years who used the internet in the last 3 months.

Information & Communication Technology (ICT) Skills

Specific Computer Skills



Percentage of women and men age 15-49 years who in the last 3 months have carried out specific computer related activities and the percentage who have carried out at least one of these activities.

Provincial Data on ICT Use & Skills among Women

Province	Computer Use	Mobile Phone Use	Internet Use	Performed at Least 1 computer-related activity
National	18.6	75.3	50.8	16.8
Torba	5.3	48.2	22.7	4.5
Sanma	12.2	68.6	41.8	10.9
Penama	4.5	60.9	29.5	3.5
Malampa	7.2	72.7	29.2	4.7
Shefa	30.5	90.4	72.9	28.2
Tafea	16.9	60.2	40.7	15.7

Percentage of women age 15-49 years who during the last 3 months used a computer, used a mobile phone and used the internet and percentage who performed at least 1 computer-related activity.

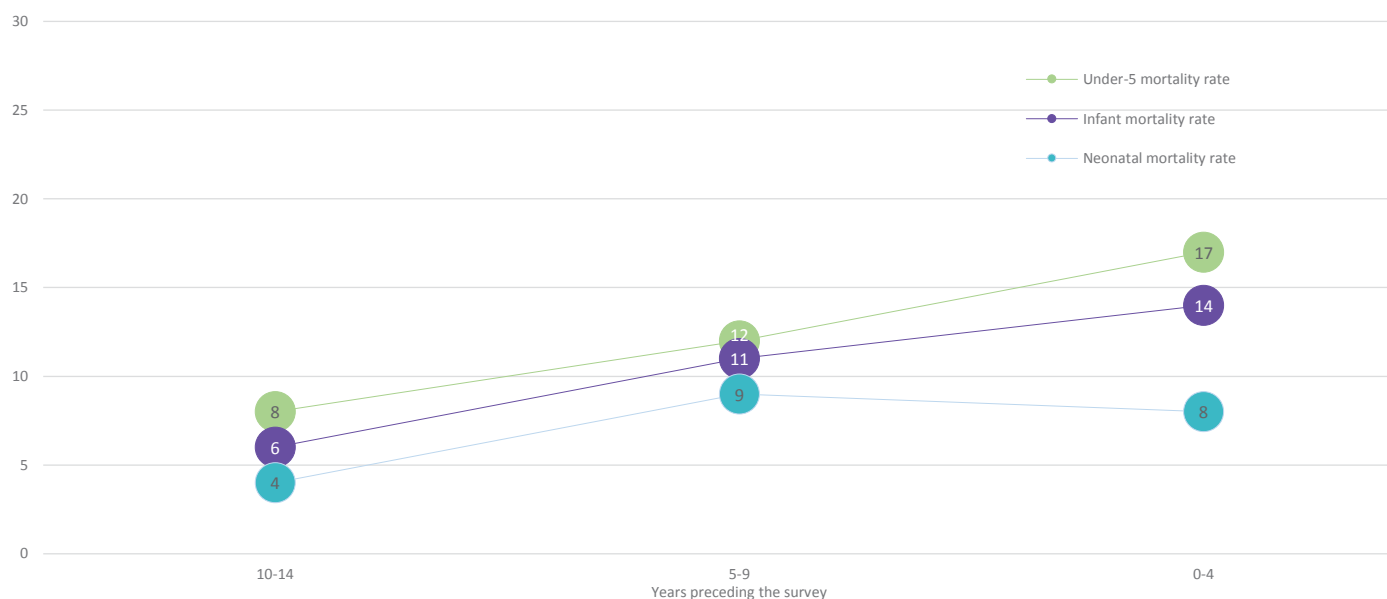
Key Messages

- In Vanuatu, the percentage of people reporting using the various forms of mass media (newspaper, television and radio) is rather low.
- More than 80 percent of households in Vanuatu have a mobile phone. In Shefa province mobile phone ownership is almost universal (93 percent).
- There are no major differences between the sexes in terms of ICT usage.
- Internet usage among women is highest in Shefa province and lowest in Torba.
- Nearly nine out of ten women age 15-49, educated to post secondary or tertiary, used the internet in the last three months.

Child Mortality



Mortality Rates among Children Under-5



Years preceding the survey	Neonatal mortality rate: SDG 3.2.2	Post-neonatal mortality rate	Infant mortality rate:	Child mortality rate	Under-5 mortality rate: SDG 3.2.1
0-4	8	6	14	3	17
5-9	9	2	11	1	12
10-14	4	2	6	2	8

Neonatal mortality (NN): probability of dying within the first month of life.

Post-neonatal mortality: calculated as the difference between infant and neonatal mortality rates.

Infant mortality (${}_1q_0$): probability of dying between birth and first birthday.

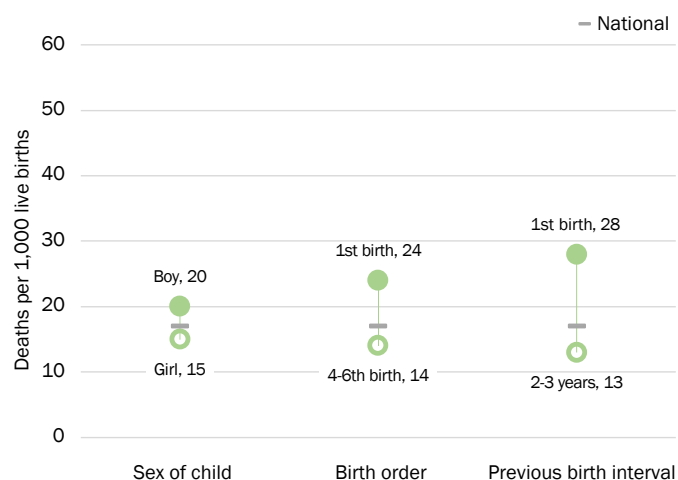
Child mortality (${}_5q_1$): probability of dying between the first and fifth birthday.

Under-5 mortality (${}_5q_0$): probability of dying between birth and fifth birthday.

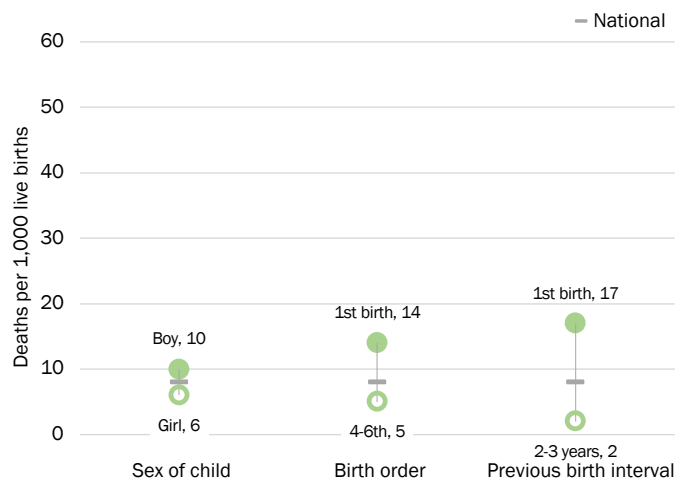
MICS uses a **direct method for estimation of child mortality**. This involves collecting **full birth histories** whereby women age 15-49 are asked for the date of birth of each child born alive, whether the child is still alive and, if not, the age at death.

Differentials in Child Mortality

Under-5 mortality rate by demographic characteristics



Neonatal mortality rate by demographic characteristics



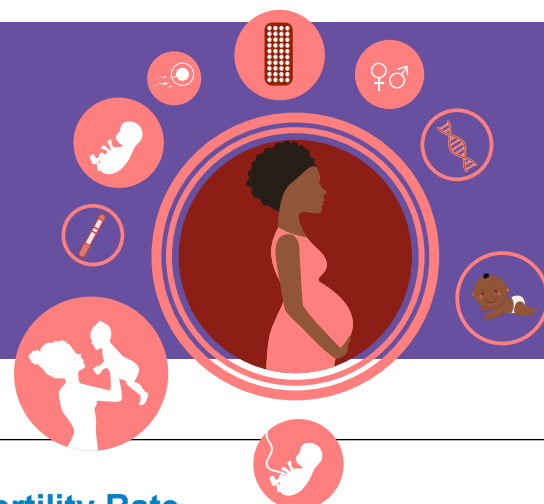
Under-five mortality rates and neonatal mortality rates for the five-year period preceding the survey demographic characteristics.

Key Messages

- The SDG target for child mortality aims to end, by 2030, preventable deaths of newborns and children under 5 years of age, with all countries aiming to reduce neonatal mortality to at least as low as 12 deaths per 1,000 live births and under-5 mortality to at least as low as 25 deaths per 1,000 live births.
- Vanuatu has achieved both targets with a neonatal mortality rate of 8 deaths per 1,000 live births and an under-5 mortality rate of 17 deaths per 1,000 live births.

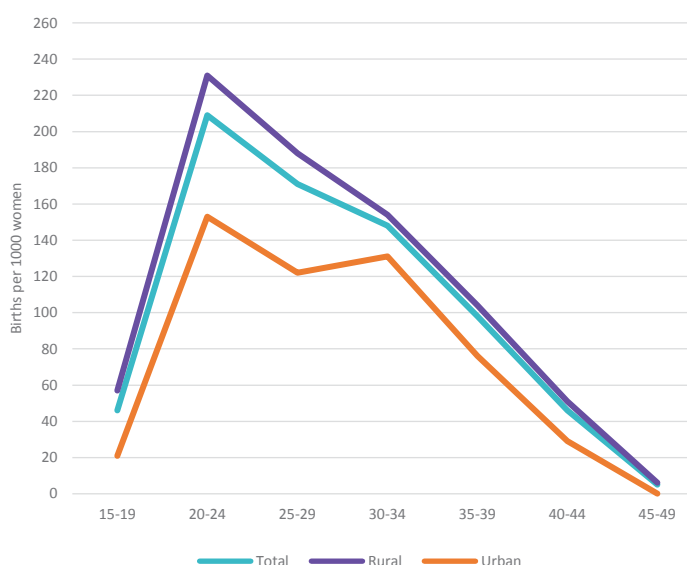


Fertility & Family Planning



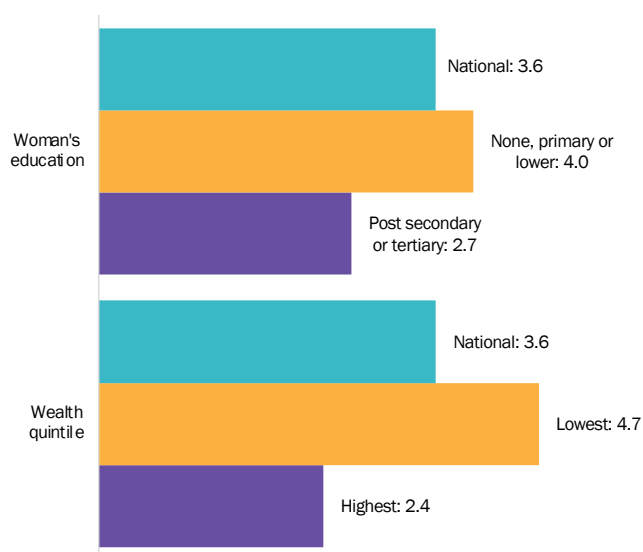
Fertility

Age Specific Fertility Rates



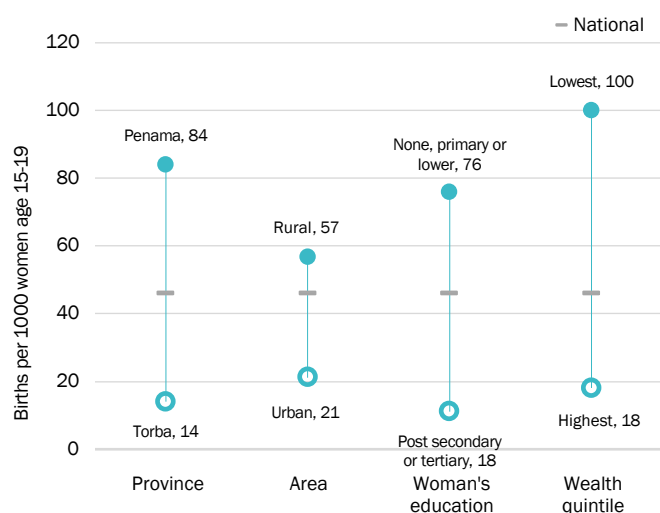
Age-specific fertility rates (ASFR) are the number of live births in the last 3 years, divided by the average number of women in that age group during the same period, expressed per 1,000 women.

Total Fertility Rate



The total fertility rate (TFR) is calculated by summing the age-specific fertility rates (ASFRs) calculated for each of the five-year age groups of women, from age 15 through to age 49.

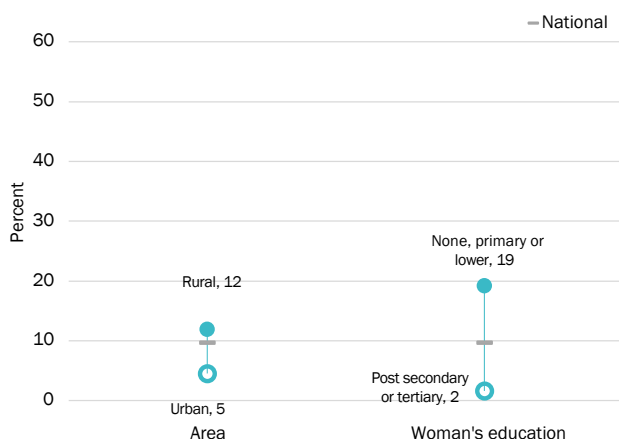
Adolescent Birth Rate: SDG indicator 3.7.2



Age-specific fertility rate for girls age 15-19 years for the three-year period preceding the survey.

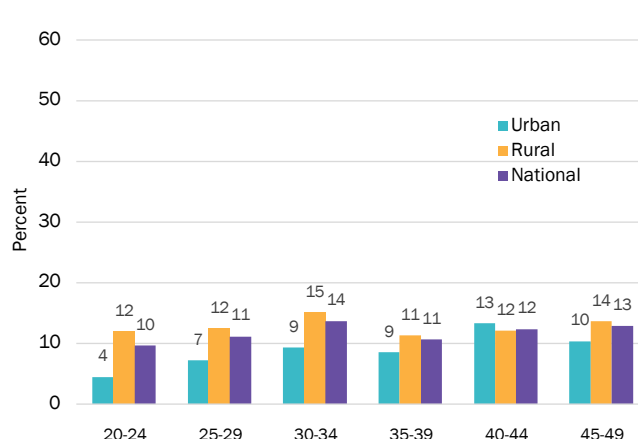
Adolescent Birth rate SDG 3.7.2 indicator is under target 3.7: By 2030, ensure universal access to sexual and reproductive health-care services, including for family planning, information and education, and the integration of reproductive health into national strategies and programmes. Reducing adolescent fertility and addressing the multiple factors underlying it are essential for improving sexual and reproductive health and the social and economic well-being of adolescents. Preventing births very early in a woman's life is an important measure to improve maternal health and reduce infant mortality.

Early Child Bearing - by Age 18



Percentage of women age 20-24 years who have had a live birth before age 18, by background characteristics.

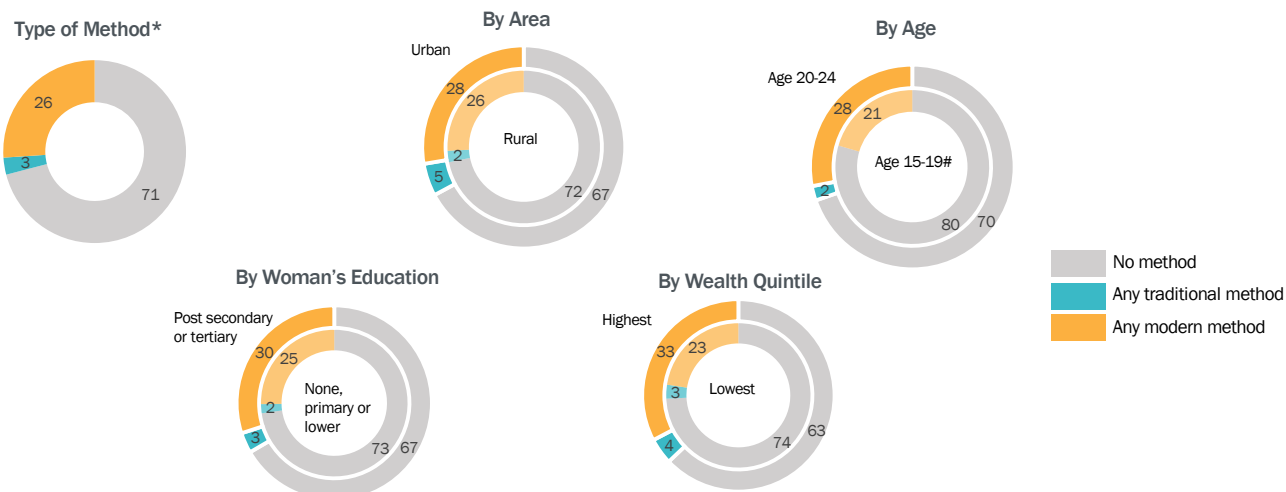
Trends in Early Child Bearing - by Age 18



Percentage of women age 20-49 years who have had a live birth before age 18.

Family Planning

Method of Family Planning by Various Characteristics



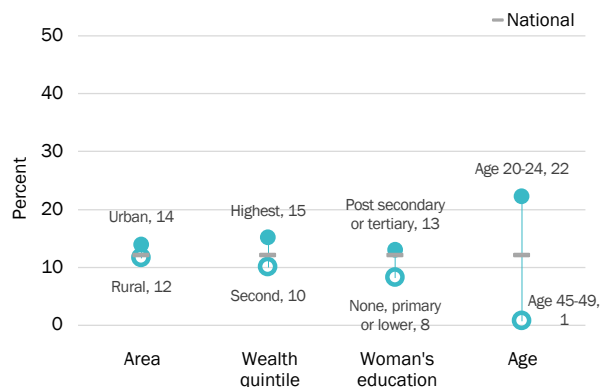
Percentage of women age 15-49 years currently married or in union who are using (or whose partner is using) a contraceptive method.

*Modern methods include female sterilization, male sterilization, IUD, injectables, implants, pills, male condom, female condom, diaphragm, foam, jelly and contraceptive patch; Traditional methods refer to periodic abstinence and withdrawal.

Figures that are based on 25-49 unweighted cases.

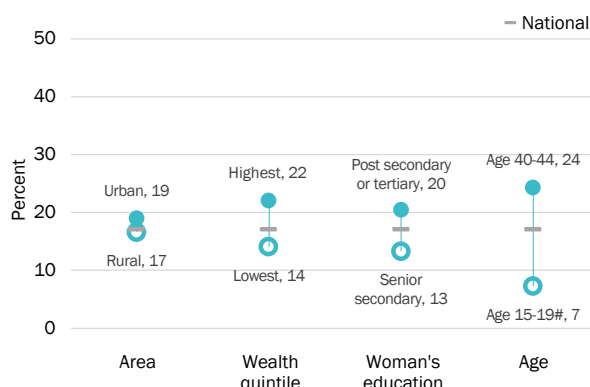
Met Need for Family Planning

Met Need for Family Planning - Spacing



Percentage of women age 15-49 years currently married or in union with met need for family planning for spacing, by background characteristics.

Met Need for Family Planning - Limiting



Percentage of women age 15-49 years currently married or in union with met need for family planning for limiting, by background characteristics.
Figure that are based on 25-49 unweighted cases.

Percentage of Demand for Family Planning Satisfied with Modern Methods - SDG indicator 3.7.1

The proportion of demand for family planning satisfied with modern methods (SDG indicator 3.7.1) is useful in assessing overall levels of coverage for family planning programmes and services. Access to and use of an effective means to prevent pregnancy helps enable women and their partners to exercise their rights to decide freely and responsibly the number and spacing of their children and to have the information, education and means to do so. Meeting demand for family planning with modern methods also contributes to maternal and child health by preventing unintended pregnancies and closely spaced pregnancies, which are at higher risk for poor obstetrical outcomes.

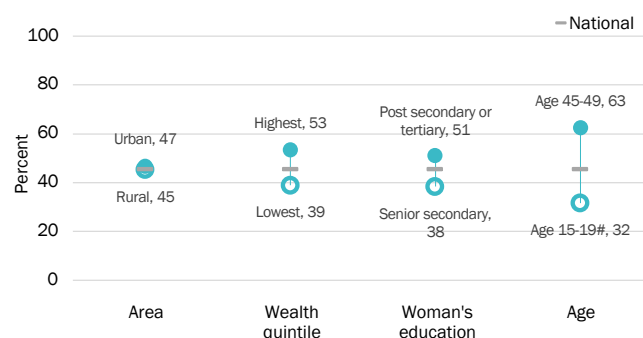


Figure that are based on 25-49 unweighted cases

Provincial Data on Fertility & Family Planning

Province	Adolescent Birth Rate	Total Fertility Rate	Child bearing before 15 ¹	Child bearing before 18 ²	Contraception Use of modern method among married / in-union women	Contraception Use of any method among married / in-union women	Demand for family planning satisfied with modern methods among married / in-union women
National	46	3.6	0.2	9.7	26.2	29.2	45.6
Torba	14	3.1	(0.0)	(16.1)	18.7	18.7	45.6
Sanma	52	3.4	0.0	12.5	20.2	21.4	42.1
Penama	84	4.4	1.8	(19.5)	30.9	38.4	43.6
Malampa	57	3.7	(0.0)	(6.5)	18.5	18.8	37.4
Shefa	30	3.0	0.0	5.2	32.0	35.3	51.8
Tafea	64	5.0	0.0	14.8	24.4	28.1	41.2

¹ Percentage of women age 15-19 years who have had a live birth before age 15.

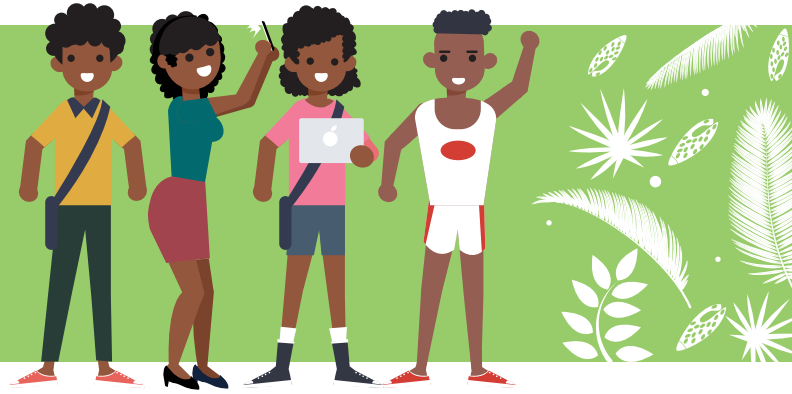
² Percentage of women age 20-24 years who have had a live birth before age 18.

() Figures that are based on 25-49 unweighted cases.

Key Messages

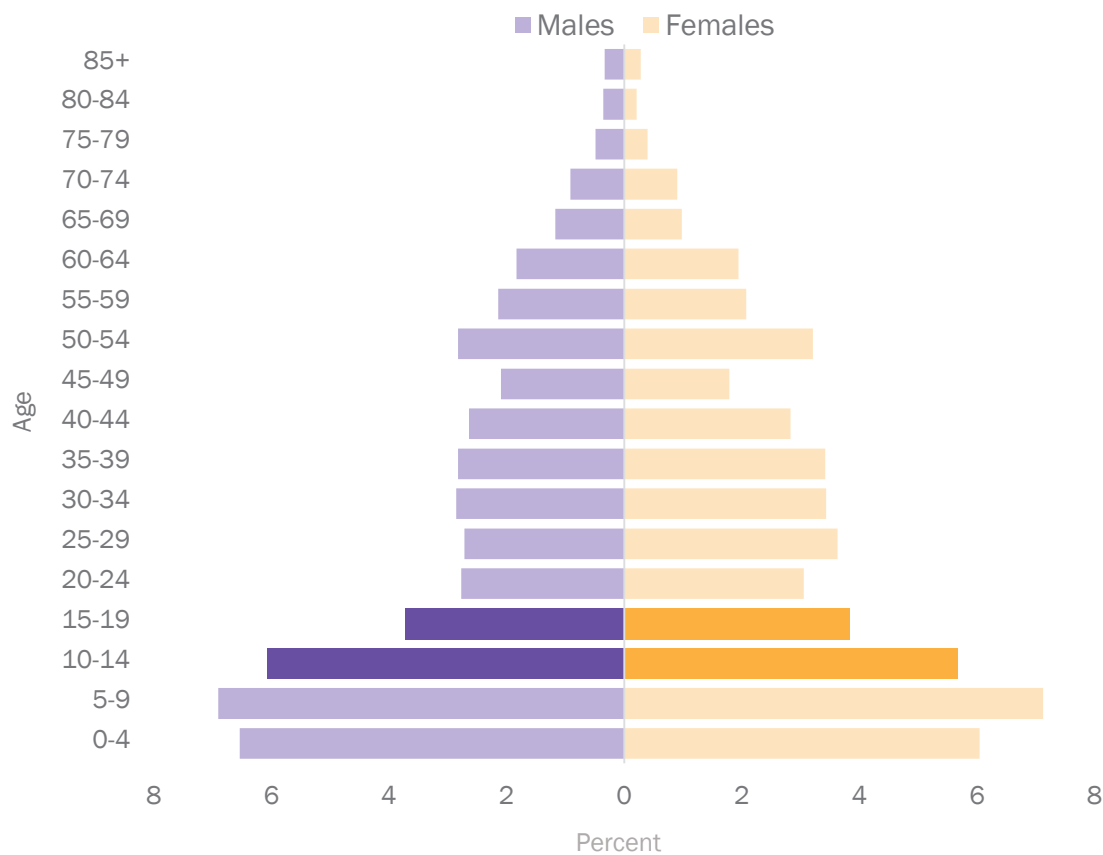
- In Vanuatu, the Total Fertility Rate (TFR), which measures the average number of children born to a woman in her lifetime, is 3.6. The TFR among the lowest wealth quintile is higher (4.7) compared to the highest at 2.4.
- The Adolescent birth rate is 46 per 1,000 for women age 15-19 years.
- About 30 percent of currently married or in-union women age 15-49 are using a method of contraception. Of these 90 percent are using modern methods.

Adolescents



The Adolescent Population: Age 10-19 years

Age & Sex Distribution of Household Population

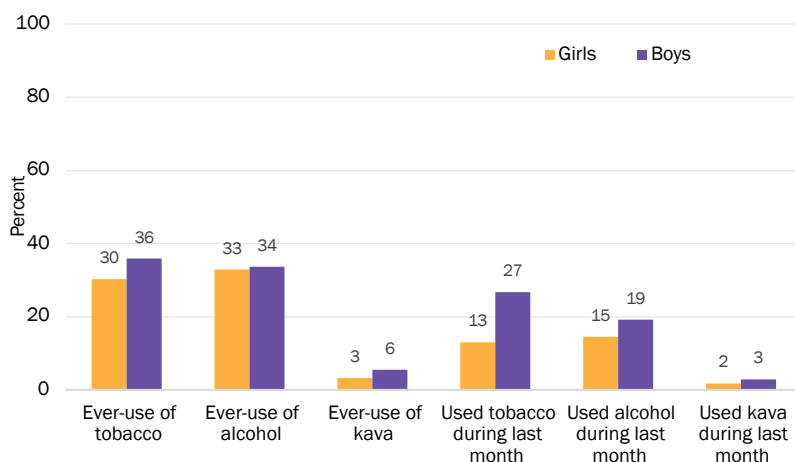


This Snapshot of adolescent well-being is organized around key priority areas for adolescents:

- Every adolescent survives and thrives.
- Every adolescent learns.
- Every adolescent is protected from violence and exploitation.
- Every adolescent lives in a safe and clean environment.
- Every adolescent has an equitable chance in life.

Every Adolescent Survives & Thrives

Tobacco*, Alcohol & Kava Use



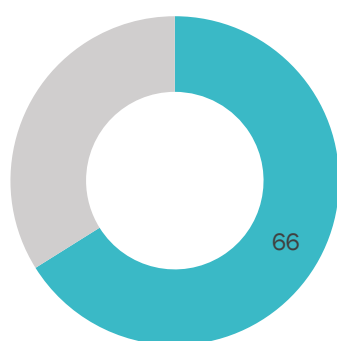
Percentage of adolescent girls and boys age 15-19 who have, ever used tobacco or alcohol or kava, and used tobacco or alcohol or kava in the last 1 month.

* Tobacco use in last month among adolescents is an age disaggregate of SDG 3.a.1.

Alcohol, tobacco and kava use typically have their onset in adolescence and are major risk factors for adverse health and social outcomes, as well as for non-communicable diseases later in life. Adolescence is a time of heightened risk-taking, independence seeking and experimentation. Adolescents are at increased risk of substance use due to social, genetic, psychological or cultural reasons. Yet, adolescence is also an opportune time for education on the negative consequences of substance use and promote healthy behaviours that will last into adulthood.

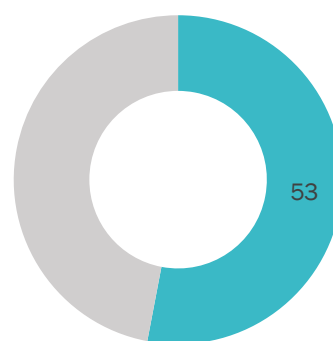
Every Adolescent Learns

Foundational Reading Skills



Percentage of children age 10-14 who can 1) read 90% of words in a story correctly, 2) Answer three literal comprehension questions, and 3) Answer two inferential comprehension questions.

Foundational Numeracy Skills

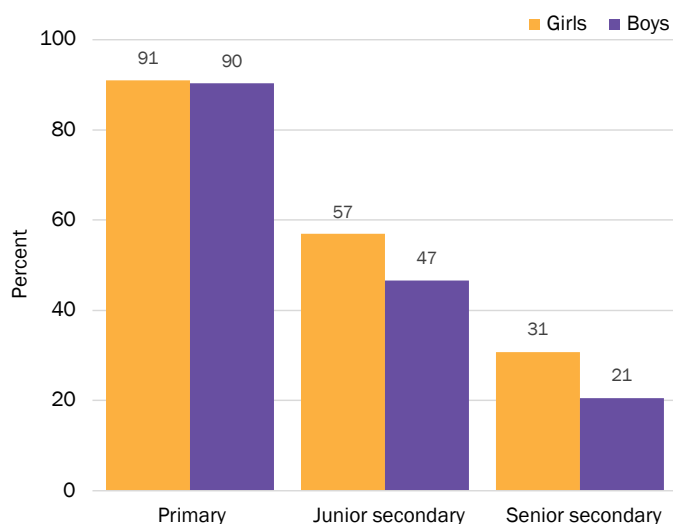


Percentage of children age 10-14 who can successfully perform 1) a number reading task, 2) a number discrimination task, 3) an addition task and 4) a pattern recognition and completion task.

Quality education and experiences at school positively affect physical and mental health, safety, civic engagement and social development. Adolescents, however, can also face the risk of school drop-out, early marriage or pregnancy, or being pulled into the workforce prematurely.

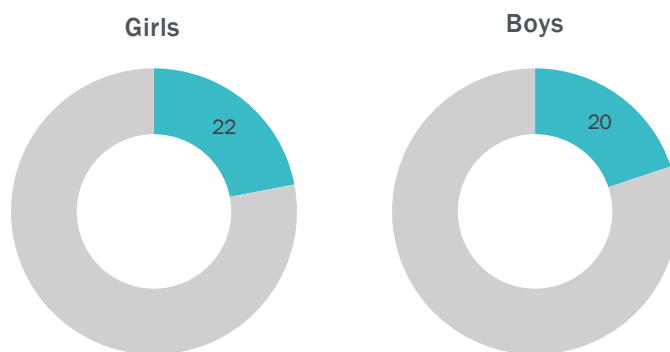
Data on reading and numeracy skills are collected in MICS through a direct assessment method. The Foundational Learning module captures information on children's early learning in reading and numeracy at the level of Grade 2 in primary education.

School Attendance Rates



Adjusted net attendance rate, by level of education and by gender.

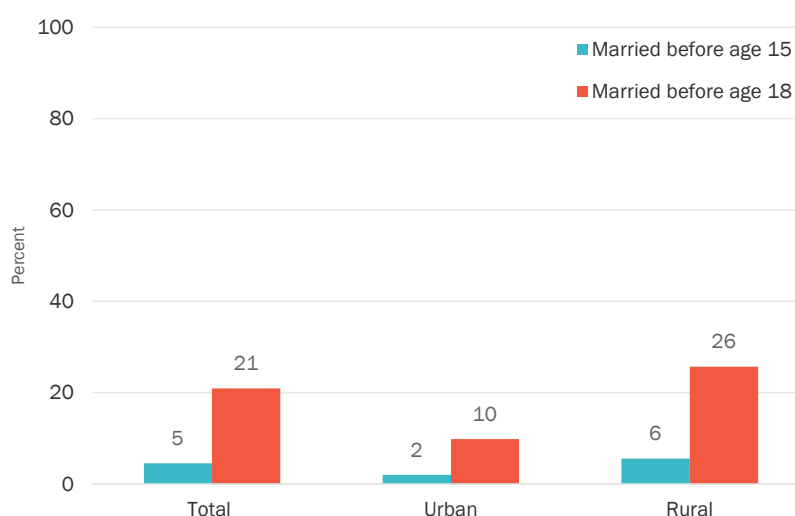
Information & Communications Technology (ICT) Skills*



Percentage of girls age 15-19 who in the last 3 months have performed at least one of nine specific computer related activities
*Age disaggregate of SDG 4.4.1: Proportion of youth and adults with information and communications technology (ICT) skills.

Every Adolescent is Protected from Violence & Exploitation

Child Marriage: SDG 5.3.1

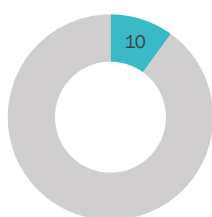


Percentage of women age 20-24 years who were first married or in union before age 15 and 18, by area.

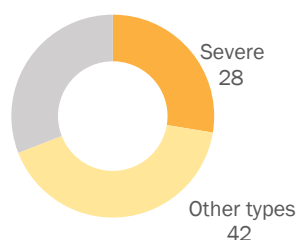
Adolescence is a period of heightened risk to certain forms of violence and exploitation. The onset of puberty marks an important transition in girls' and boys' lives whereby gender, sexuality and sexual identity begin to assume greater importance, increasing vulnerability to particular forms of violence, particularly for adolescent girls. Certain harmful traditional practices, such as female genital mutilation/cutting and child marriage, often take place at the onset of puberty. At the same time, as children enter adolescence, they begin to spend more time outside their homes and interact more intimately with a wider range of people, including peers and romantic partners. This change in social worlds is beneficial in many respects, but also exposes adolescents to new forms of violence.

Child Discipline

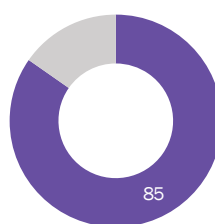
Only non-violent



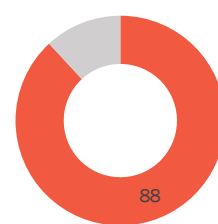
Physical punishment



Psychological aggression



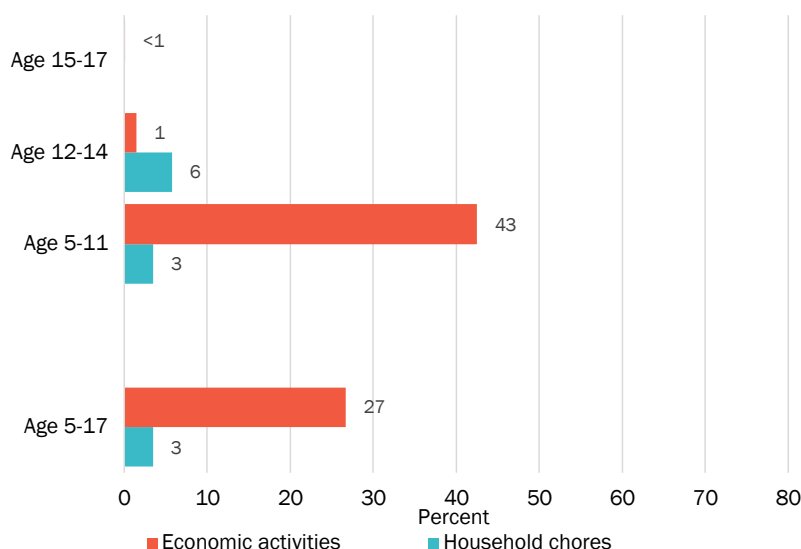
Any violent discipline*



Percentage of children age 10 to 14 years who experienced any discipline in the past month, by type.

*Age disaggregate of SDG 16.2.1.

Child Labour: SDG 8.7.1



Percentage of adolescents age 5-17 years engaged in child labour, by type of activity and by age.

Note: These data reflect the proportions of children engaged in the activities at or above the age specific thresholds outlined in the definitions box.

Definition of Child Labour

Age 5 to 11 years: At least 1 hour of economic activities or 21 hours of unpaid household services per week.

Age 12 to 14 years: At least 14 hours of economic activities or 21 hours of unpaid household services per week.

Age 15 to 17 years: At least 43 hours of economic activities. No threshold for number of hours of unpaid household services.

Economic activities include paid or unpaid work for someone who is not a member of the household, work for a family farm or business. Household chores include activities such as cooking, cleaning or caring for children.

Note that the child labour indicator definition has changed during the implementation of the sixth round of MICS. Changes include age-specific thresholds for household chores and exclusion of hazardous working conditions. While the overall concept of child labour includes hazardous working conditions, the definition of child labour used for SDG reporting does not.

Every Adolescent Lives in a Safe & Clean Environment

Water, Sanitation & Clean Fuel Use



The data presented here are at the household level. Evidence suggests that adolescent access to these services are comparable to household-level data.

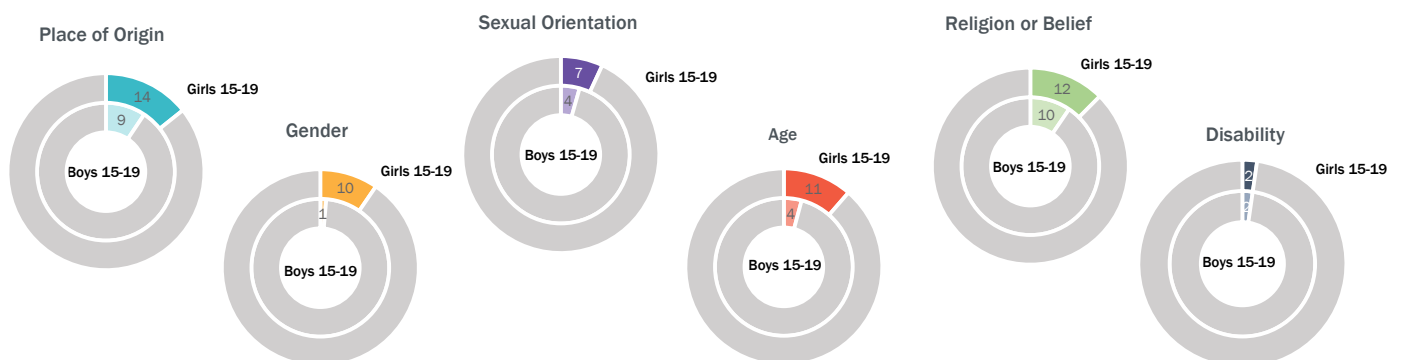
Basic Drinking Water SDG 1.4: Drinking water from an improved source, provided collection time is not more than 30 minutes for a roundtrip including queuing. Improved drinking water sources are those that have the potential to deliver safe water by nature of their design and construction, and include: piped water, boreholes or tubewells, protected dug wells, protected springs, rainwater, and packaged or delivered water.

Basic Sanitation Services SDG 1.4.1/6.2.1: Use of improved facilities which are not shared with other households. Improved sanitation facilities are those designed to hygienically separate excreta from human contact, and include: flush/pour flush to piped sewer system, septic tanks or pit latrines; ventilated improved pit latrines, composting toilets or pit latrines with slabs.

Clean Fuels SDG 7.1.2: Primary reliance on clean fuels and technologies for cooking, space heating and lighting.

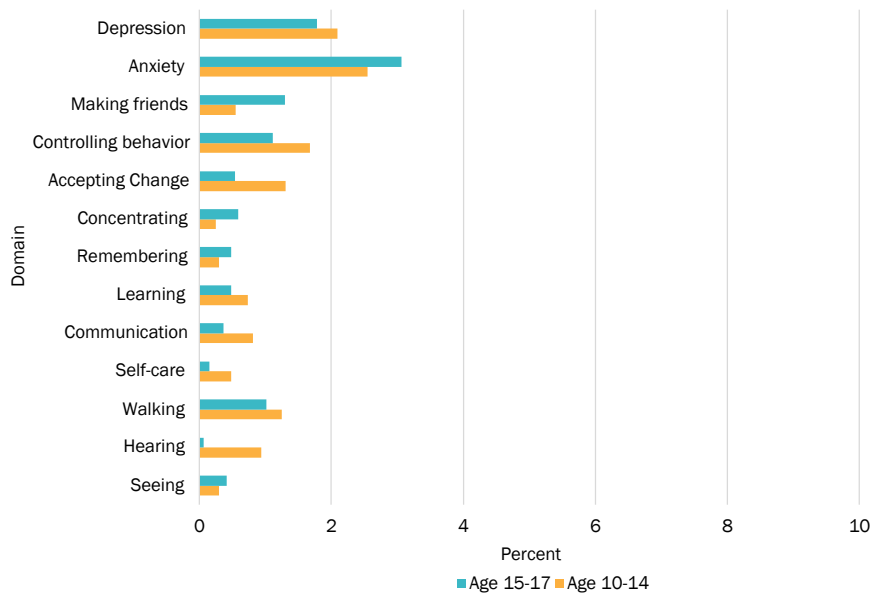
Every Adolescent has an Equitable Chance in Life

Discrimination & Harassment



Percentage of adolescent girls and boys age 15-19 years who in the last 12 months have felt discriminated against or harassed on the basis of different grounds.

Functioning Difficulties in Adolescents



Percentage of adolescents who have a functioning difficulty, by domain and age.

Achieving sustainable progress and results with regard to equity demands a human rights-based approach. At the core of international human rights legal framework is the principle of non-discrimination, with instruments to combat specific forms of discrimination, including against women, indigenous peoples, migrants, minorities, people with disabilities, and discrimination based on race and religion, or sexual orientation and gender identity. As adolescents begin to form more of an individual identity, discrimination can often become more pronounced, taking form in harassment, bullying, or exclusion from certain activities. At the same time, research has shown that discrimination during adolescence has a particularly strong effect on stress hormones, potentially leading to life-long mental or physical health side effects.

Children and adolescents with disabilities are one of the most marginalized groups in society. Facing daily discrimination in the form of negative attitudes, lack of adequate policies and legislation, adolescents with disabilities are effectively barred from realizing their rights to health, education, and even survival.

HIV & Sexual Behaviour

HIV indicators



Knowledge

Percent who know of the two ways of HIV prevention (having only one faithful uninfected partner and using a condom every time), who know that a healthy looking person can be HIV-positive, and who reject the two most common misconceptions

Stigma

Percent of those who report discriminatory attitudes towards people living with HIV, including 1) would not buy fresh vegetables from a shopkeeper or vendor who is HIV-positive and 2) think children living with HIV should not be allowed to attend school with children who do not have HIV

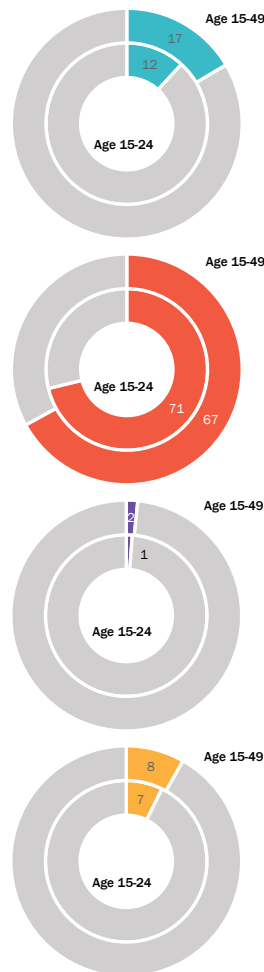
Testing

Percent who have been tested for HIV in the last 12 months and know the result

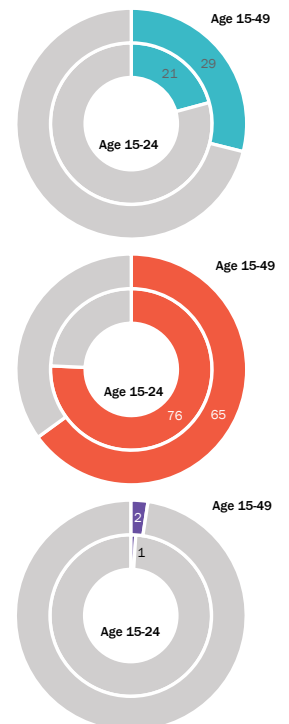
Testing during Antenatal Care

Percent of women who during their antenatal care for their last pregnancy were offered an HIV test, accepted and received results, and received post-test health information or counselling related to HIV

Women



Men

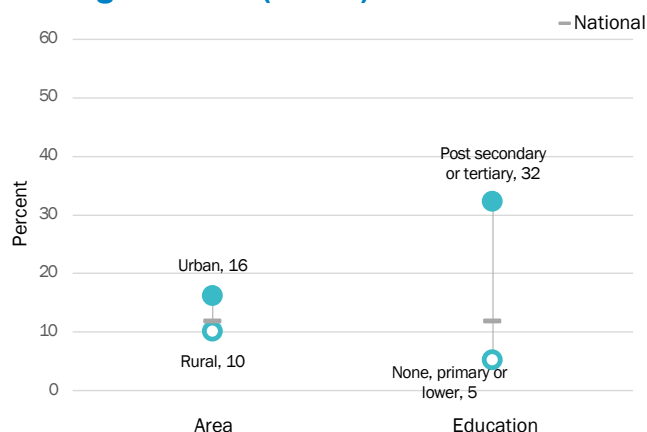


Key Messages

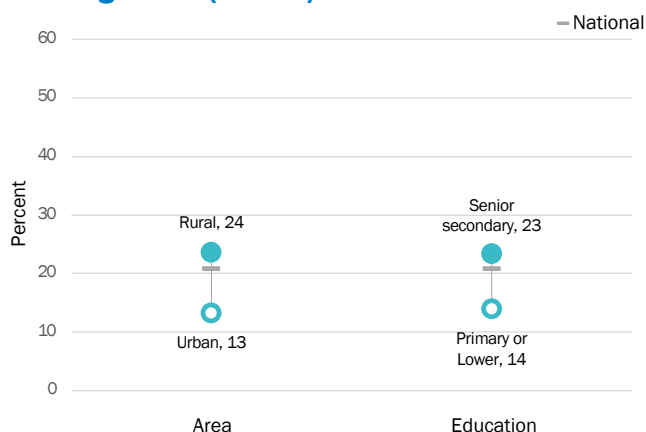
- Comprehensive knowledge about HIV is low among women (17 percent) and somewhat higher among men (29 percent), age 15-49 years.
- A high percentage of women (67 percent) and men (65 percent) age 15-49 years report discriminatory attitudes towards people living with HIV.
- About 2 percent of women and men age 15-49 years have been tested for HIV and know the results
- 8 percent of women age 15-49 years who sought antenatal care were offered and accepted a HIV test, received their result and were offered counselling relating to HIV

HIV Indicators by Key Characteristics

Knowledge among Adolescent Girls & Young Women (15-24)*

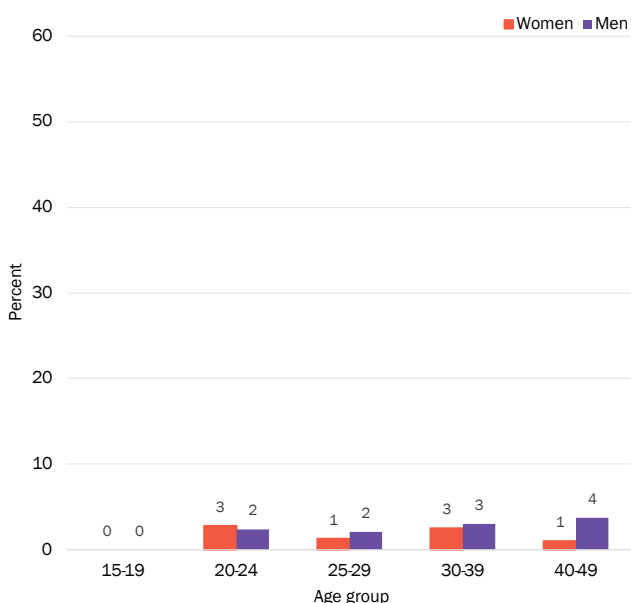


Knowledge among Adolescent Boys & Young Men (15-24)*



*Percent of population age 15-24 years who know two ways of HIV prevention, who know that a healthy-looking person can be HIV-positive, and who reject two most common misconceptions.

Tested for HIV in last 12 months



Percent age 15-49 who have been tested for HIV in the last 12 months and know the result, by age group.

Provincial Data on HIV Testing

Province	Men who tested in last 12 months	Women who tested in last 12 months	Women testing at ANC
National	2.4	1.6	8.2
Torba	0.6	0.4	(3.6)
Sanma	4.4	1.5	9.4
Penama	0.6	0.8	1.0
Malampa	0.0	1.2	7.5
Shefa	3.2	2.0	11.0
Tafea	0.8	1.8	8.1

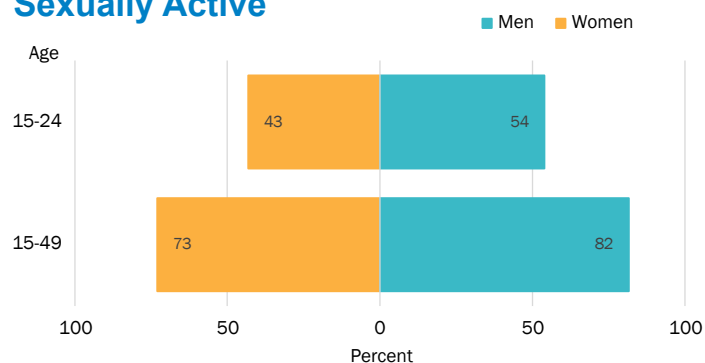
Tested in last 12 months: percent of persons age 15-49 who have been tested in the last 12 months and know the result.

HIV testing during ANC: percent of women age 15-49 who during antenatal care for their last pregnancy were offered an HIV test, accepted and received results, and received posttest health information or counselling related to HIV.

() Figures that are based on 25-49 unweighted cases.

Sexual Behaviour by Key Characteristics

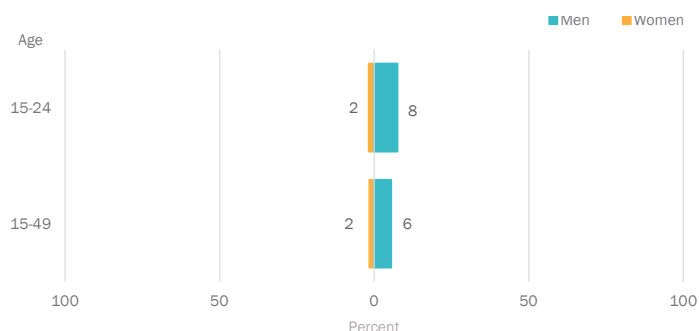
Sexually Active



Sexually active: Percent of women and men age 15-24 and 15-49 who had sexual intercourse within the last 12 months.

In many settings, sexual behaviour can be considered a risk factor for health and social issues. These include reproductive health, HIV and other sexually transmitted infections, and gender equality and empowerment. An understanding of the population's sexual behaviour patterns can inform both disease prevention and health promotion programmes.

Multiple Partners



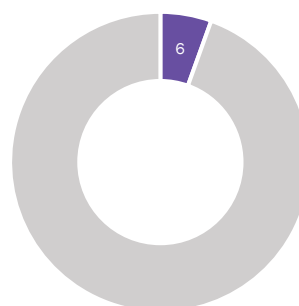
Multiple partners: Percent of women and men age 15-24 and 15-49 of those who had sex with more than 1 partner in the last 12 months.

Sex before age 15: Percent of women and men age 15-24 who had sex before age 15.

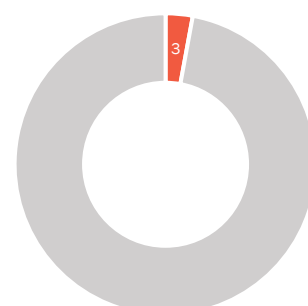
Sex with man 10 years or older: Percent of adolescent girls age 15-19 who had sex in the last 12 months who report having had sex with a man 10 or more years older in the last 12 months.

Young People who had Sex Before Age 15

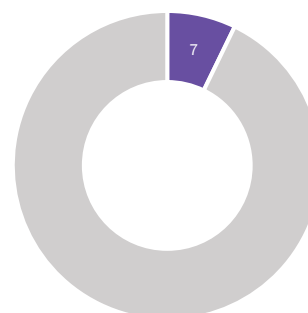
Adolescent boys & young men 15-24



Adolescent girls & young women 15-24



Girls age 15-19 years who Report Sex with Partner 10 or more years older



Key Messages

- Comprehensive knowledge of HIV is significantly higher among children and young people with higher education attainment.
- Girls and young women living in urban areas have more comprehensive knowledge of HIV compared to those living in rural areas. Interestingly the inverse is the case for boys and young men, those living in rural areas having more comprehensive knowledge of HIV compared to those living in urban areas.
- HIV testing during antenatal care visits is highest in Shefa province (11 percent) and lowest in Penama province (1 percent).
- Overall, the level of HIV testing in Vanuatu is very low (3 percent).
- Reporting of having sex with multiple partners during the last twelve months is 8 percent for males and 2 percent for females age 15-24 years.
- The incidence of having sex before the age of fifteen is very low for both males and females age 15-24 years, at 6 percent for males and 3 percent for females.

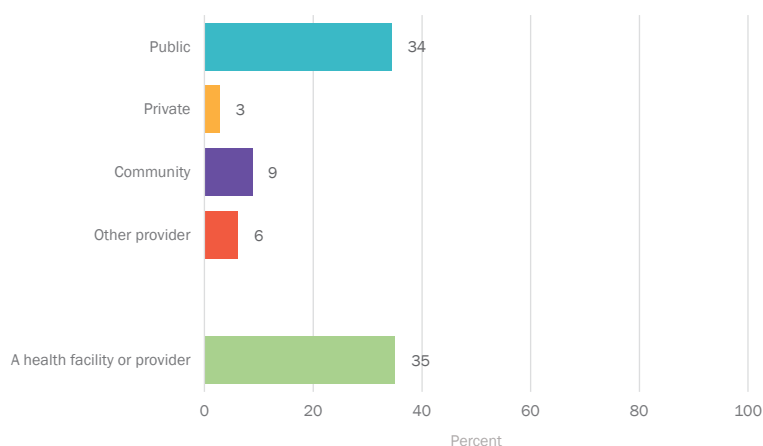


Child Health & Care of Illness



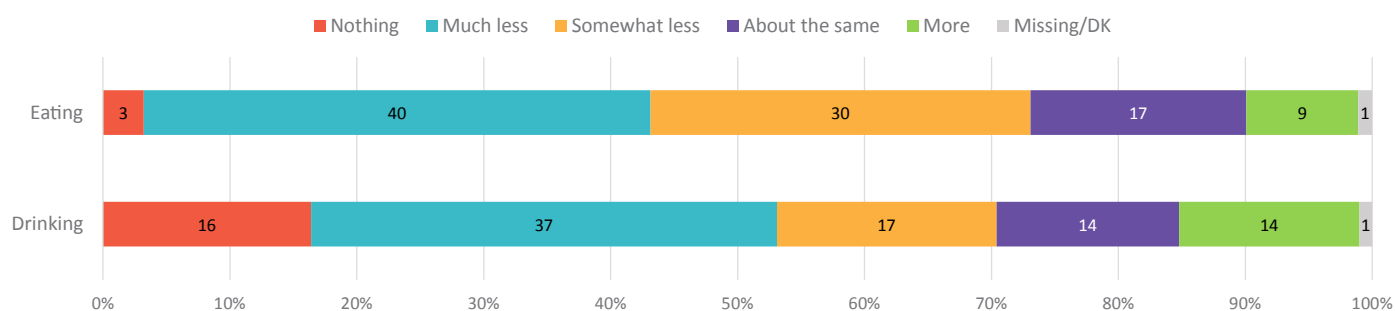
Diarrhoea

Care-seeking for Diarrhoea



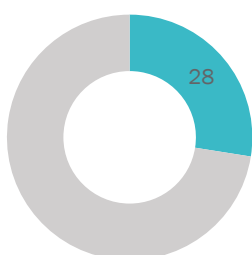
Percentage of children age 0-59 months with diarrhoea in the last two weeks for whom advice or treatment was sought by source of provider.

Feeding during Diarrhoea



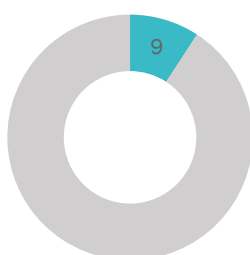
Percent distribution of children age 0-59 months with diarrhoea in the last two weeks by amount of liquids and food given during episode of diarrhoea.

ORS Treatment for Diarrhoea



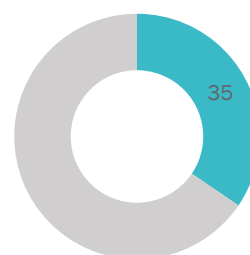
Percentage of children age 0-59 months with diarrhoea in the last two weeks treated with oral rehydration salt solution (ORS).

ORS + Zinc Treatment for Diarrhoea



Percentage of children age 0-59 months with diarrhoea in the last two weeks treated with oral rehydration salt solution (ORS) and zinc.

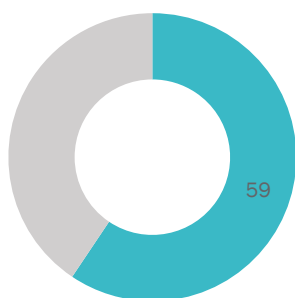
ORT + Continued Feeding for Diarrhoea



Percentage of children age 0-59 months with diarrhoea in the last two weeks who were given oral rehydration therapy (ORT) with continued feeding.

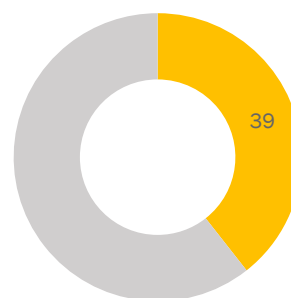
Malaria

Household Availability of Insecticide Treated Nets (ITNs)



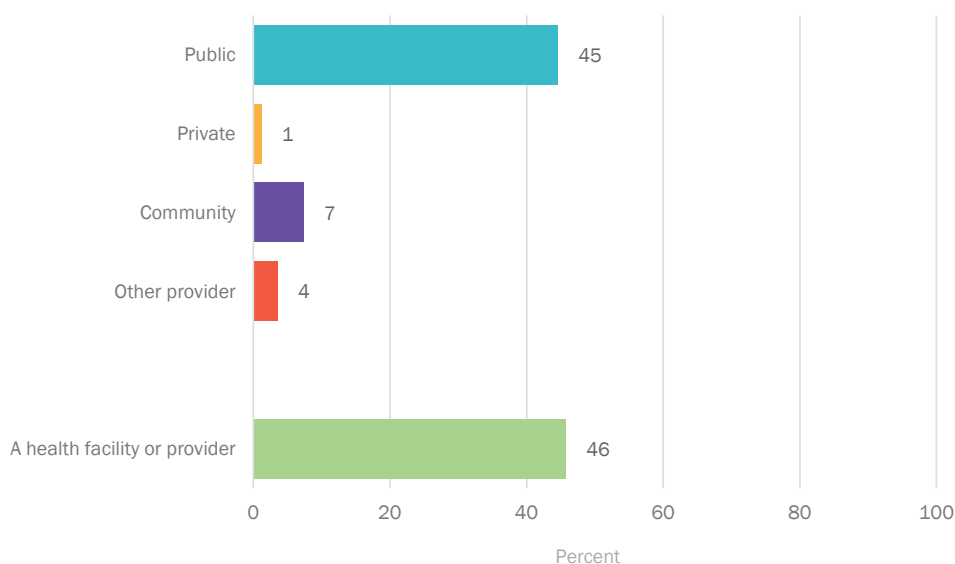
Percentage of households with at least one insecticide-treated net (ITN).

Children Under-Five who slept under an ITN



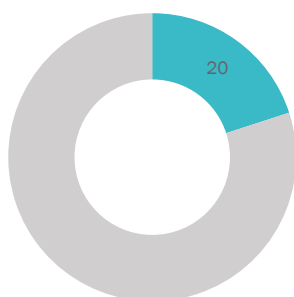
Percentage of children age 0-59 months who slept under an ITN last night.

Care-seeking during Fever



Percentage of children age 0-59 months with fever in the last two weeks for whom advice or treatment was sought, by source of advice or treatment.

Malaria Diagnosis Usage

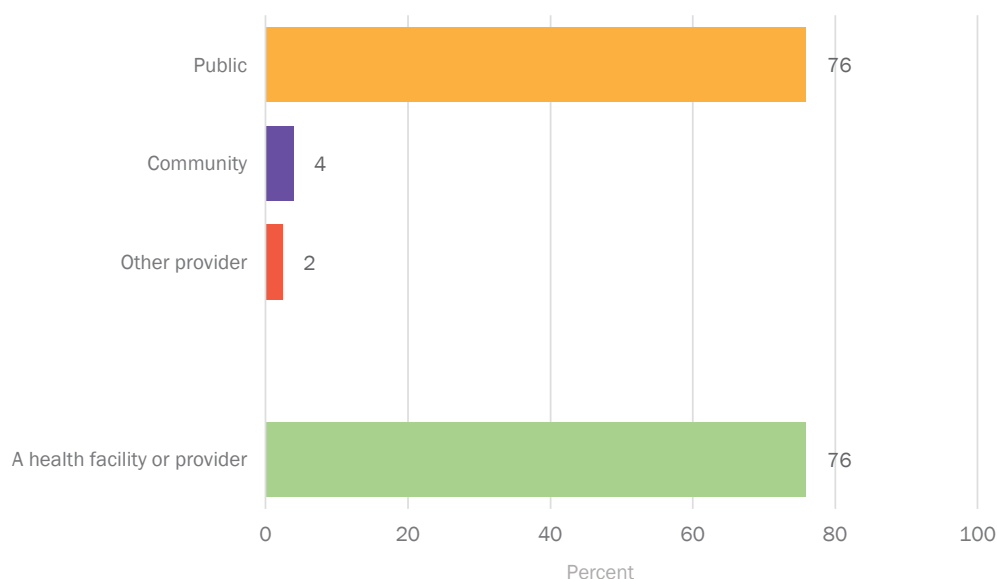


Percentage of children with fever who had blood taken from a finger or heel for testing.



Symptoms of Acute Respiratory Infection (ARI)

Care-seeking for Symptoms of ARI



Percentage of children age 0-59 months with symptoms of ARI in the last two weeks for whom advice or treatment was sought, by source of advice or treatment.

Key Messages

- In terms of illnesses among children under 5 years in the last two weeks, the incidence of an episode of diarrhoea was 6 percent, symptoms of Acute Respiratory Infection (ARI) 3 percent and an episode of a fever 11 percent (see Table TC 2.1 of the Survey Finding Report). The low number of cases means achieving statistically significant breakdowns. For all three types of illnesses, majority of children received medical intervention from a public health facility.
- In relation to diarrhoea, the most used treatment was Oral Rehydration Therapy (ORT) and continued feeding (35 percent).
- Almost three out of five households (59 percent) have an insecticide treated mosquito net (ITN) and 39 percent of children under-five years slept under an ITN the night before the MICS interview took place.

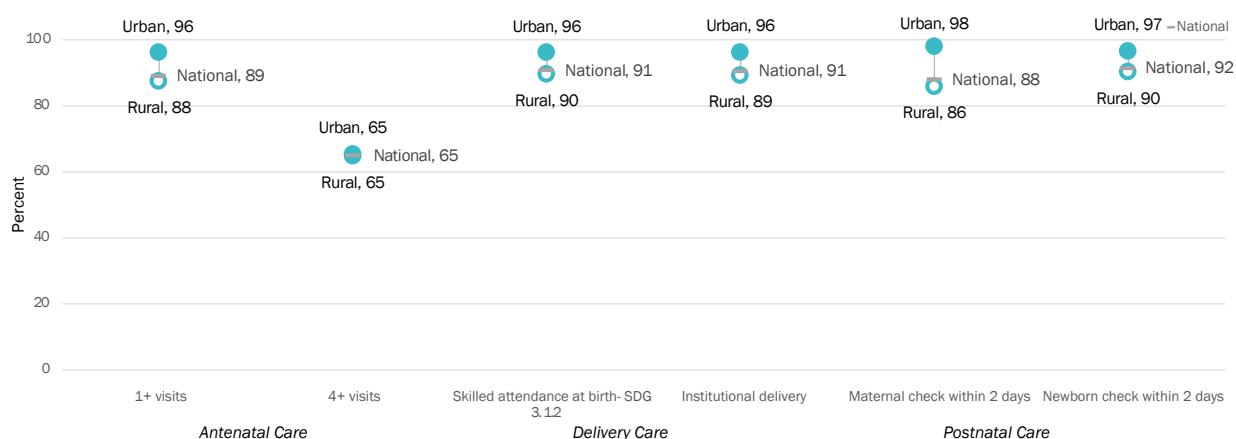


Maternal & Newborn Health



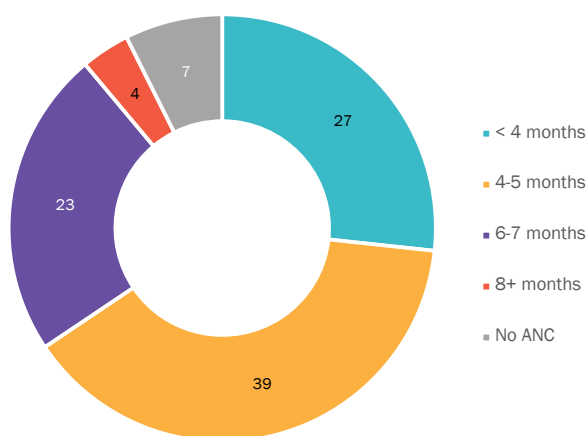
Key Elements of Maternal & Newborn Health

Maternal & Newborn Health Cascade by Area



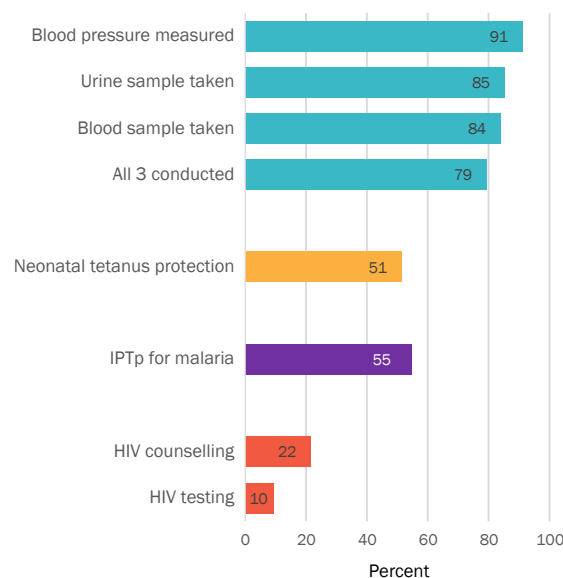
Percentage of women age 15-49 years with a live birth in the last 2 years who were attended during their last pregnancy that led to a live birth at least once by skilled health personnel or at least four times by any provider, who were attended by skilled health personnel during their most recent live birth (SDG 3.1.2), whose most recent live birth was delivered in a health facility, who received a health check while in facility or at home following delivery, or a post-natal care visit within 2 days after delivery of their most recent live birth and percentage of last live births in the last 2 years who received a health check while in facility or at home following delivery, or a post-natal care visit within 2 days after delivery, by area.

Timing of First Antenatal Care Visit



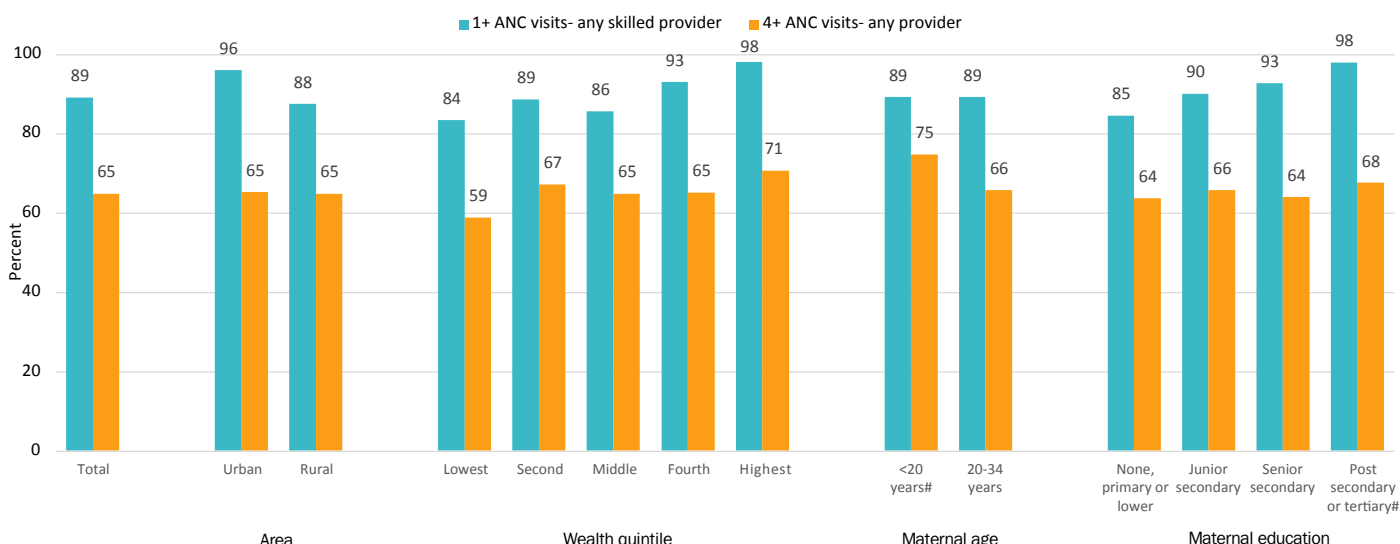
Percentage of women age 15-49 years with a live birth in the last 2 years who were attended during their last pregnancy that led to a live birth at least once by skilled health personnel, by the timing of first ANC visit.

Content & Coverage of Antenatal Care Services



Percentage of women age 15-49 years with a live birth in the last 2 years who had their blood pressure measured and gave urine and blood samples, were given at least two doses of tetanus toxoid vaccine within the appropriate interval, took three or more doses of SP/Fansidar to prevent malaria, reported that during an ANC visit they received information or counselling on HIV, and reported that they were offered and accepted an HIV test during antenatal care and received their results during the last pregnancy that led to a live birth.

Coverage of Antenatal Care by Various Characteristics

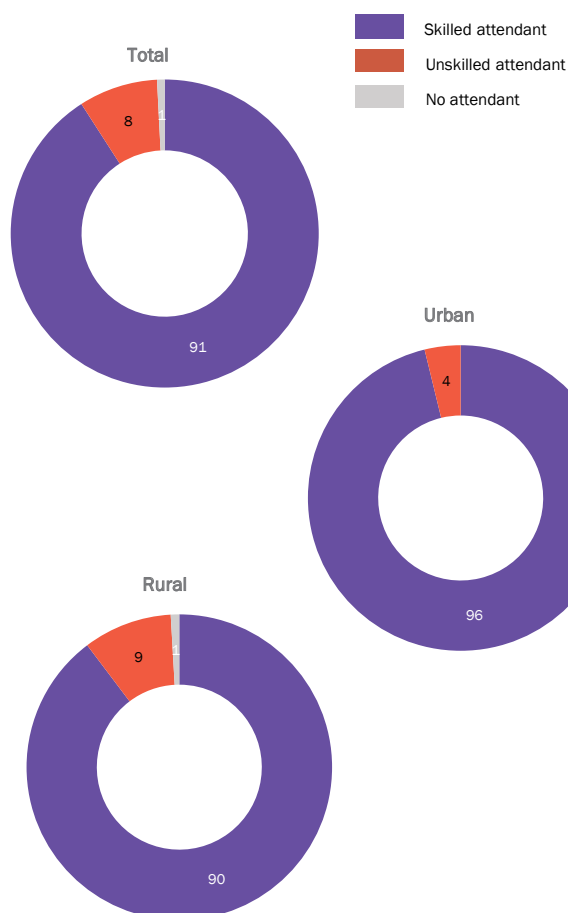


Percentage of women age 15-49 years with a live birth in the last 2 years who were attended during their last pregnancy that led to a live birth at least once by skilled health personnel or at least four times by any provider.

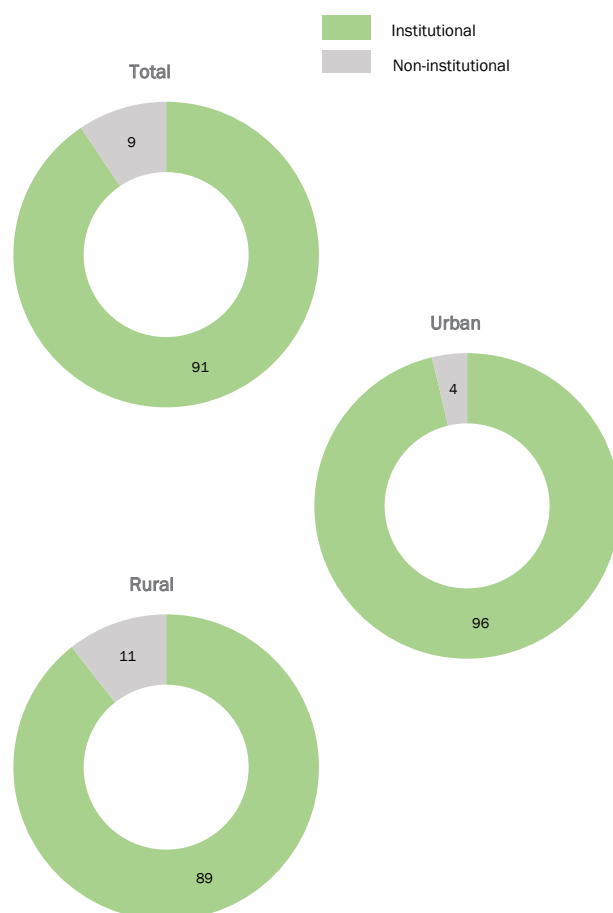
Figures based on 25-49 unweighted cases.

Coverage of Skilled Attendance at Birth & Institutional Delivery by Area

Skilled Attendance at Birth

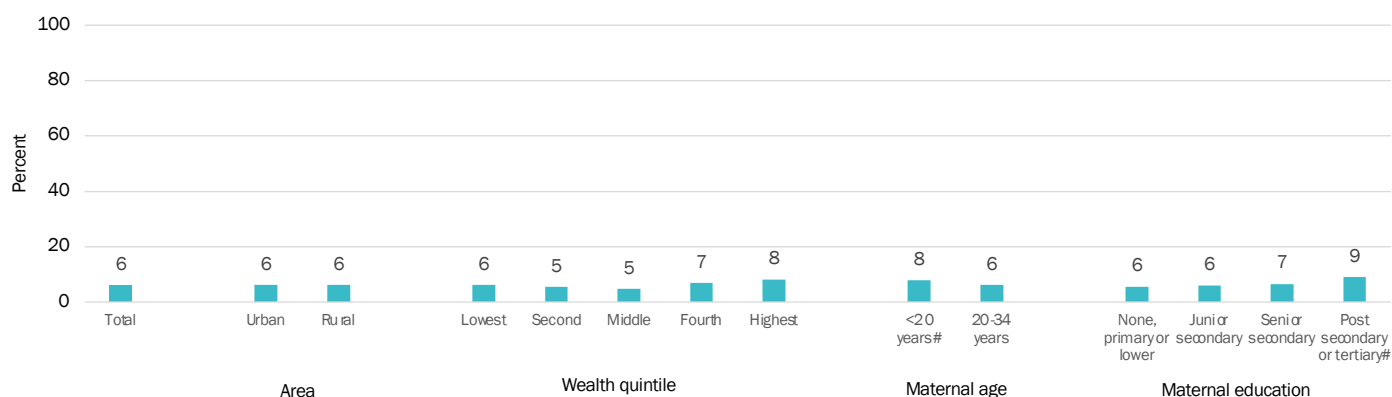


Institutional Delivery



Percentage of women age 15-49 years with a live birth in the last 2 years who were attended by skilled health personnel during their most recent live birth and percentage whose most recent live birth was delivered in a health facility (institutional delivery) by area.

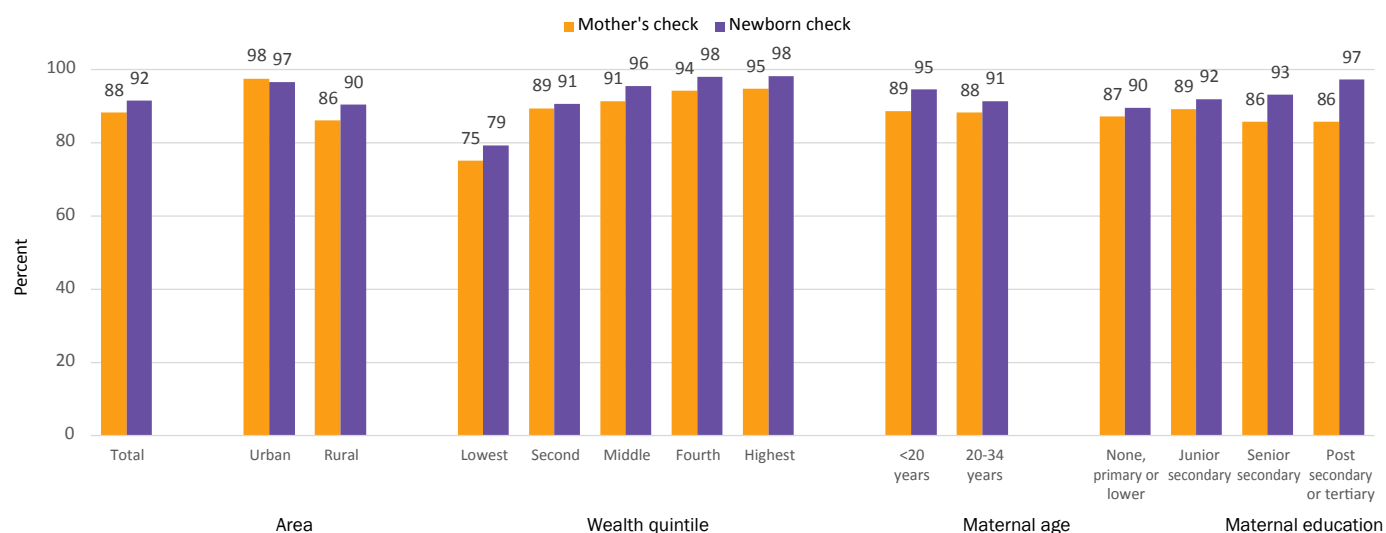
Caesarian Section by Various Characteristics



Percentage of women age 15-49 years with a live birth in the last 2 years whose most recent live birth was delivered by caesarean section by various characteristics.

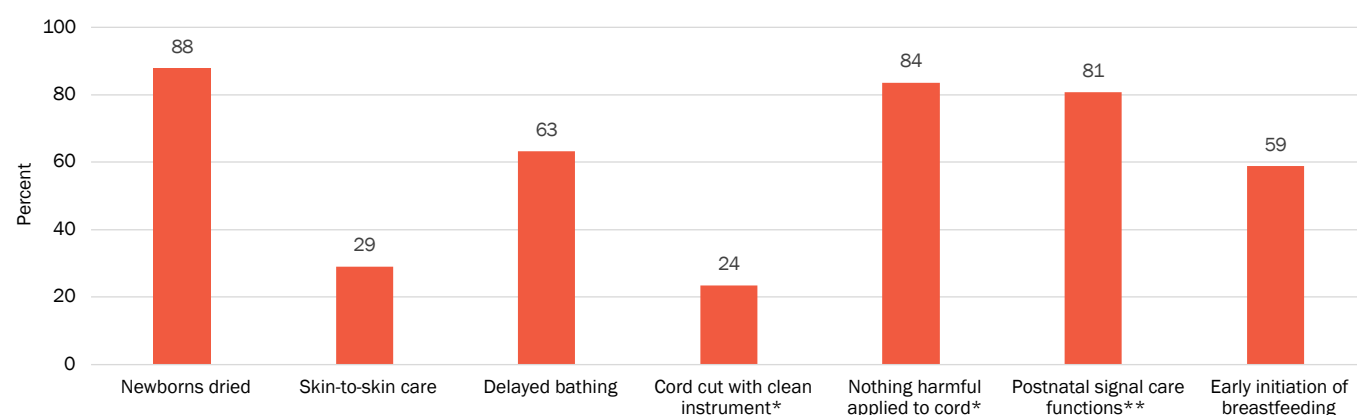
Figures based on 25-49 unweighted cases.

Postnatal Care within 2 Days of Birth by Various Characteristics



Percentage of women age 15-49 years with a live birth in the last 2 years who received a health check while in facility or at home following delivery, or a post-natal care visit within 2 days after delivery of their most recent live and percentage of last live births in the last 2 years who received a health check while in facility or at home following delivery, or a post-natal care visit within 2 days after delivery, by various characteristics.

Coverage of Newborn Care



Among the last live-birth in the last 2 years, percentage who were dried after birth; percentage who were given skin to skin contact; percentage who were bathed after 24 hours of birth; percentage where the umbilical cord was cut with a new blade or boiled instrument*; percentage where nothing harmful was applied to the cord*; percentage where the newborn received at least 2 postnatal signal care functions within 2 days after birth**; and percentage put to the breast within one hour of birth.

* Among the last live-births in the last 2 years delivered outside a facility.

** At least 2 of i) umbilical cord examination, ii) temperature assessment, iii) breastfeeding counselling or observation, iv) weight assessment, and v) counselling on danger signs for newborns.

Provincial Data on Maternal and Newborn Cascade

Province	ANC: At least 1 visit (skilled provider)	ANC: At least 4 visit (any provider)	Skilled Attendance at Delivery	institutional Delivery	Post-natal Care for Mother <2 days	Post-natal Care for Newborn <2 days
National	89.2	65.0	90.9	90.6	88.2	91.5
Torba	(70.2)	(59.7)	(73.7)	(94.0)	(88.2)	(94.3)
Sanma	84.7	62.1	92.8	95.1	97.0	98.5
Penama	92.8	82.2	92.7	93.7	88.1	93.2
Malampa	92.4	70.2	93.9	95.6	95.3	98.5
Shefa	91.8	67.0	95.2	95.6	94.6	96.8
Tafea	87.5	50.9	81.2	72.7	65.0	70.6

For indicator definitions, see earlier charts.

() Figures that are based on 25-49 unweighted cases.

Key Messages

- In terms of antenatal care (ANC) visits, almost two-fifths of these took place during the fourth - fifth month of the pregnancy.
- During the ANC visits the test that was completed most often was the measurement of blood pressure (91 percent) and the least was HIV counselling (22 percent) and HIV testing (10 percent).
- Around 51 percent of women received a neonatal tetanus injection as part of their antenatal care.
- Nine in ten women (91 percent) age 15-49 who had a live birth in the last 2 years, had their delivery in a health facility. The same percentage of deliveries were assisted by a skilled professional.
- Women living in rural areas were twice as likely to have an unskilled attendant during the birth as women living in towns (9 percent compared to 4 percent).
- The level of caesarian births is low (6 percent). It is slightly higher for younger women and for women in households in the highest wealth quintile.
- Levels of post natal care varied more widely by different characteristics. For example, a check of the mother taking place within two days of the birth took place in three-quarters (75 percent) of women living in the lowest wealth quintile households, compared to almost universal coverage (95 percent) for women in the highest wealth quintile.
- Tafea province is below average on all six measures of the maternal and newborn cascade.

Infant & Young Child Feeding (IYCF)



Infant & Young Child Feeding

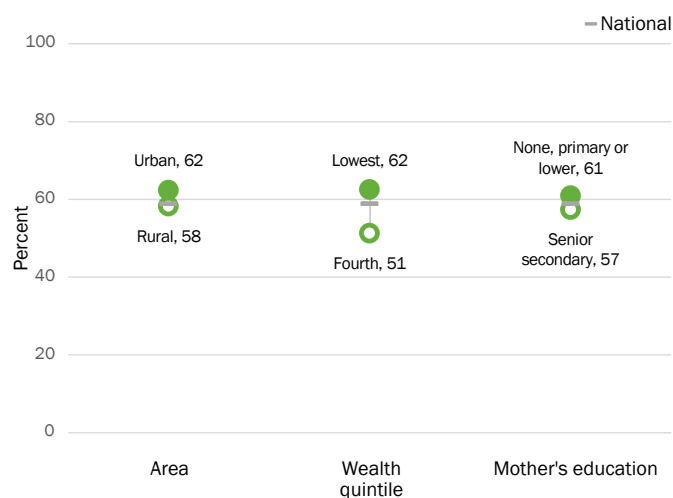


Early initiation: percentage of newborns put to breast within 1 hour of birth; **Exclusive breastfeeding:** percentage of infants age 0-5 months receiving only breastmilk; **Introduction to solids:** percentage of infants age 6-8 months receiving solid or semi-solid food; **Minimum diet diversity:** percentage of children age 6-23 months receiving 5 of the 8 recommended food groups; **Minimum meal frequency:** percentage of children age 6-23 months receiving the recommended minimum number of solid/liquid feeds as per the age of child; **Minimum acceptable diet:** percentage of children age 6-23 months receiving the minimum diversity of foods and minimum number of feeds; **Continued breastfeeding at 1 year:** percentage of children age 12-15 months who continue to receive breastmilk; **Continued breastfeeding at 2 years:** percentage of children age 20-23 months who continue to receive breastmilk.



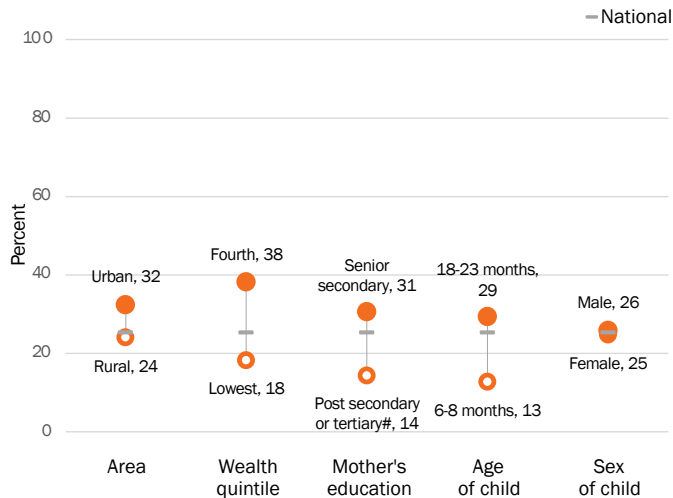
IYCF: Equity

Early Initiation of Breastfeeding



Percent of newborns put to the breast within one hour of birth, by background characteristics.

Minimum Diet Diversity



Percent of children age 6-23 months that were fed food from at least 5 out of 8 food groups, by background characteristics.
Figures based on 25-49 unweighted cases.

Provincial Data

Province	Early Initiation of breastfeeding	Minimum Diet Diversity
National	58.8	25.2
Torba	(64.1)	(40.2)
Sanma	49.1	17.8
Penama	61.1	12.3
Malampa	52.1	39.1
Shefa	60.3	31.3
Tafea	67.5	23.4

Percent of newborns put to the breast within one hour of birth, and percent of children age 6-23 months that were fed food from at least 5 out of 8 food groups by province.

() Figures that are based on 25-49 unweighted cases.

Key Messages

- Overall, 59 percent of newborns during the two years preceding the survey are breastfed within the first hour of birth. Early initiation of breastfeeding is highest in Tafea province and lowest in Sanma.
- Early initiation to breastfeeding is higher for children born to the least wealthy households (62 percent) compared to those in the fourth wealth quintile (51 percent).
- More than three-quarters (77 percent) of children age 0-5 months are exclusively breastfed.
- Among children age 6-23 months, 23 percent receive the recommended minimum number of feeds, but 25 percent receive five out of the eight recommended food groups, while only 9 percent of them receive both the minimum diversity of foods and the minimum number of feeds.

Nutritional Status of Children

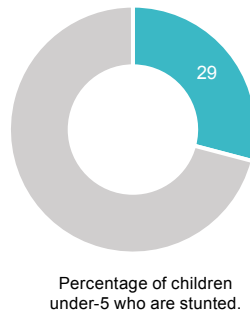


Anthropometric Malnutrition Indicators

Stunting: SDG 2.2.1



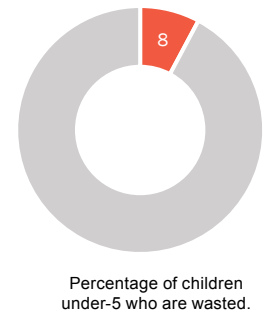
Stunting refers to a child who is too short for his or her age. Stunting is the failure to grow both physically and cognitively and is the result of chronic or recurrent malnutrition.



Wasting: SDG 2.2.2



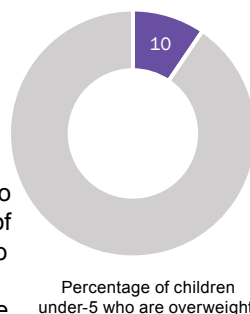
Wasting refers to a child who is too thin for his or her height. Wasting, or acute malnutrition, is the result of recent rapid weight loss or the failure to gain weight. A child who is moderately or severely wasted has an increased risk of death, but treatment is possible.



Overweight: SDG 2.2.2



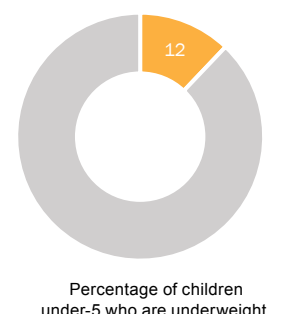
Overweight refers to a child who is too heavy for his or her height. This form of malnutrition results from expending too few calories for the amount consumed from food and drinks and increases the risk of noncommunicable diseases later in life.



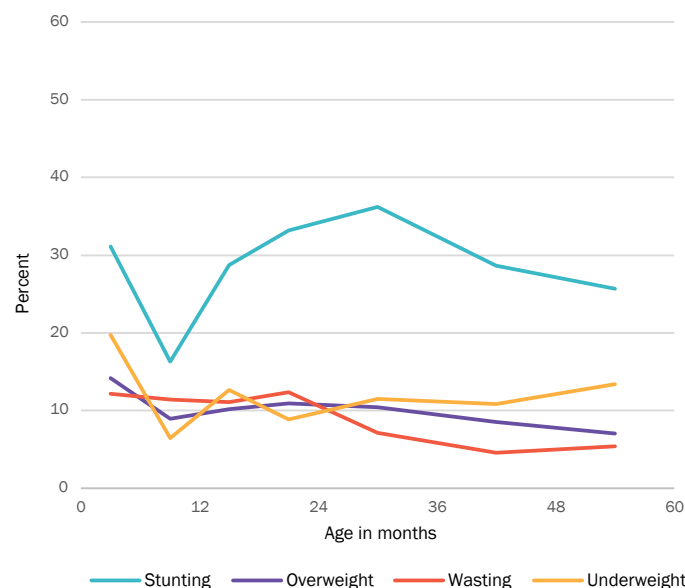
Underweight



Underweight is a composite form of undernutrition that can include elements of stunting and wasting (i.e. an underweight child can have a reduced weight for their age due to being too short for their age and/or being too thin for their height).



Anthropometric Malnutrition Indicators by Age



Percentage children who are underweight, stunted, wasted and overweight, by age in months.

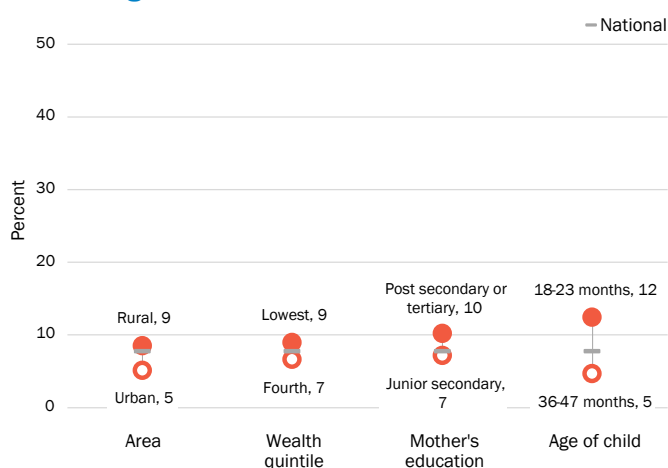
Nutritional Status of Children: Disaggregates

Stunting: SDG 2.2.1



Percentage of under 5 children who are stunted, by background characteristics.

Wasting: SDG 2.2.2



Percentage of under 5 children who are wasted, by background characteristics.

Provincial Data on Stunting, Overweight & Wasting

Province	Stunting: SDG 2.2.1	Overweight: SDG 2.2.2	Wasting	
	% stunted (moderate and severe)	% overweight (moderate and severe)	% wasted (moderate and severe, SDG 2.2.2)	% wasted (severe)
National	29.1	9.5	7.8	3.6
Torba	29.9	23.9	11.1	5.7
Sanma	21.5	7.4	11.2	5.9
Penama	24.3	4.4	6.5	3.6
Malampa	28.9	5.9	15.2	6.5
Shefa	30.8	10.7	6.6	2.9
Tafea	37.6	14.1	3.2	0.7

Key Messages

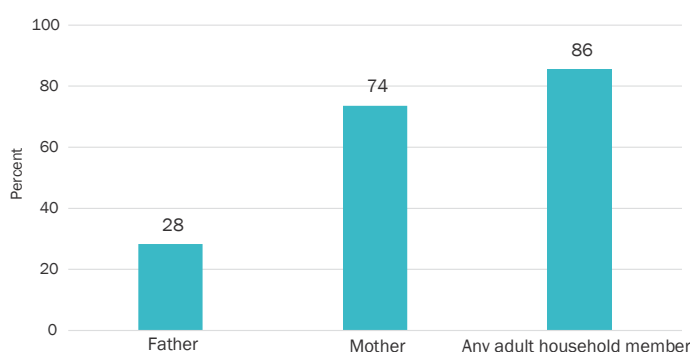
- In Vanuatu, almost a third (29 percent) of children under five are stunted. This means they fail to grow physically and cognitively, due to chronic or recurrent malnutrition. Stunting is at the peak between age two to three years. There are no major disparities between different groups, except in relation to province where the prevalence in Tafea rises to almost two out of five children being stunted (38 percent).
- The levels of being underweight, affected by wasted or overweight are a lot lower than the prevalence of stunting.
- Wasting is a severe form of malnutrition and consists of recent rapid weight loss or the failure to gain weight. In Vanuatu, the peak of wasting occurs just before the second birthday and is most prevalent in Malampa province. Wasting increases the risk of death for children, but treatment is possible.

Early Childhood Development (ECD)



Early Stimulation and Early Childhood Education

Early Stimulation & Responsive Care



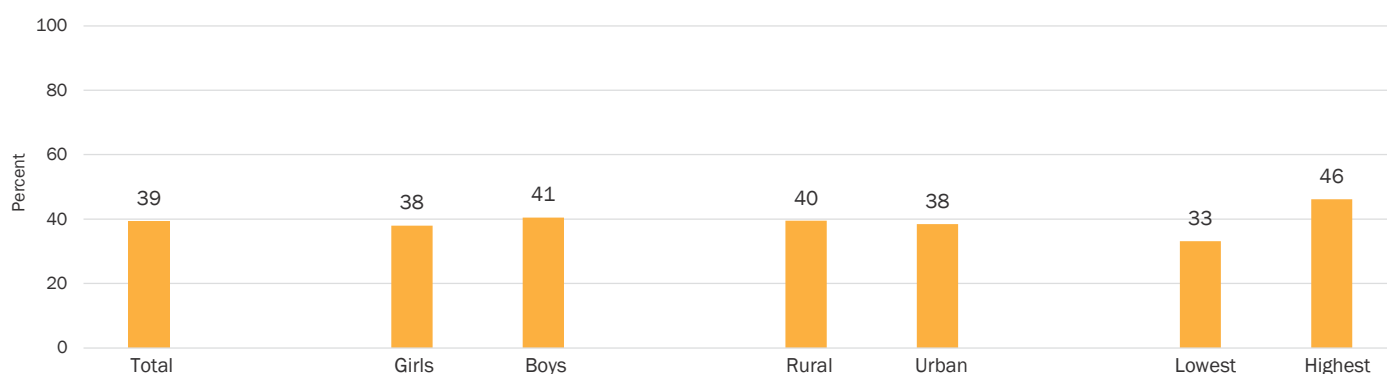
Percentage of children age 2-4 years with whom the father, mother or adult household members engaged in activities that provide early stimulation and responsive care during the last three days.

Note: Activities include: reading books to or looking at picture books with the child; telling stories to the child; singing songs to or with the child; taking the child outside the home; playing with the child; naming, counting or drawing things for or with the child.

Early childhood, which spans the period up to 8 years of age, is critical for cognitive, social, emotional, and physical development. During these years, a child's newly developing brain is highly plastic and responsive to change. Optimal early childhood development requires a stimulating and nurturing environment, access to books and learning materials, interactions with responsive and attentive caregivers, adequate nutrients, access to good quality early childhood education, and safety and protection. All these aspects of the environment contribute to developmental outcomes for children.

A broad range of factors can prevent children from reaching their full developmental potential. These risks are often interrelated and include poverty, poor health, exposure to violence and high stress levels, inadequate care and limited learning opportunities. Timely and effective interventions can prevent these risks and address the barriers disproportionately affecting children living in the most vulnerable contexts. Investments during the early years are one of the most cost-effective ways countries can reduce inequalities among children and promote the best start in life for all.

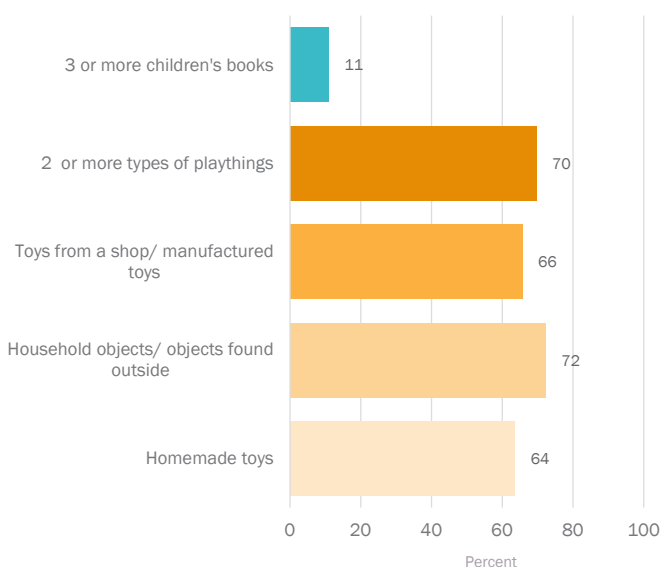
Attendance at Early Childhood Education Programmes



Percentage of children age 3-4 years attending an early childhood education programme, by background characteristics.

Access to Books and Playthings, and Child Supervision

Toys and children's books



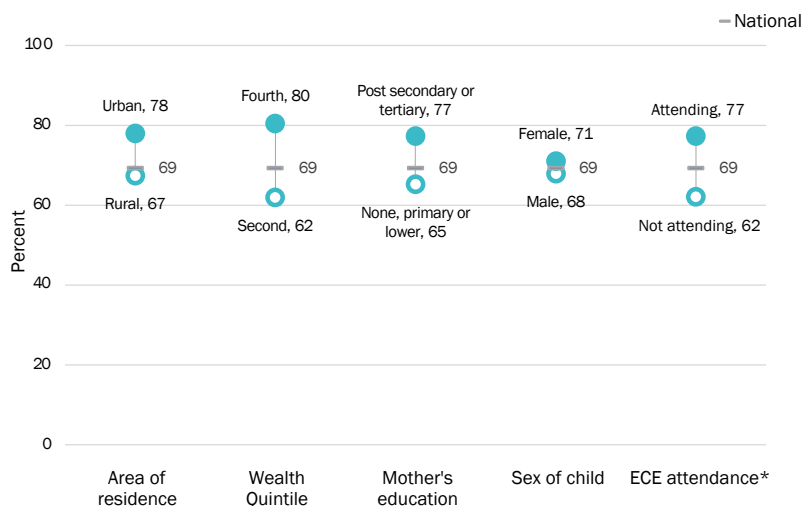
Percentage of children under age five according to the number of children's books available in their homes, and their access to different types of playthings and toys.

Inadequate supervision of children

Province	Receive inadequate supervision
National	29.2
Torba	24.5
Sanma	17.2
Penama	49.2
Malampa	38.1
Shefa	24.8
Tafea	29.2

Percentage of children under age five left alone or under the supervision of another child younger than 10 years of age for more than one hour at least once in the last week, by province.

Early Childhood Development Index 2030 (ECDI2030)



Percentage of children age 2-4 years who have achieved the minimum number of milestones expected for their age group, by background characteristics.

* Children age 2 are excluded, as early childhood education (ECE) attendance is only collected for age 3-4 years.

The ECDI2030 captures the achievement of key developmental milestones by children between the ages of 24 and 59 months.

The measure includes 20 questions about the way children behave in certain everyday situations, and the skills and knowledge they have acquired, reflecting the increasing difficulty of the skills children acquire as they grow. The 20 items are organized according to the three general domains of health, learning and psychosocial well-being. A child is considered to be developmentally on track if they have achieved the minimum number of milestones expected for their age group.

The data generated by the ECDI2030 can be used for monitoring and reporting on SDG indicator 4.2.1, and to inform government efforts to improve developmental outcomes among young children.

<https://data.unicef.org/resources/early-childhood-development-index-2030-ecdi2030/>

Key Messages

- Mothers play three times more of a role in early stimulation of young children than fathers.
- Less than half of households (39 percent) send their young children to early childhood education programmes. The differences between the subgroups of area and gender of the child are small. The biggest difference is seen in relation to the level of wealth of the household.
- Generally, there appears to be an absence of books in households with young children and almost a third (30 percent) of households do not have two playthings for their young child.
- There is quite a high proportion of children under five left alone or under the supervision of another child younger than 10 years of age for more than one hour at least once in the last week (29 percent). The incidence is highest in Penama and lowest in Sanma.
- Overall, 69 percent of children age 24-59 months are developmentally on track in health, learning and psychosocial wellbeing.

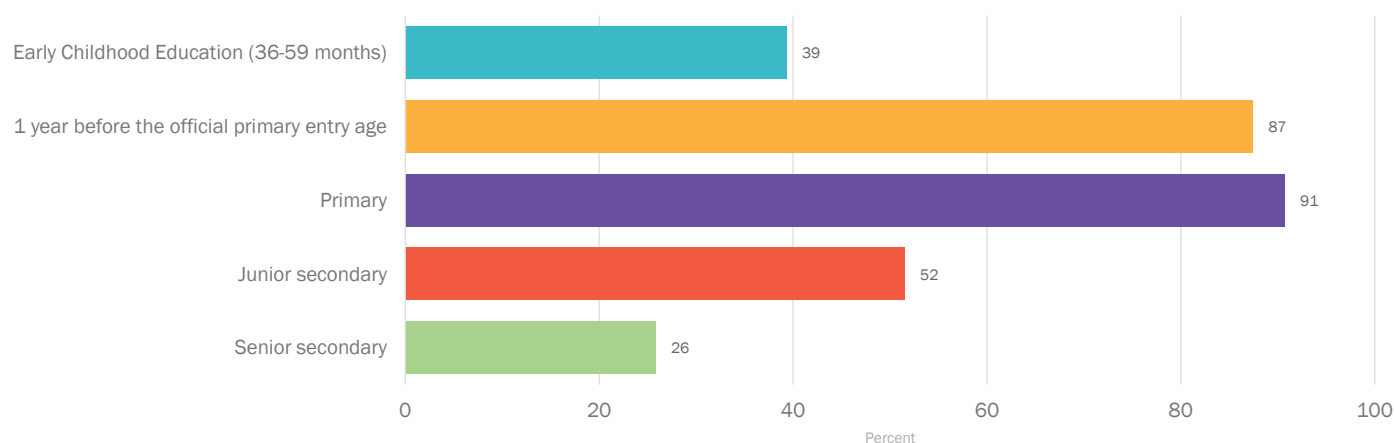


Education



Attendance Rates & Inequalities

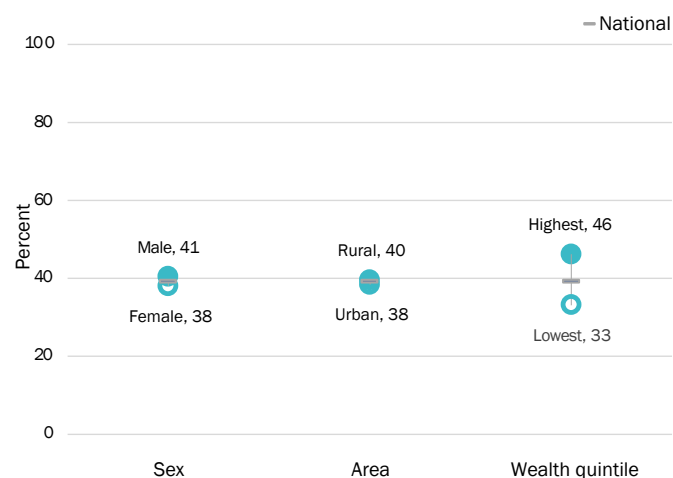
School Net Attendance Rates (adjusted)



Percentage of children of intended age for level of education attending level of education for age or higher, by level of education.

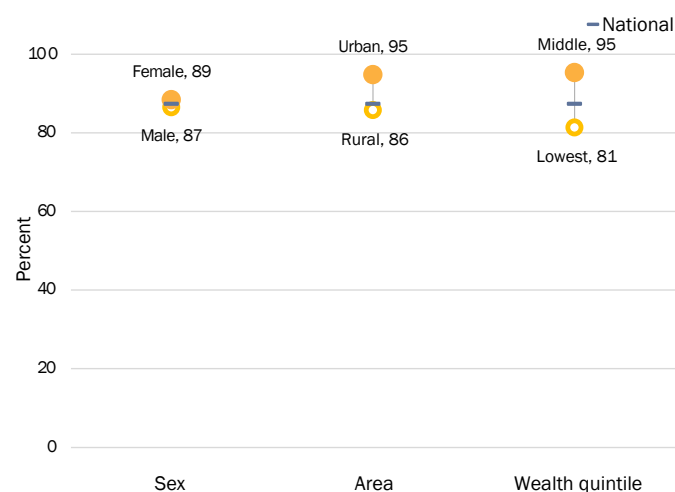
Inequalities in Attendance in Early Childhood Education & Participation in Organized Learning

Early Childhood Education Attendance Rate (age 3-4)



Percentage of children age 36-59 months who are attending early childhood education.

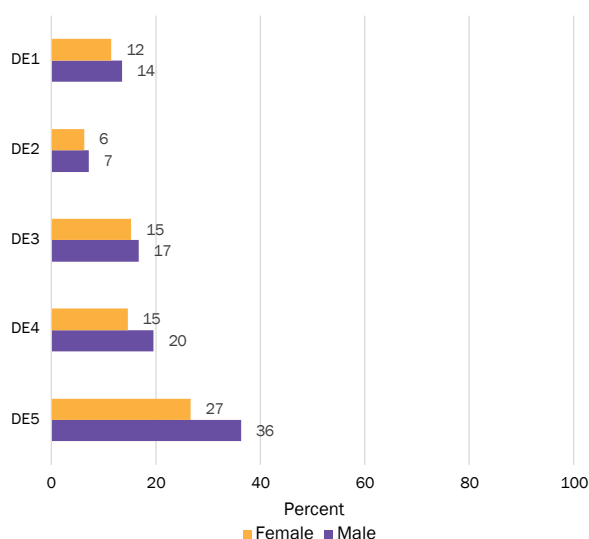
Participation Rate in Organised Learning (1 Year Before the Official Primary Entry Age): SDG 4.2.2



Percentage of children age one year younger than the official primary school entry age at the beginning of the school year who are attending an early childhood education programme or primary school (adjusted net attendance rate).

Out of School Rates

Out of School Dimensions for Levels of Education



Dimension 1: Children age one year younger than primary entry age not attending an early childhood education programme or primary school.

Dimension 2: Children of primary school age who are not attending any level of education.

Dimension 3: Children of junior secondary school age who are not attending any level of education.

Dimension 4: Children who are in primary school but at risk of dropping out (over-age for grade by 2 or more years).

Dimension 5: Children who are in junior secondary school but at risk of dropping out (over-age for grade by 2 or more years).

SDG Summary for Education

SDG	MICS Indicator	Definition & Notes	Value		
			Primary	Junior Secondary	Senior Secondary
4.1.2	LN.8a,b,c	Completion rate	81%	45%	14%
4.5.1	LN.5a	Gender Parity Indices (attendance, girls/boys)	1.01	1.22	1.50
4.5.1	LN.5b	Wealth Parity Indices (attendance, lowest/highest)	0.87	0.34	0.07
4.5.1	LN.5c	Area Parity Indices (attendance, rural/urban)	0.98	0.66	0.46
			Total	Boys	Girls
4.2.2	LN.2	Participation rate in organized learning (one year before the official primary entry age)	87%	87%	89%

Key Messages

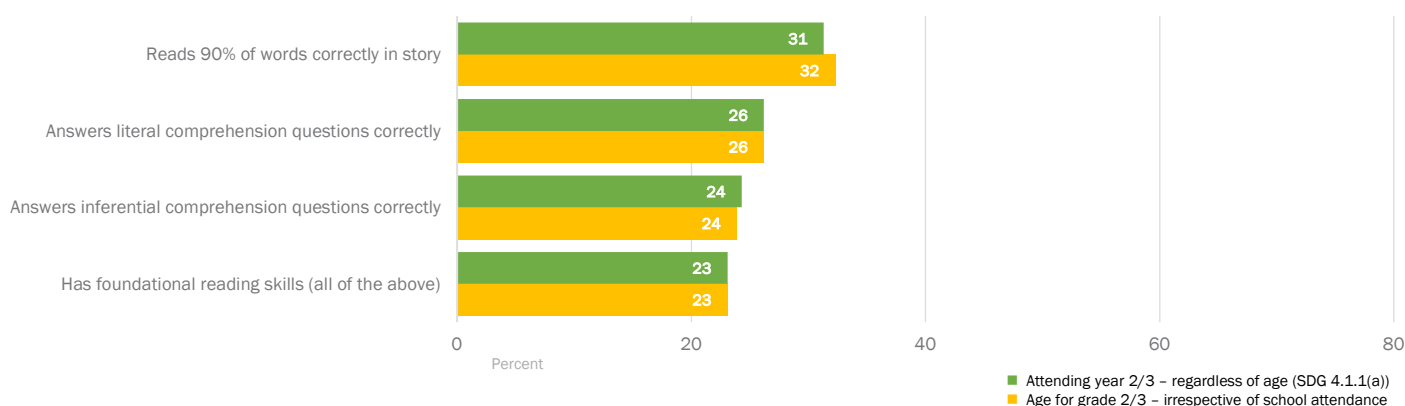
- Gender disparity in senior secondary school attendance strongly in favour of girls (GPI of 1.50).
- Overall, the disparities in gender, wealth and whether living in an urban or rural area increase as children go through the education system. At senior secondary level there is secondary level, a massive disparity in attendance of children of households in the lowest quintile to those in the highest.

Early Grade Learning & Parental Involvement



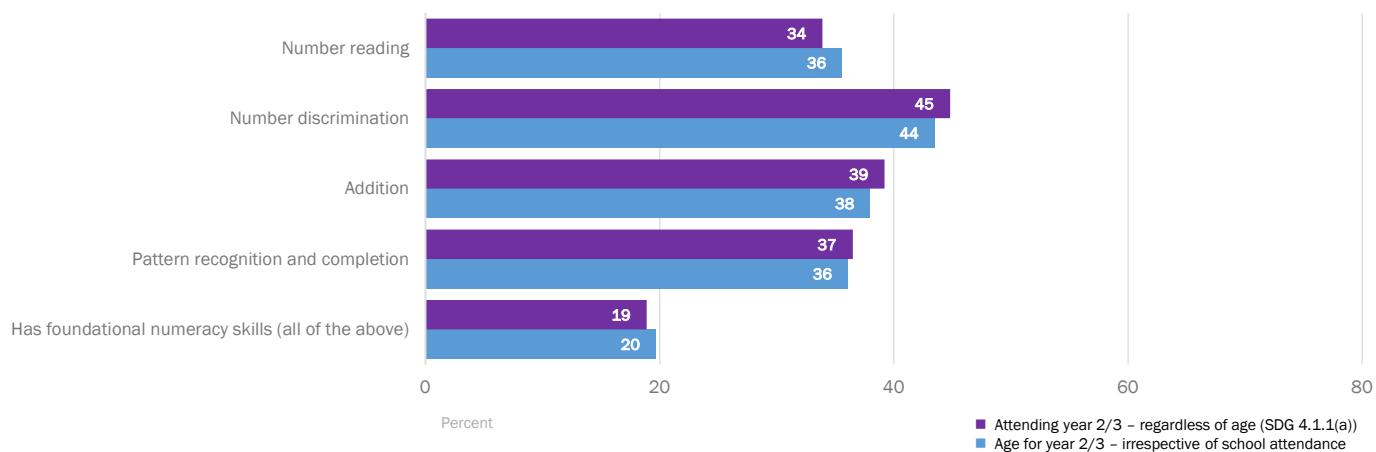
Early Grade Learning: SDG 4.1.1(a)

Foundational Reading Skills: SDG 4.1.1(a) (i: reading)



Percentage of children attending year 2/3 and at age for year 2/3 who can 1) read at least 90% of words in a story correctly, 2) answer three literal comprehension questions, 3) answer two inferential comprehension questions.

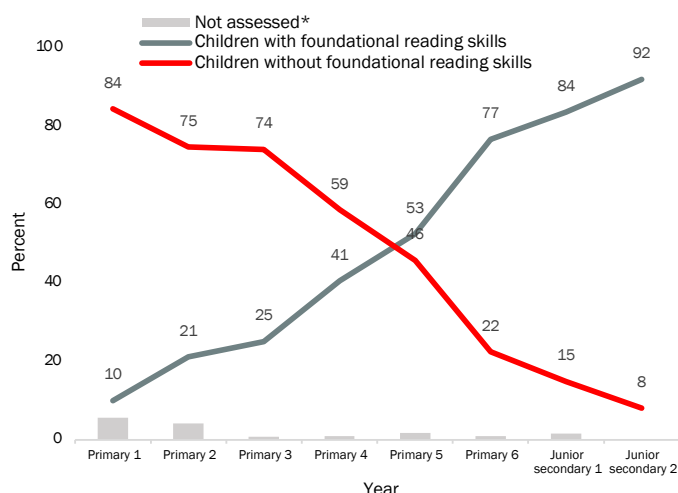
Foundational Numeracy Skills: SDG 4.1.1(a) (ii: numeracy)



Percentage of children attending grade 2/3 and at age for grade 2/3 who can successfully perform 1) a number reading task, 2) a number discrimination task, 3) an addition task and 4) a pattern recognition and completion task.

Early Grade Learning: Disaggregates (age 7-14 years)

Foundational Reading Skills, by grade of attendance



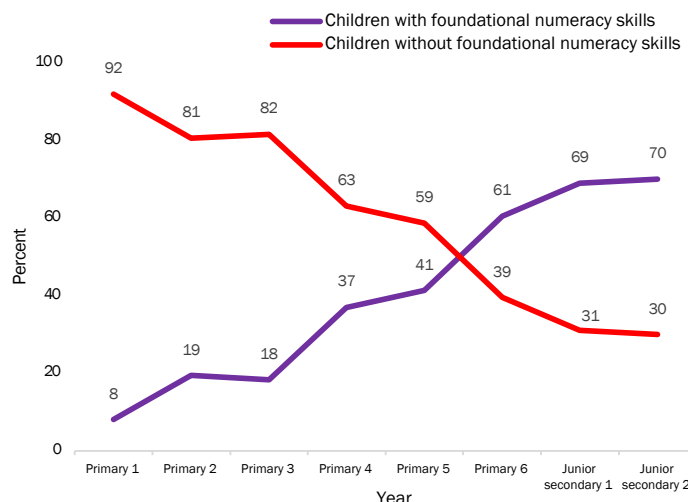
Percentage of children age 7-14 years attending primary or junior secondary school by foundational reading skills, by grade of attendance.

Note that the chart excludes children out of school or attending lower or higher levels of education.

The percentage of children without foundational reading skills is calculated by subtracting the children with foundational reading skills and children for whom the reading tasks were not available in the main language used by teachers and in the main language used at home from the total number of children.

* The reading tasks were available in English, Bislama and French. Children were assessed in the main language used by teachers. If the reading tasks were not available in that language, children were offered the reading tasks in any of the other available languages. Children for whom the reading tasks were not available in the main language used by teachers and in the main language used at home are recorded here.

Foundational Numeracy Skills, by grade of attendance



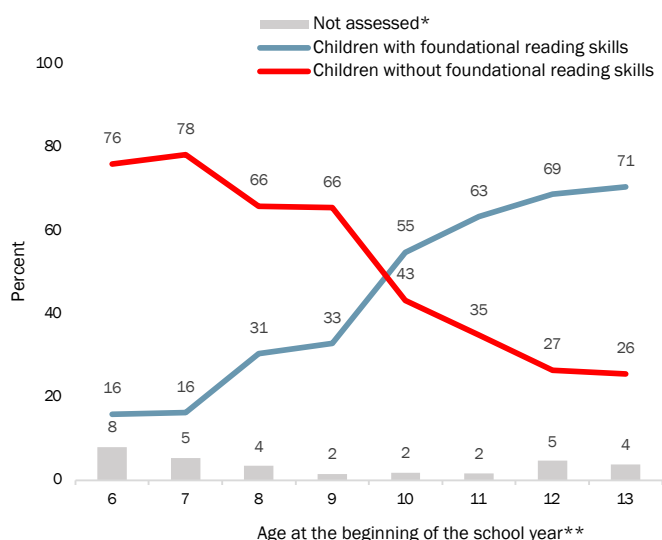
Percentage of children age 7-14 years attending primary or junior secondary school by foundational numeracy skills, by grade of attendance.

Note that the chart excludes children out of school or attending lower or higher level of education.

The percentage of children without foundational numeracy skills is calculated by subtracting the children with foundational reading skills from the total number of children.



Foundational Reading Skills, by age



Percentage of children age 7-14 years by foundational reading skills, by age at beginning of school year.**

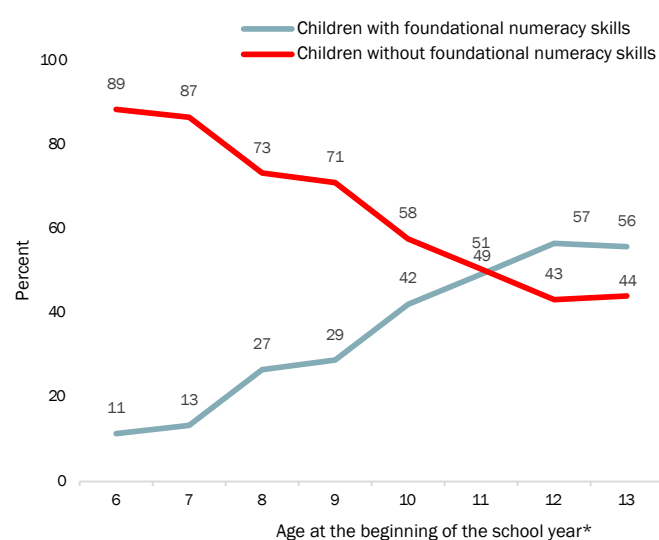
The percentage of children without the foundational reading skills is calculated by subtracting the children with foundational reading skills and children for whom reading tasks were not available in the main language used by teachers and in the main language used at home from the total number of children.

* The reading tasks were available in English, French and Bislama. Children were assessed in the main language used by teachers or, for those who never attended school, in the main language used at home. If the reading tasks were not available in those languages, children were offered the reading tasks in any of the other available languages. Children for whom the reading tasks were not available in the main language used by teachers and in the main language used at home are recorded here.

** As eligibility for the Parental Involvement and Foundational Learning Skills modules was determined based on age at time of interview (age 7-14 years), age at beginning of school year inevitably presents children who were age 6 years at the beginning of the school year.

Note: Not enough cases to show results for age 14.

Foundational Numeracy Skills, by age



Percentage of children age 7-14 years by foundational numeracy skills, by age at beginning of school year.*

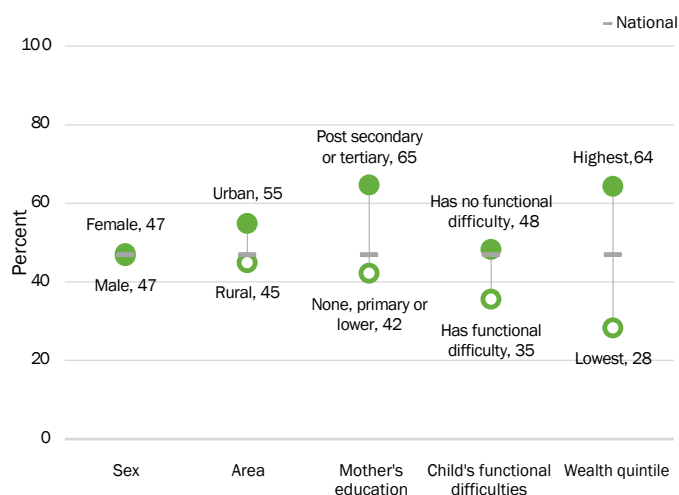
The percentage of children without foundational numeracy skills is calculated by subtracting children with foundational reading skills from the total number of children.

As eligibility for the Parental Involvement and Foundational Learning Skills modules was determined based on age at time of interview (age 7-14 years), age at beginning of school year inevitably presents children who were age 6 years at the beginning of the school year.

Note: not enough cases to show results for age 14.

Early Grade Learning: Disaggregates (age 7-14 years)

Disaggregates in Foundational Reading Skills



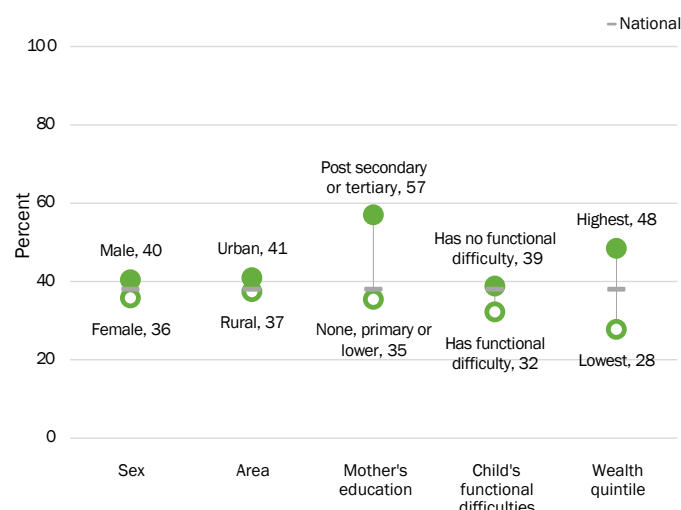
Percentage of children age 7-14 years who demonstrate foundational reading skills by successfully completing three foundational reading tasks, by background characteristics.

() Figures that are based on 25-49 unweighted cases.

Provincial Data on Foundational Reading Skills

Province	Boys	Girls	Total
National	46.6	47.0	46.8
Torba	50.7	(69.7)	59.0
Sanma	42.9	43.9	43.4
Penama	33.0	37.4	35.5
Malampa	52.1	47.5	49.9
Shefa	54.8	55.3	55.0
Tafea	40.7	42.8	41.8

Disaggregates in Foundational Numeracy Skills



Provincial Data on Foundational Numeracy Skills

Province	Boys	Girls	Total
National	46.6	47.0	38.0
Torba	41.1	(39.8)	40.6
Sanma	34.4	38.1	36.2
Penama	25.6	40.6	34.1
Malampa	35.6	35.9	35.7
Shefa	40.2	40.7	40.4
Tafea	35.9	45.8	41.1

Percentage of children age 7-14 years who demonstrate foundational numeracy skills by successfully completing four foundational numeracy tasks, by background characteristics.

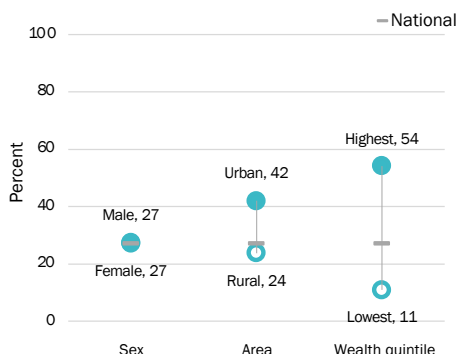
() Figures based on 25-49 unweighted cases.

Measuring Reading & Numeracy Skills in MICS

- The Foundational Learning (FL) skills module is a direct assessment of children's reading and numeracy competencies. It is designed to assess foundational learning skills expected upon completion of second grade of primary education, thus contributing to SDG indicator 4.1.1(a).
- The FL module is part of the Questionnaire for Children age 5-17 administered to one randomly selected child in each household. Children age 7-14 years are eligible for the module.
- The reading assessment in the FL module consists of a reading passage and a set of comprehension questions related to the story. The assessment is customised in each country to ensure vocabulary and cultural references are relevant and appropriate. The numeracy assessment consists of four number tasks based on universal math skills expected at 2nd grade level.
- The reading assessment of Vanuatu MICS 2023 was conducted in English, Bislama and French. The reading skills of 4 percent of the interviewed children could not be evaluated in their home or school language.
- As MICS also collects data on school attendance and numerous individual and household characteristics, such as location and household socio-economic status, the most marginalized sub-populations of children can be identified for support to improve learning outcomes.

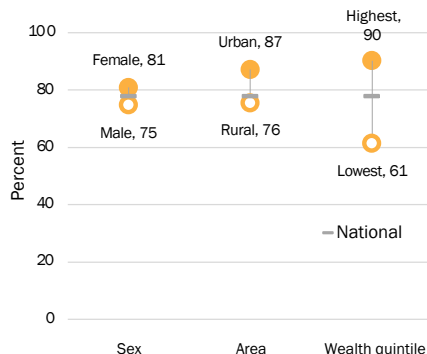
Parental Involvement: Learning Environment at Home

Children with 3 or more books to read at home



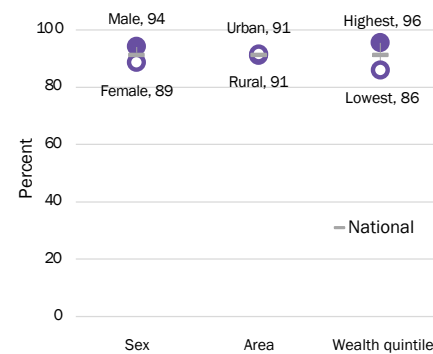
Percentage of children age 7-14 years with 3 or more books at home, by background characteristics.

Children who read books or are read to at home



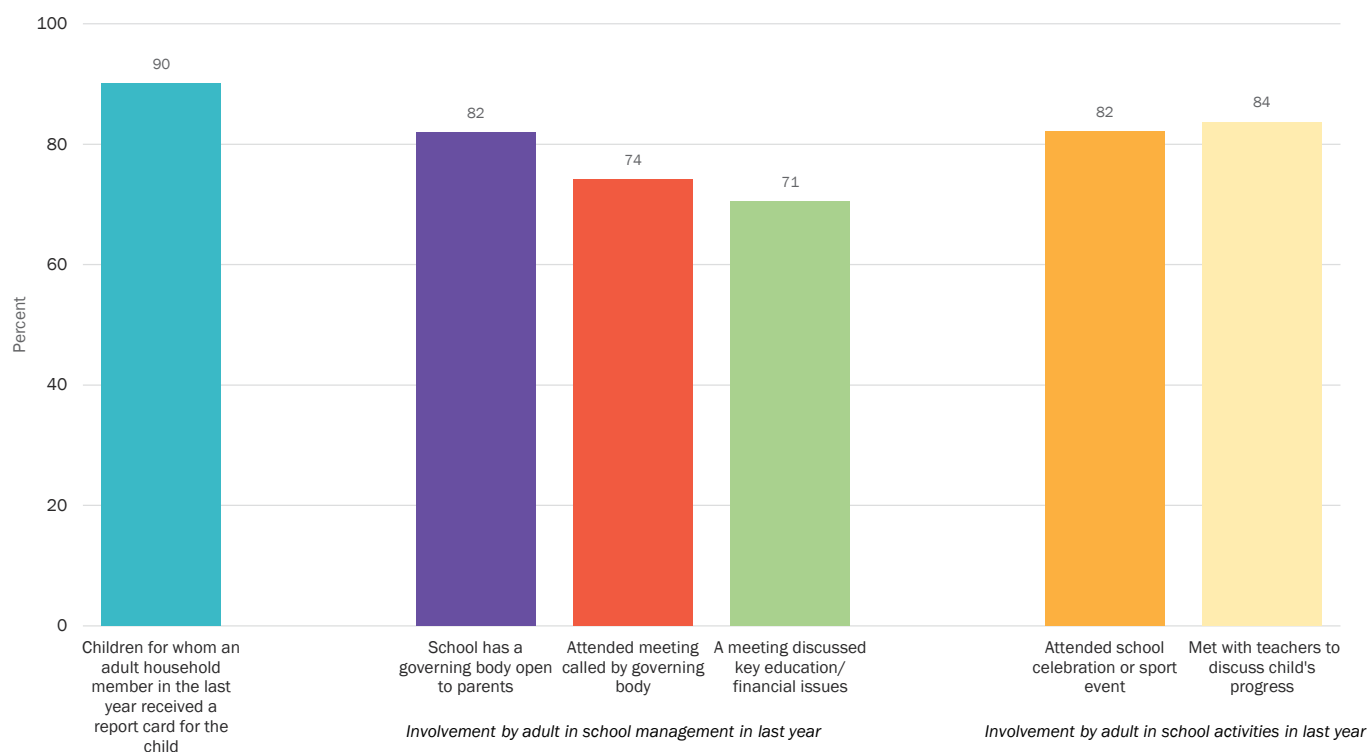
Percentage of children age 7-14 years who read books or are read to at home, by background characteristics.

Children who receive help with homework



Percentage of children age 7-14 years attending school and having homework who receive help with homework, by background characteristics.

Parental Involvement in school



Percentage of children age 7-14 years attending school, by indicators of parental support.

Key Messages

- Only 23 percent of children attending year 2 or 3 were able to read and understand a year 2 level reading test.
- Less than a fifth (19 percent) of children attending year 2 or 3 had foundational numeracy skills.
- The proportion of children demonstrating foundational reading and numeracy skills increases by age. In terms of reading, the biggest improvement appears to come during age 9 to 10 when the level rises from 33 percent to 55 percent.
- The level of wealth of a household is correlated with the attainment of foundational learning skills for children age 7-14 years. For example, the level of foundational numeracy skills from households in the lowest quintile is 34 percent compared to 55 percent in the highest quintile.
- Overall, 77 percent of children age 7-14 years read books at home or are read to. This is higher (90 percent) for children living in the wealthiest households compared to 61 percent in households in the lowest wealth quintile. Only one in ten (11 percent) of households in the lowest quintile have three or more books to read in the house.
- More than 80 percent of parents participate in the school activities of their children, such as receiving report cards and meeting with teachers to discuss progress, attending governance meetings, and school sporting and cultural events.

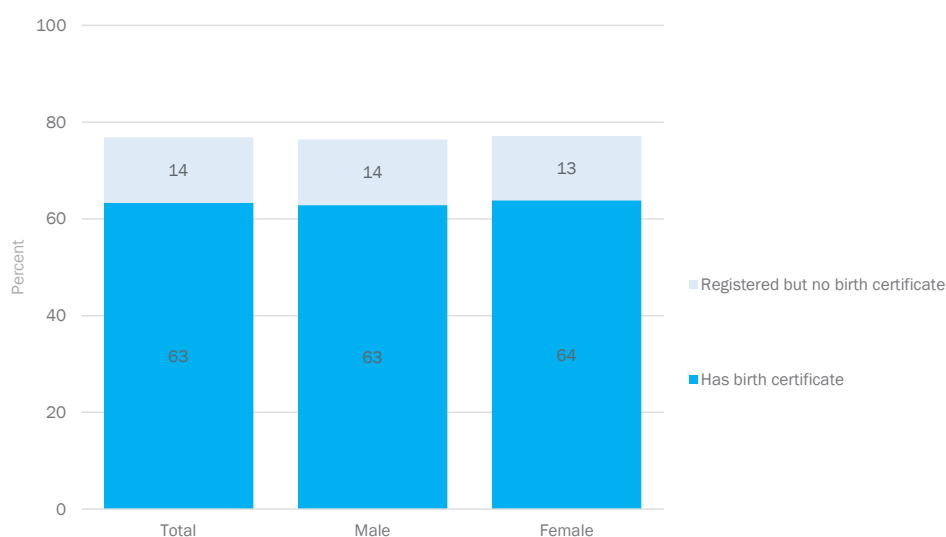


Birth Registration



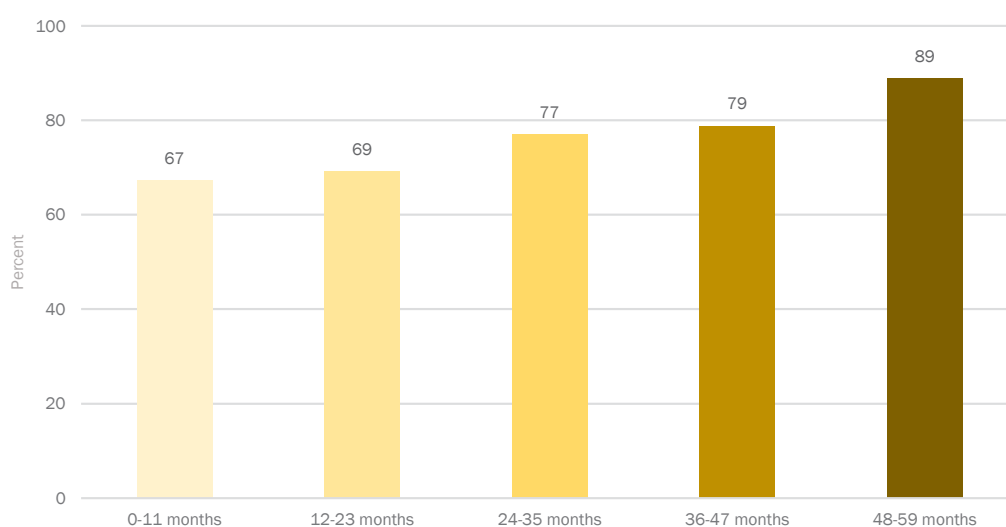
Birth Registration Levels

Birth registration for Children Under-Five: SDG 16.9.1



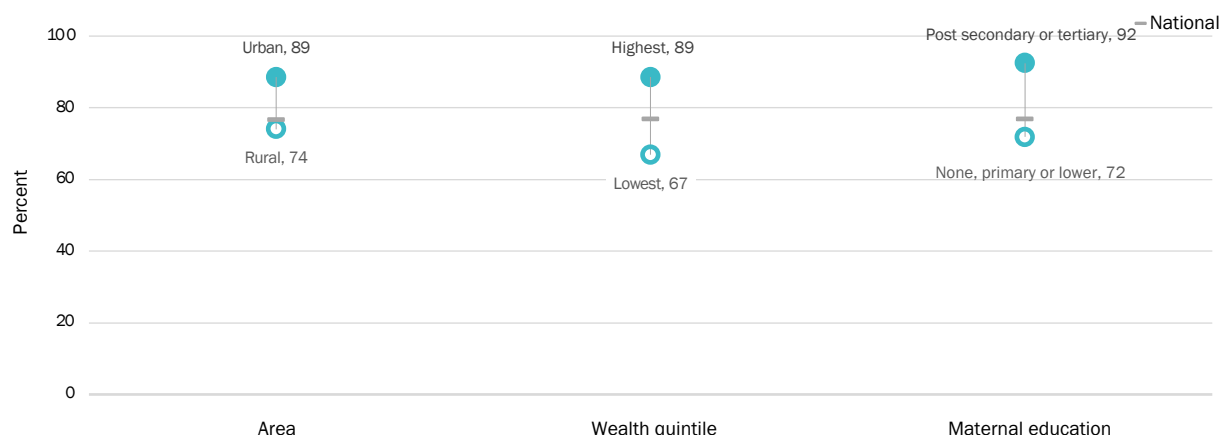
Percentage of children under age 5 whose births are registered, by whether they have a birth certificate and by sex.

Birth Registration by Age



Percentage of children under age 5 whose births are registered, by age in months.

Birth Registration: Inequalities



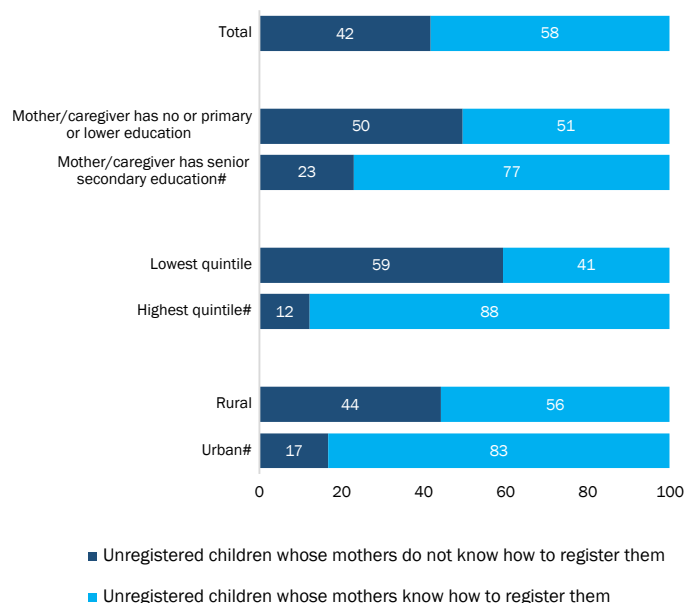
Percentage of children under age 5 whose births are registered, by background characteristics.

Provincial Data on Birth Registration

Province	Total registered
National	76.7
Torba	83.8
Sanma	76.7
Penama	70.5
Malampa	62.2
Shefa	84.7
Tafea	76.0

Percentage of children under age 5 whose births are registered, by province.

Mother's (or Caregiver's) Knowledge of How to Register



Percentage of children under age 5 whose births are not registered, by mother's (or caregiver's) knowledge of how to register a child.

Key Messages

- The level of birth registration in Vanuatu is at 77 percent. Of the registered births, 14 per cent do not have a birth certificate.
- Birth registration in the first year stands at 67 percent, and at 4 years of age 89 percent.
- Births of children born in the lowest wealth quintile households and rural areas are less likely to be registered compared to their counterparts.



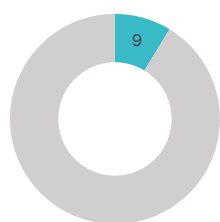
Child Discipline



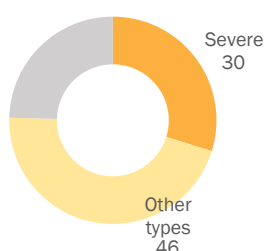
Child Discipline

Types of Child Discipline

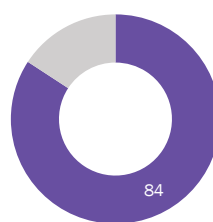
Only non-violent



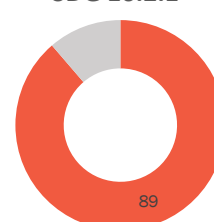
Physical punishment



Psychological aggression

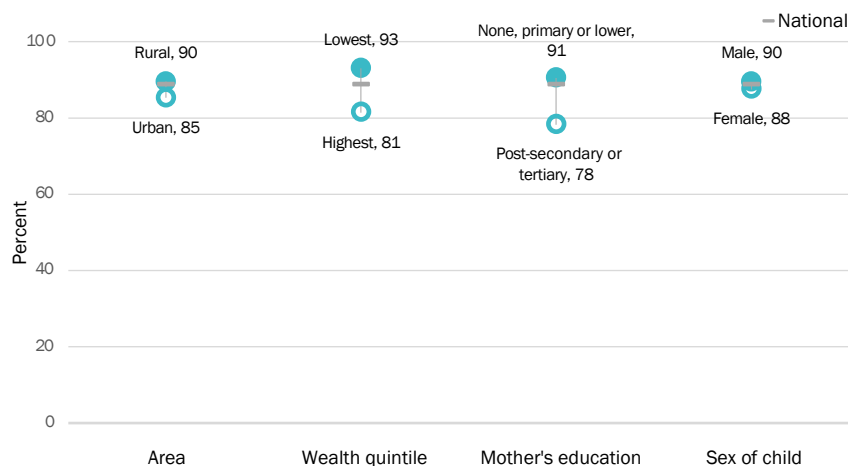


Any violent discipline:
SDG 16.2.1



Percentage of children age 1 to 14 years who experienced any discipline in the past month, by type.

Violent Discipline: Inequalities



Percentage of children age 1 to 14 years who experienced any violent discipline in the past month, by background characteristics.

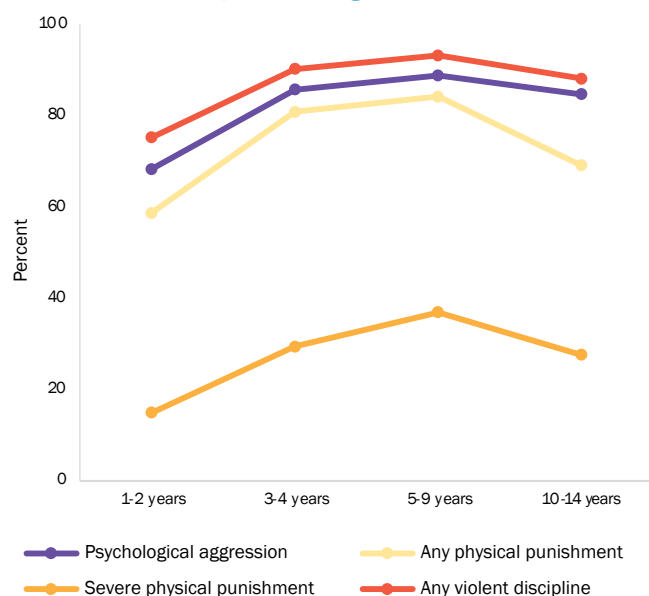
Physical punishment: Shaking, hitting or slapping a child on the hand/arm/leg, hitting on the bottom or elsewhere on the body with a hard object, spanking or hitting on the bottom with a bare hand, hitting or slapping on the face, head or ears, and hitting or beating hard and repeatedly.

Severe physical punishment: Hitting or slapping a child on the face, head or ears, and hitting or beating a child hard and repeatedly.

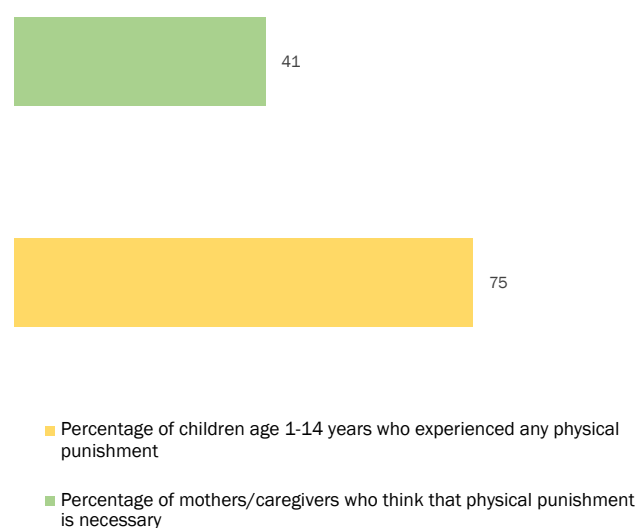
Psychological aggression: Shouting, yelling or screaming at a child, as well as calling a child offensive names such as 'dumb' or 'lazy'.

Violent discipline: Any physical punishment and/or psychological aggression.

Violent Discipline: Age Patterns

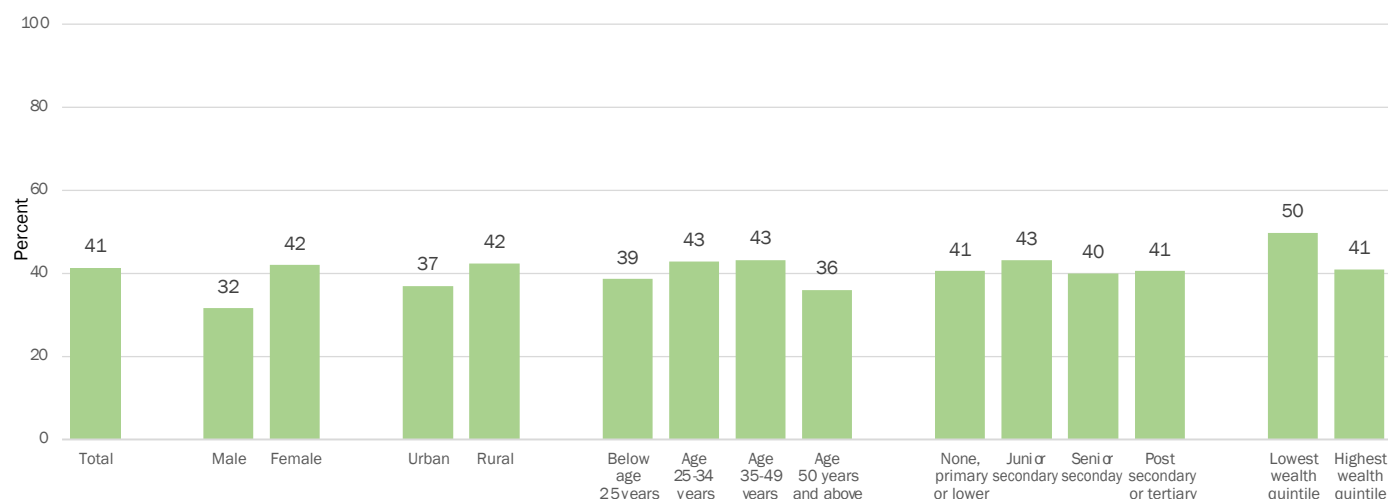


Physical Punishment: Attitudes & Experiences



Percentage of children age 1 to 14 years who experienced any violent discipline in the past month, by type and by age.

Attitudes to Physical Punishment



Percentage of mothers/caregivers who think that physical punishment is necessary to raise or educate children, by their background characteristics.

Key Messages

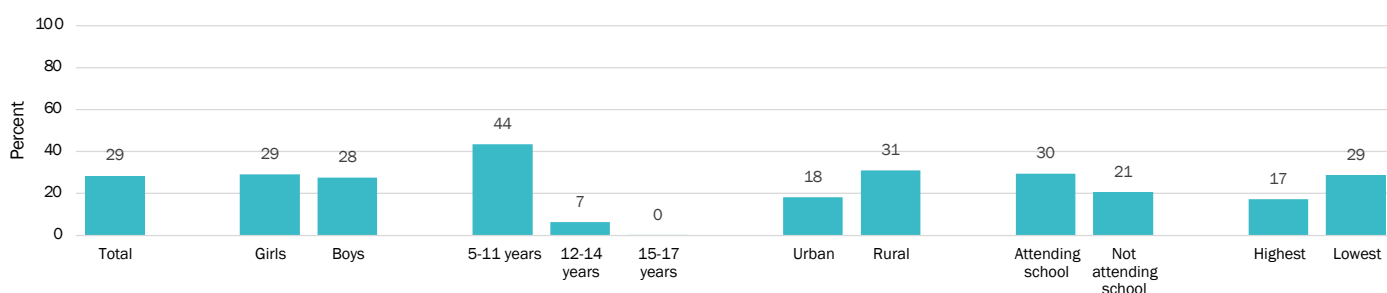
- Violent discipline remains alarmingly high in Vanuatu with 87 percent of children age 1-14 years experiencing violent discipline in the last month.
- Severe physical punishment is also very high, with 28 percent of children experiencing it in the last month.
- Given the almost universal existence of violent discipline, background characteristics such as location, wealth or gender do not have a big impact on the prevalence of violent discipline.
- About 60 percent of caregivers do not believe that physical punishment is necessary to raise or educate a child. Among male caregivers the level is higher, with 68 percent not believing physical punishment is needed, compared to 58 percent among females.

Child Labour



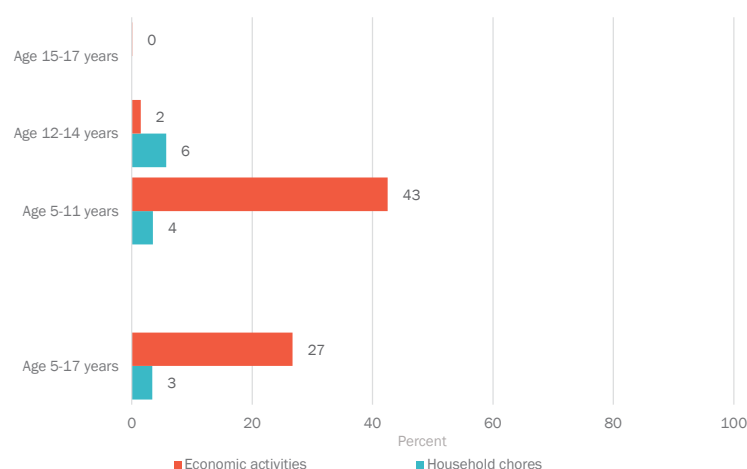
Child Labour: Levels & Disaggregates

Child Labour for Age 5-17 years: SDG 8.7.1



Percentage of children age 5 to 17 years engaged in child labour, by background characteristics.

Types of Child Labour



Percentage of children age 5-17 years engaged in child labour, by type of activity and by age.

Note: These data reflect the proportions of children engaged in the activities at or above the age specific thresholds outlined in the definitions box.

Definition of Child Labour

Age 5 to 11 years: At least 1 hour of economic activities or 21 hours of unpaid household services per week.

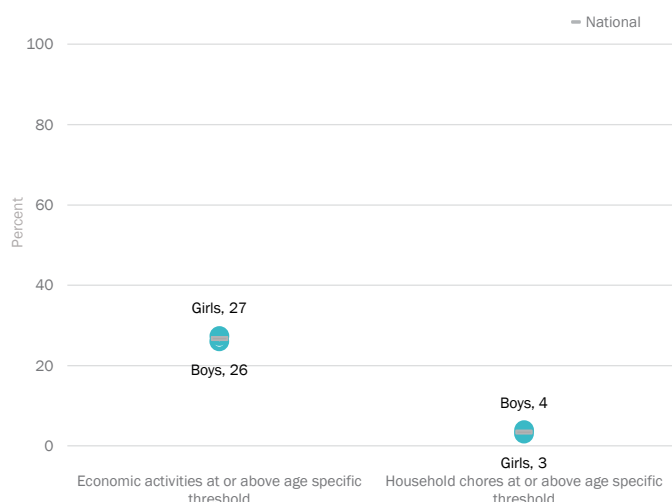
Age 12 to 14 years: At least 14 hours of economic activities or 21 hours of unpaid household services per week.

Age 15 to 17 years: At least 43 hours of economic activities. No threshold for number of hours of unpaid household services.

Economic activities include paid or unpaid work for someone who is not a member of the household, work for a family farm or business. Household chores include activities such as cooking, cleaning or caring for children.

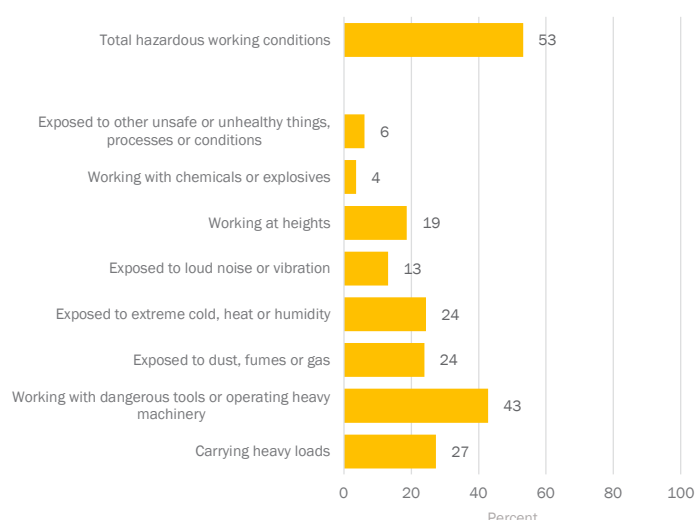
Note that the child labour indicator definition has changed during the implementation of the sixth round of MICS. Changes include age-specific thresholds for household chores and exclusion of hazardous working conditions. While the overall concept of child labour includes hazardous working conditions, the definition of child labour used for SDG reporting does not.

Inequalities in Child Labour



Percentage of children age 5-17 years engaged in child labour, by type of activity and by sex.

Hazardous Working Conditions



Percentage of children age 5-17 years working under hazardous conditions, by background characteristics.

Provincial Data on Child Labour

Province	Total Child Labour
National	28.5
Torba	36.1
Sanma	22.7
Penama	34.6
Malampa	29.9
Shefa	28.9
Tafea	26.6

Percentage of children age 5-17 years engaged in child labour, by province.



Key Messages

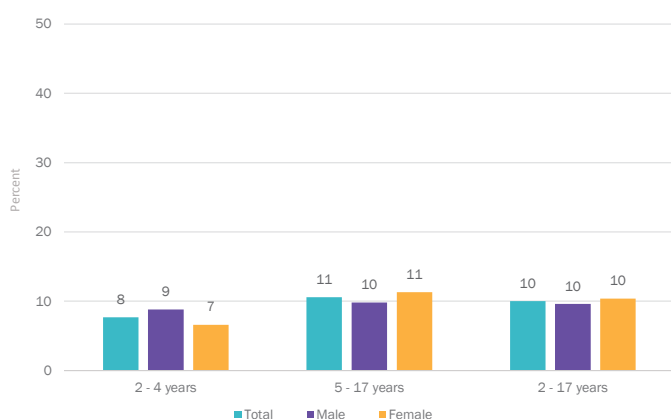
- About a third (29 percent) of all children age 5-17 years are engaged in child labour in Vanuatu and the gender differences are very small.
- Most children engaged in child labour are children age 5-11 years old, and they are primarily engaged in economic activities.
- The level of child labour is higher in rural areas, among children attending school and among the poorest households.
- Child labour among children age 15-17 years is virtually non-existent (0.1 percent), and among children age 12-14 years the prevalence is 7 percent.
- Of the children engaged in child labour, half of them (53 percent) are engaged in hazardous work, particularly working with dangerous tools or operating heavy machinery (43 percent).
- The prevalence of child labour is highest in Torba province and lowest in Tafea province.

Child Functioning



Child Functioning: Levels & Domains

Child Functioning Levels by Age-Group



Percentage of children age 2–17 years with functional difficulty, by age-group.

Children with disabilities are among the most marginalized groups in society. Facing daily discrimination in the form of negative attitudes, and lack of adequate policies and legislation, they are often likely to be among the poorest members of the population and are less likely to attend school, access medical services, or have their voices heard in society. Discrimination against and exclusion of children with disabilities also puts them at a higher risk of physical and emotional abuse or other forms of neglect, violence and exploitation.

The Convention on the Rights of the Child (UNICEF, 1989) and the Convention on the Rights of Persons with Disabilities (UN, 2006) explicitly state the rights of children with

disabilities on an equal basis with other children and call for improvements in their access to services, and in their participation in all aspects of life.

In order to achieve these goals, there is a need for cross-nationally comparable, reliable data. The Child Functioning module is designed in line with the WHO's International Classification of Functioning, Disability and Health and the UN Convention on the Rights of Persons with Disabilities, to collect information on functional difficulties that children experience in different domains including hearing, vision, communication/ comprehension, learning, mobility and emotions. Children with functional difficulties may be at risk of experiencing limited participation in an unaccommodating environment and limit the fulfilment of their rights.

Child Functioning Domains

	Seeing	Hearing	Walking	Fine Motor	Communication	Learning	Playing	Remembering	Concentrating	Accepting change	Controlling behaviour	Making friends	Anxiety	Depression
2-4 years	<1	<1	<1	<1	3	6	<1	N/A	N/A	N/A	N/A	N/A	N/A	N/A
5-17 years	<1	1	2	N/A	<1	2	N/A	<1	<1	1	2	<1	3	3

Percentage of children age 2–4 and 5-17 years with functional difficulty in at least one domain, by domain of difficulty.

N/A- Not Applicable.

Child Functioning: Inequalities



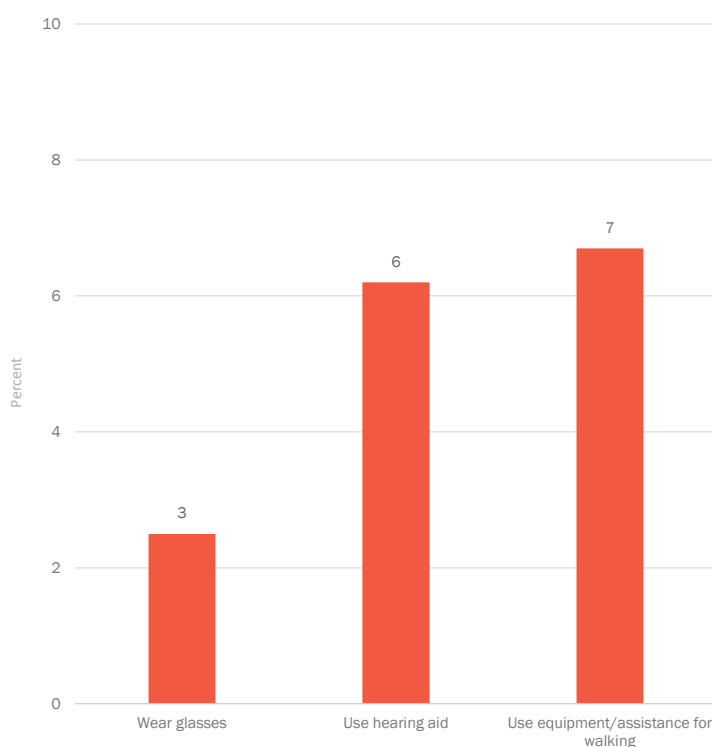
Percentage of children age 2–17 years with functional difficulty, by background characteristics.

Provincial Data on Child Functioning

Province	2-4 years	5-17 years	2-17 years
National	7.7	10.6	10.0
Torba	0.0	8.9	7.4
Sanma	1.9	5.2	4.5
Penama	3.7	10.8	9.3
Malampa	12.4	20.7	19.2
Shefa	5.7	9.1	8.4
Tafea	18.8	11.2	12.9

Percentage of children age 2–17 years with functional difficulty in at least one domain, by province.

Children who use Assistive Devices & have Functional Difficulties



Percentage of children age 2-17 years with difficulties seeing when wearing glasses among those who wear glasses, percentage of children age 2-17 years with difficulties hearing when using a hearing aid among those who use a hearing aid, and percentage of children age 2-17 years with difficulties walking when using equipment or receiving assistance among those who use equipment or receive assistance walking.

Key Messages

- The incidence of child functioning difficulties are 8 percent for younger children and slightly higher (11 percent) for 5-17 year olds. No discernible differences between boys and girls are noted for any of the age groups.
- The highest reporting is for learning difficulties for younger children and anxiety and depression for older children.
- Levels of child disability are highest in the least wealthy households.
- Among children age 2-17 years who are using assistive devices, 7 percent of those using a device to aid mobility still report having difficulties.



Vaccinations in the first years of life

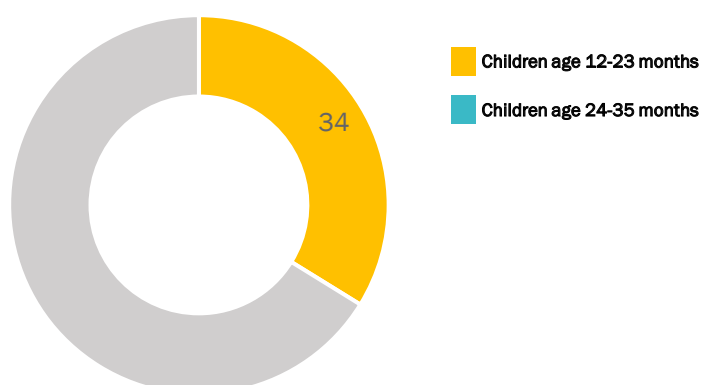


Basic and full immunisation

Immunisation is a proven cost-effective tool for controlling and eliminating life-threatening infectious diseases and is estimated to avert between 3.5 and 5 million deaths each year. The Vanuatu Ministry of Health recommends all infants and young children (especially those under 2 years of age) to be vaccinated against tuberculosis, polio, hepatitis B, diphtheria, tetanus, pertussis, pneumococcal disease, rotavirus, measles and rubella. **Basic immunisation** refers to children age 12-23 months vaccinated against tuberculosis, polio, diphtheria, tetanus, pertussis and measles. **Full immunisation** refers to children age 24-35 months who have received all the vaccines scheduled to be given in the two first years of life, according to the Vanuatu vaccination schedule.

Basic Immunization

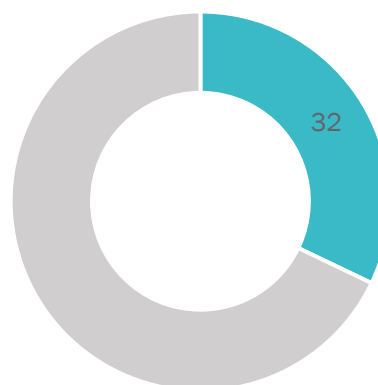
(BCG, OPV3, DTP3, and Measles - Rubella)



Percentage of children age 12-23 months who at any time before the survey have received all basic vaccinations.

Full immunisation

(BCG, OPV3/IPV, HepB, DTP3, PCV3, Rota2 and Measles - Rubella)



Percentage of children age 24-35 months who at any time before the survey have received all vaccines scheduled in the first two years of life, according to the national vaccination schedule.

Vaccines recommended by the Ministry of Health for children under age 2 years:

The Bacillus Calmette–Guérin (**BCG**) vaccine protects against some of the deadliest forms of tuberculosis (**TB**), a bacterial infection.

Oral Polio Vaccine (**OPV**) and Inactivated Polio Vaccine (**IPV**) provide protection against polio, or poliomyelitis.

The **HepB** vaccine protects against liver infection caused by the Hepatitis B virus.

The **DTP** vaccine is a combination vaccine against three infectious diseases: diphtheria, tetanus, and pertussis (or whooping cough). A DT booster is an additional administration of the DT vaccine after the primary doses.

The Pneumococcal Conjugate Vaccine (**PCV**) can prevent illnesses caused by pneumococcal bacteria (or *Streptococcus pneumoniae*), one of the leading causes of pneumonia.

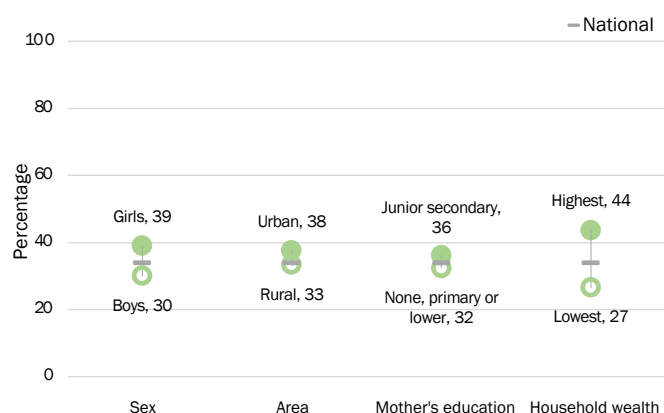
The **Rota** vaccine protects infants and young children against the rotavirus which can cause severe watery diarrhea, vomiting, fever, and abdominal pain.

The **measles and rubella** vaccines are given in a combination vaccine against two infectious viral diseases: measles and rubella.

Inequalities in vaccination coverage among young children

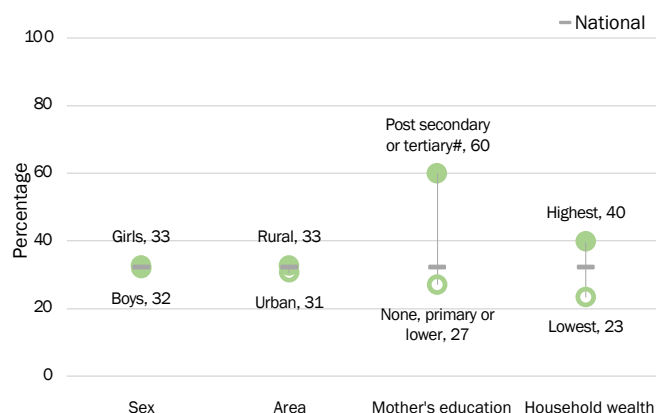
Socio-economic disparities in vaccination coverage

Basic immunisation



Percentage of children age 12-23 months who at any time before the survey had received all basic vaccinations (BCG, Polio3, DTP3, and MR1), by sex, location, mother's education, and household wealth.

Full immunisation



Percentage of children age 24-35 months who at any time before the survey had received all vaccines scheduled in the first two years of life, according to the national vaccination schedule, by sex, location, mother's education, and household wealth.

Figures based on 25-49 unweighted cases.

Provincial disparities in vaccination coverage

Province	Basic	Full	No vaccinations	
	Children 12-23 months	Children 24-35 months	Children 12-23 months	Children 24-35 months
National	33.8	32.2	5.7	4.6
Torba	*	*	*	*
Sanma	20.9	23.4	3.5	0.0
Penama	44.6	40.7	10.5	4.9
Malampa	(42.4)	43.9	(0.0)	2.2
Shefa	45.5	38.4	0.0	0.0
Tafea	19.1	13.5	16.2	18.5

(*) Figures that are based on fewer than 25 unweighted cases.

() Figures that are based on 25-49 unweighted cases.

Percentage of children age 12-23 months who had received all basic vaccinations, percentage of children age 24-35 months who had received all vaccines scheduled in the first two years of life, according to the national, vaccination schedule, and percentage of children age 12-23 and 24-35 months who had not received any vaccination doses at all, at any time, before the survey.

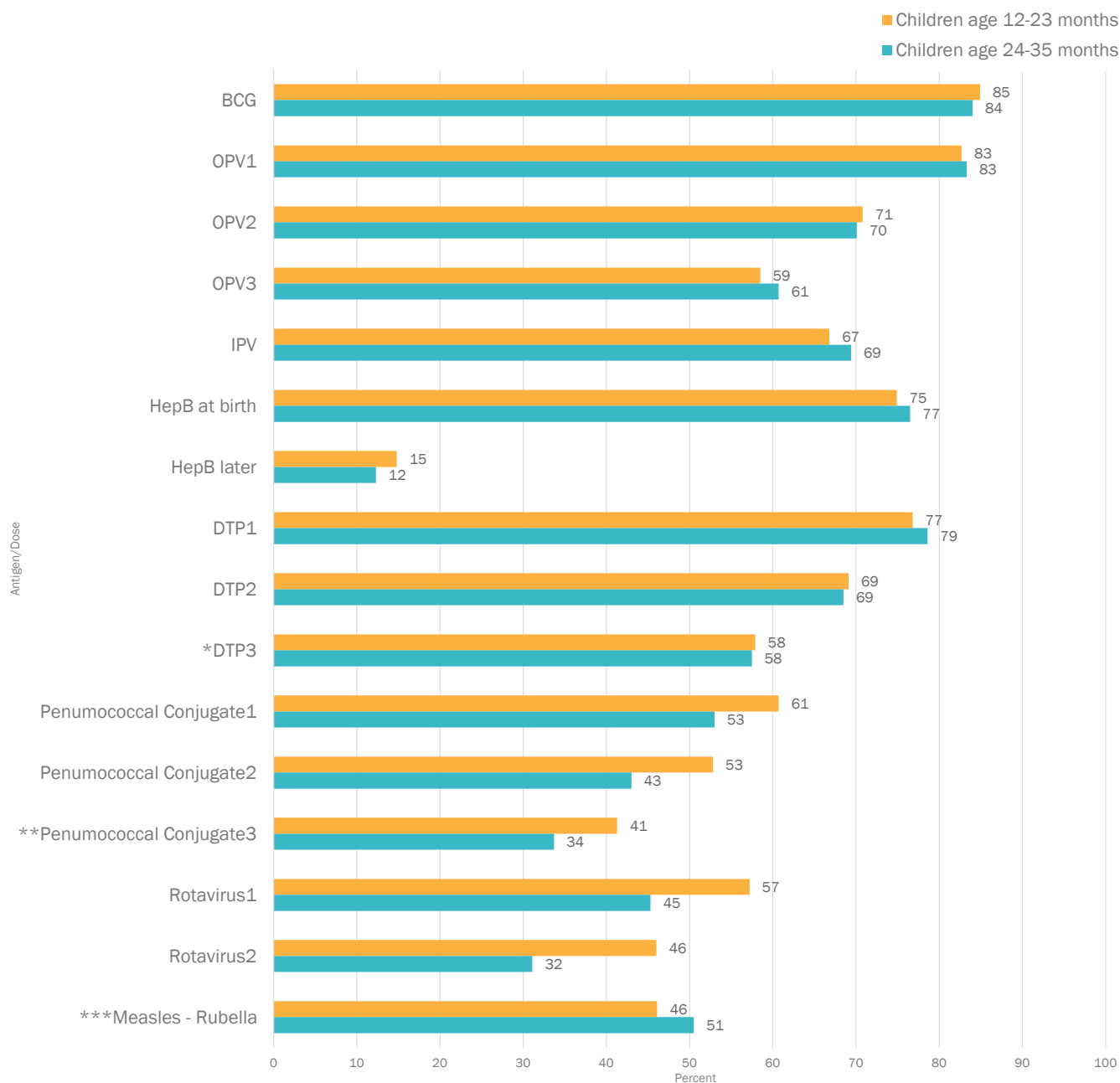
Information on vaccination coverage in MICS is collected for all children under three years of age.

Mothers or caretakers of children under 36 months of age are asked to show vaccination records, that is, cards or similar documents where vaccinations are recorded. If a vaccination record for a child is available, interviewers copy vaccination information from the document (s) onto the MICS questionnaire and asks the respondent about any vaccines not recorded. If no vaccination record is available for the child, the interviewer proceeds to ask the mother to recall whether the child has received each of the vaccinations, and, for applicable antigens, how many doses the child received. Some vaccination records were also obtained from visiting the child's health facility.

The final vaccination coverage estimates are based on information obtained from vaccination records and mothers' report of vaccinations received by the child.

Vaccination coverage by antigen

Vaccination coverage among children age 12-23 months and 24-35 months, by vaccine



Percentage of children age 12-23 months and 24-35 months vaccinated against vaccine preventable childhood diseases at any time before the survey (crude coverage), by specific vaccine.

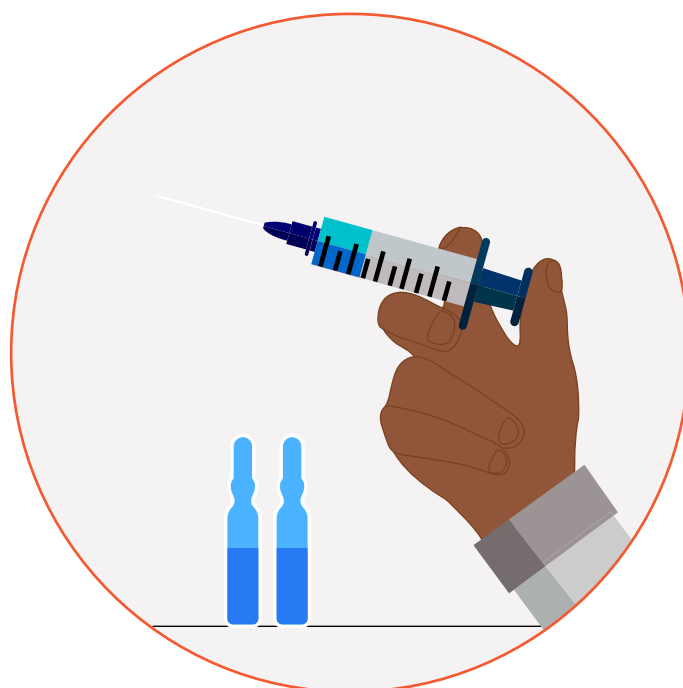
*Diphtheria, Tetanus and Pertussis (DTP) Immunisation coverage; SDG indicator 3.b.1 & 3.8.1.

**Pneumococcal (Conjugate) Immunisation coverage; SDG indicator 3.b.1.

***Measles immunization coverage; SDG indicator 3.b.1.

Key Messages

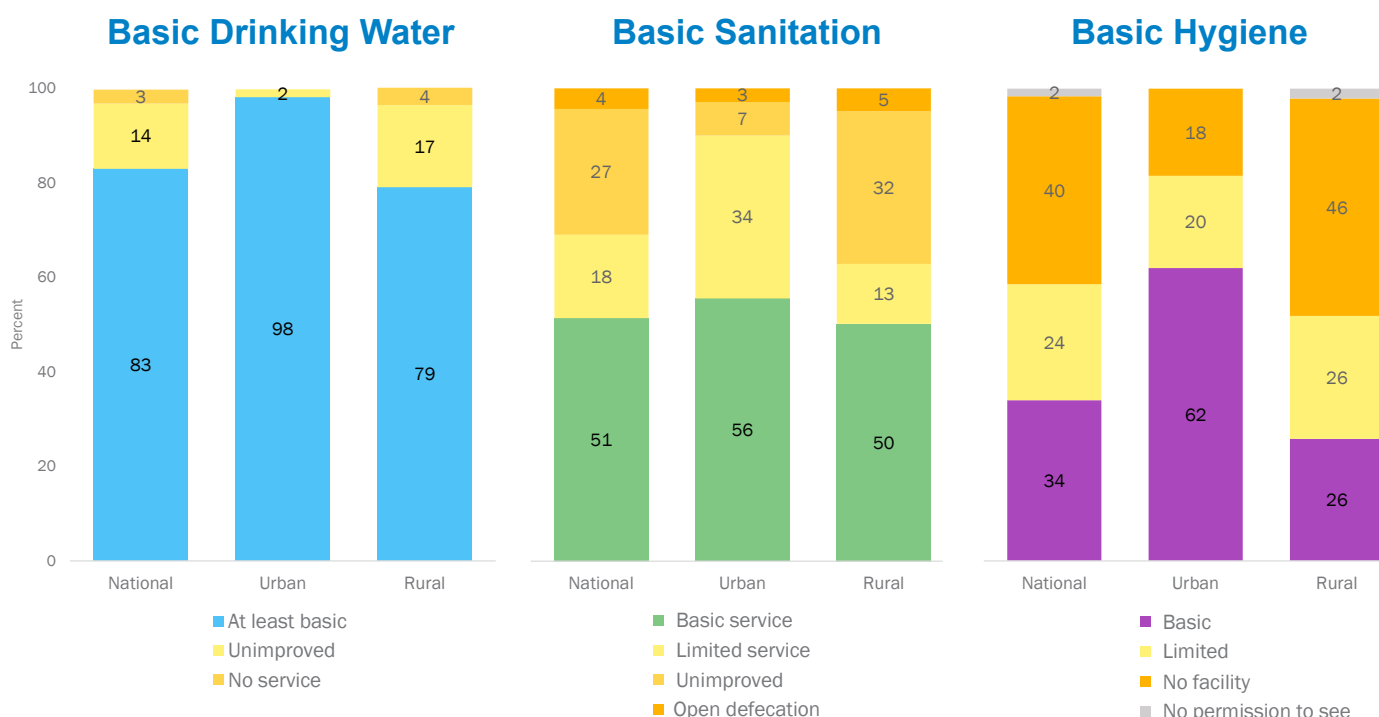
- In Vanuatu, only 34 percent of children age 12-23 months received basic vaccination.
- Only a third (32 percent) of children age 23-35 months received all vaccination as per the vaccination schedule.
- In general, disparities between different groups are small. However, the level of wealth of the household seems to be correlated. Children in households in the highest wealth quintile are almost twice as likely to be vaccinated for basic and full coverage.
- In terms of Provinces, Penama and Shefa have the highest levels of vaccination coverage.
- The BCG vaccine, against the deadliest forms of tuberculosis, has the greatest level of coverage.



Drinking Water, Sanitation & Hygiene (WASH)



Basic Drinking Water, Sanitation & Hygiene Services



Percent of population by drinking water, sanitation and hygiene coverage.

Drinking water ladder: **Safely managed** drinking water services (SDG 6.1.1) refer to the use of an improved drinking water source on-premises, whose source water was tested and free of E. coli and available when needed. **Basic** drinking water services (SDG 1.4.1) refer to an improved source, provided collection time is not more than 30 minutes for a roundtrip including queuing. **Improved** drinking water sources are those that have the potential to deliver safe water by nature of their design and construction, and include: piped water, boreholes or tubewells, protected dug wells, protected springs, rainwater, and packaged or delivered water. **Limited** refers to an improved source more than 30 minutes roundtrip. **Unimproved** sources include unprotected dug wells and unprotected springs. **No service** refers to the direct collection of water from surface waters such as rivers, lakes or irrigation channels.

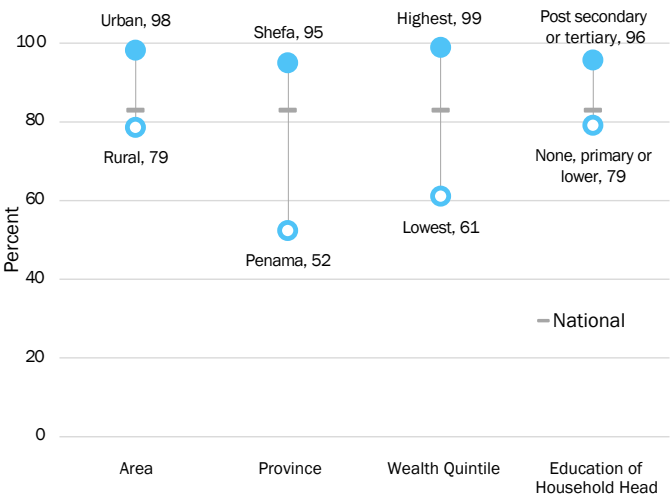
Sanitation ladder: **Safely managed** sanitation services (SDG 6.1.2) refer to the use of improved sanitation facilities that are not shared with other households and where excreta are safely disposed of in situ or transported and treated offsite or pit latrines that are sealed when they become full and new pits dug. **Basic** sanitation services (SDG 1.4.1)

refer to the use of improved facilities that are not shared with other households. Improved sanitation facilities are those designed to hygienically separate excreta from human contact and include: flush/pour flush to piped sewer system, septic tanks or pit latrines; ventilated improved pit latrines, composting toilets or pit latrines with slabs. **Limited** sanitation service refers to an improved facility shared with other households. **Unimproved** sanitation facilities include flush/pour flush to an open drain, pit latrines without a slab, hanging latrines, and bucket latrines. **No service** refers to the practice of open defecation.

Hygiene ladder: A **basic** hygiene service (SDG 1.4.1 & SDG 6.2.1) refers to the availability of a handwashing facility on premises with soap and water. Handwashing facilities may be fixed or mobile and include a sink with tap water, buckets with taps, tippy-taps, and jugs or basins designated for handwashing. Soap includes bar soap, liquid soap, powder detergent, and soapy water but does not include ash, soil, sand or other handwashing agents. **Limited** hygiene service refers to a facility lacking water and/or soap. **No facility** means there is no handwashing facility on the household's premises.

WASH: Inequalities in Basic Services

Basic Drinking Water



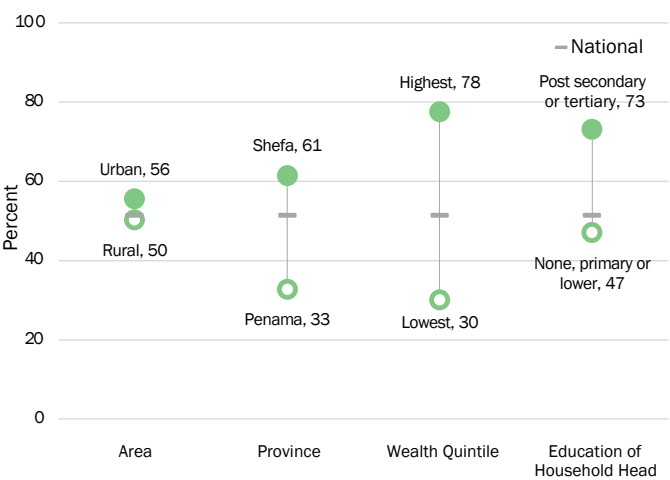
Percent of population using basic drinking water services by background characteristics.

Provincial Data on Basic Services

Province	Basic Drinking Water	Basic Sanitation	Basic Hygiene
National	58.0	51.4	34.0
Torba	55.7	45.1	5.6
Sanma	87.5	54.7	21.8
Penama	52.3	32.6	22.5
Malampa	89.7	56.3	19.2
Shefa	94.9	61.3	60.4
Tafea	74.4	37.3	15.9

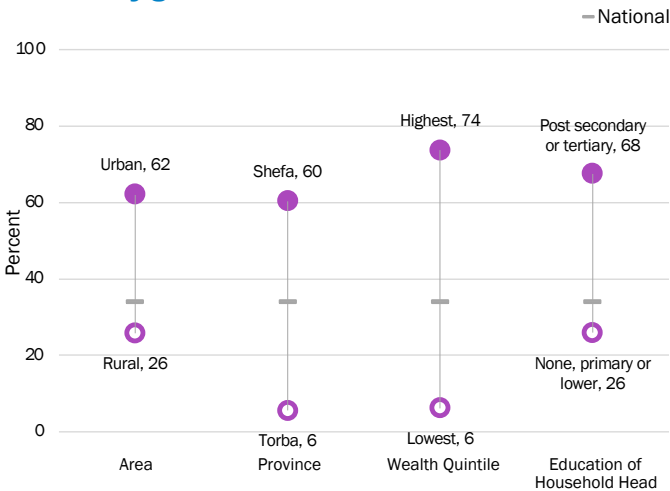
Percent of population using basic drinking water, sanitation and hygiene services by province.

Basic Sanitation



Percent of population using basic sanitation services by background characteristics.

Basic Hygiene

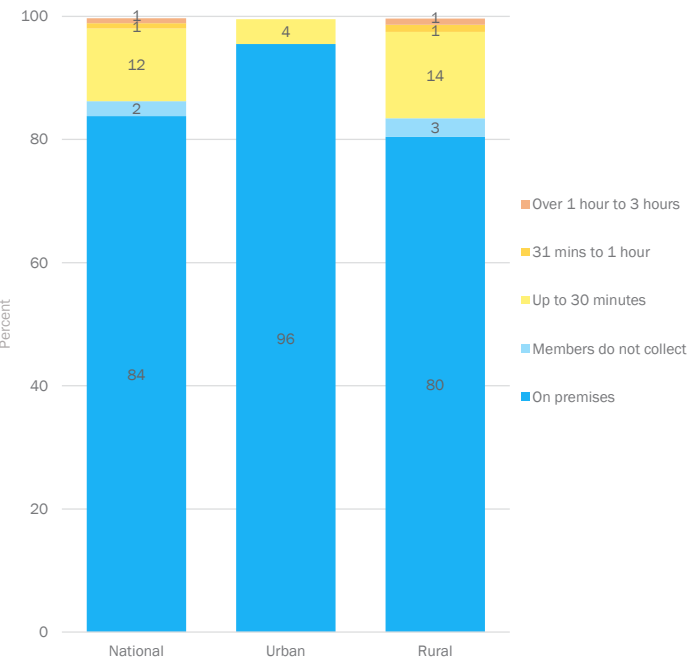


Percent of population using basic hygiene services by background characteristics.



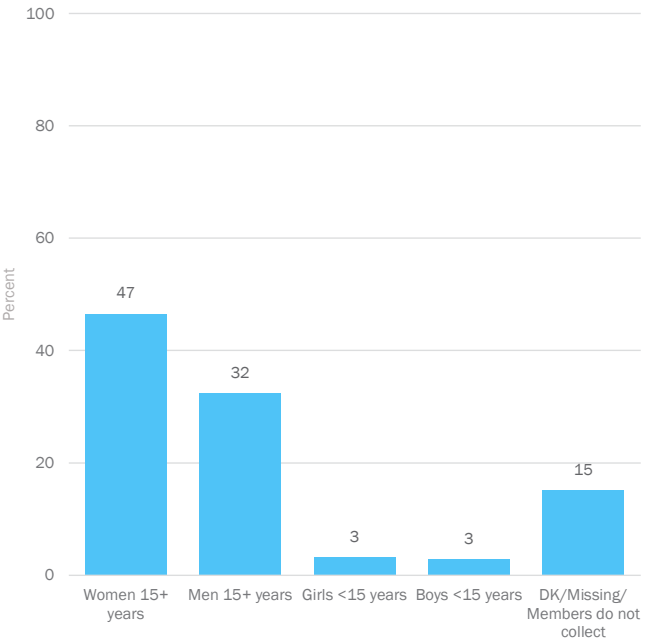
Accessibility of Drinking Water & Sanitation Facilities

Accessibility of drinking water



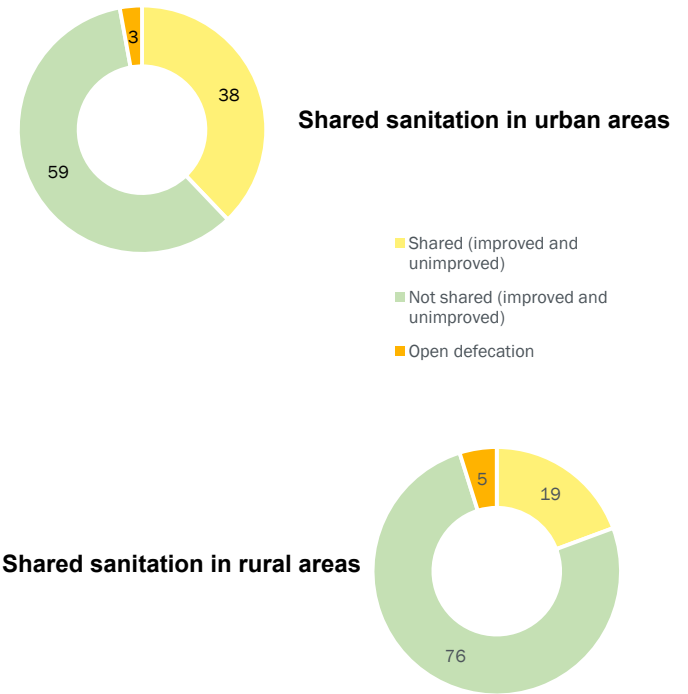
Percent of population by average time spent per day by household members collecting drinking water.

Who Primarily Collects Drinking Water for the Household



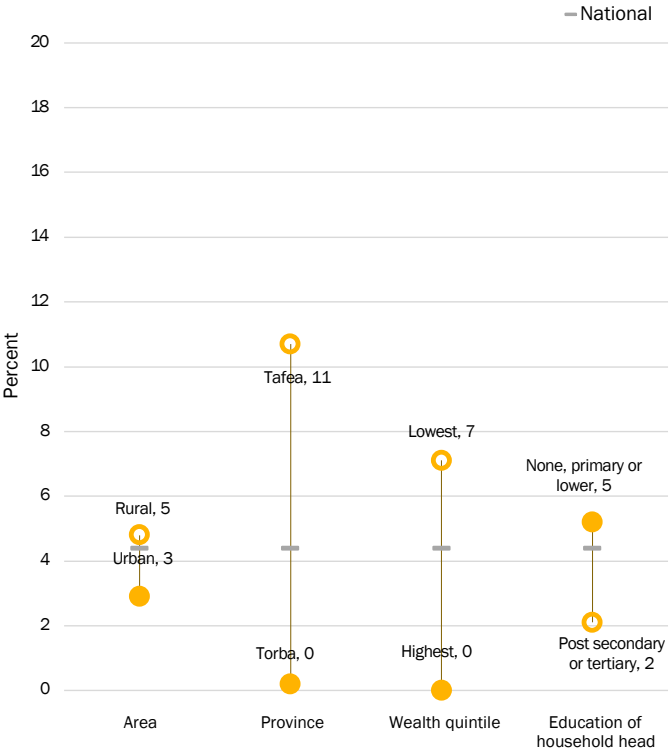
Percent of population in households without drinking water on premises, by gender and age of person primarily responsible for collecting drinking water.

Shared sanitation



Percent of the population sharing sanitation facilities, by residence.

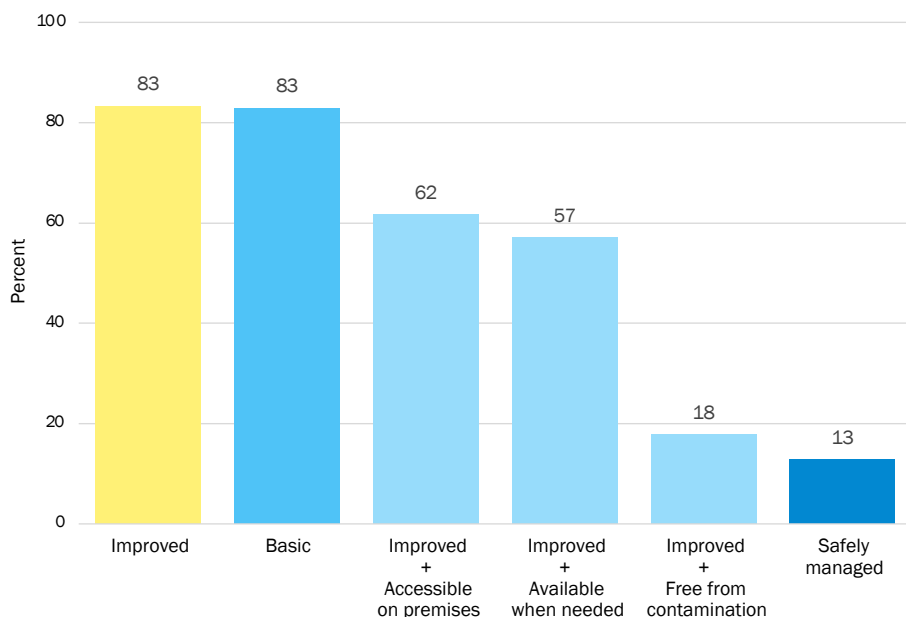
Open Defecation



Percent of the population practising open defecation, by background characteristics.

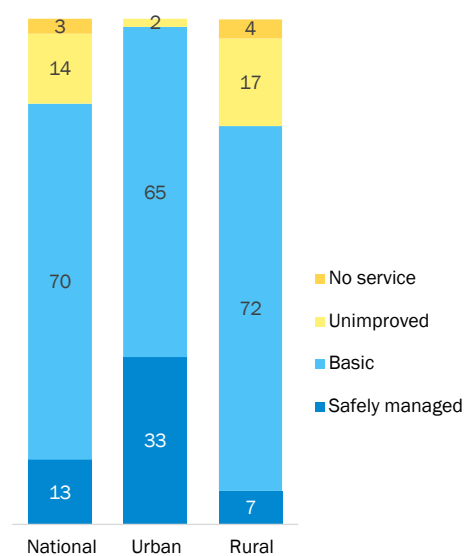
Safely Managed Drinking Water Services: SDG.6.1.1

Improved, basic & safely managed drinking water



Percent of population using improved, basic and safely managed drinking water services.

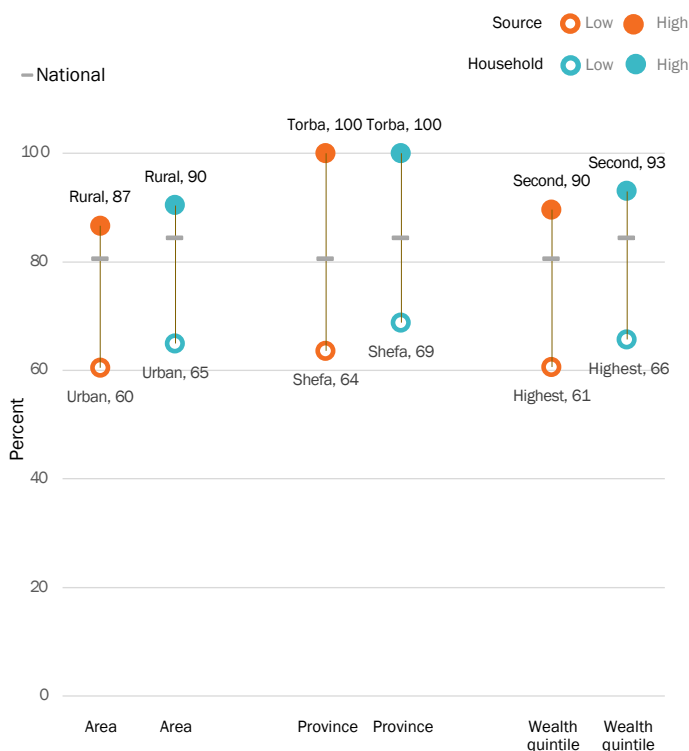
Drinking water coverage: National, urban & rural



Percent of population by drinking water coverage.

Safely managed (SDG 6.1) are improved sources: accessible on premises, available when needed, free from contamination.

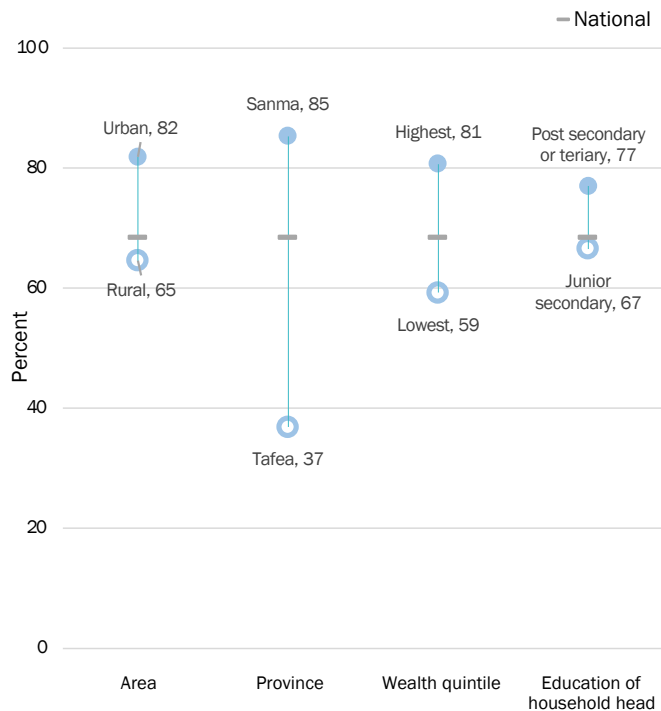
Drinking Water Quality at Source & Home



Percent of population using drinking water sources with E. coli (orange) and proportion with E. coli in glass of drinking water in household drinking water (teal).

Water Quality Testing response rates for Household and Source testing are 95.9% and 93.4% respectively.

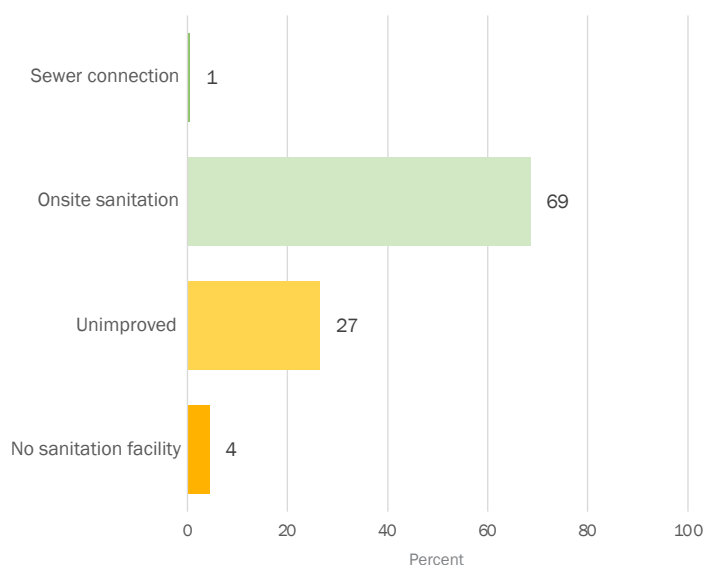
Availability of Drinking Water



Percent of population using drinking water sources with sufficient drinking water in the last month.

Safely Managed Sanitation Services: SDG 6.2.1

Types of Sanitation Facility



Percent of population by type of sanitation facility, grouped by type of disposal.

Sewer connections include “Flush/pour flush to piped sewer system” and “Flush to DK where”.

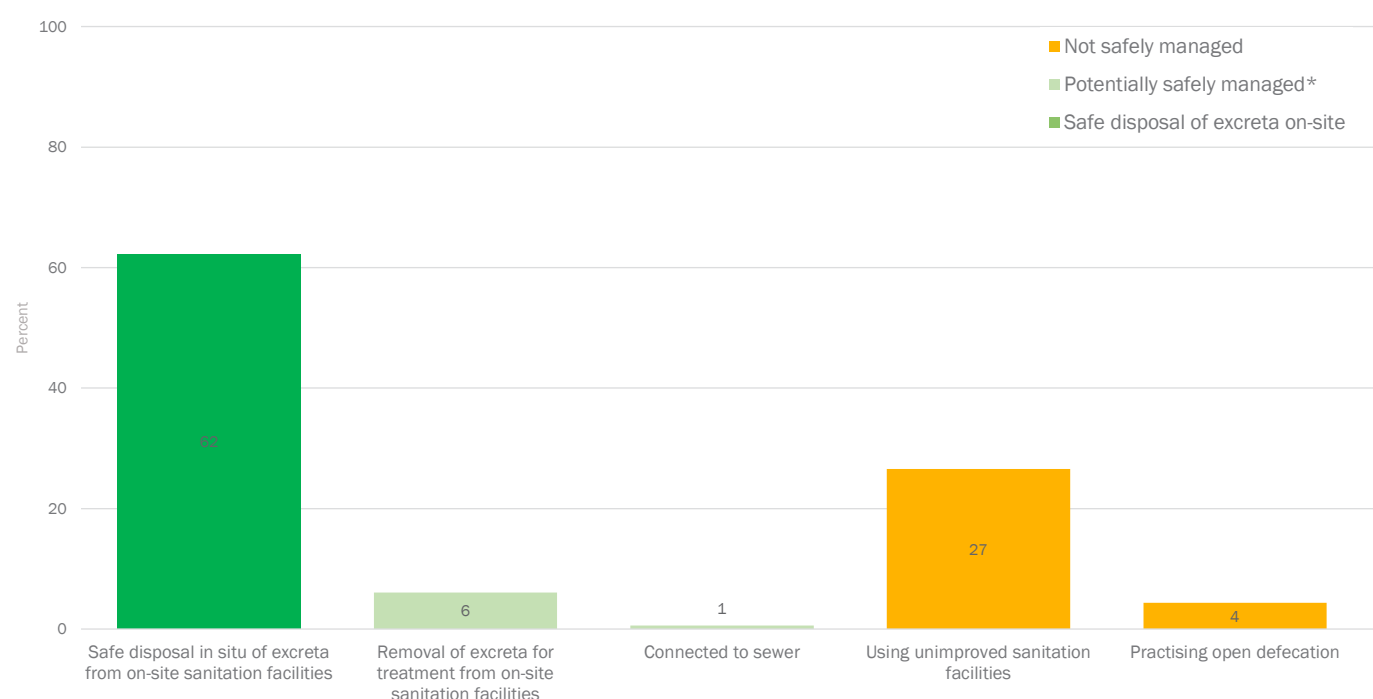
Onsite sanitation facilities include “Flush/pour flush to septic”, “Flush/pour flush to latrine”, “Ventilated improved pit latrine”, “Pit latrine with slab” and “Composting toilet”.

Types of Sanitation Facility by Province

Province	Sewer connection	Onsite sanitation
National	0.6	68.5
Torba	0.0	56.1
Sanma	0.7	70.8
Penama	2.0	43.8
Malampa	0.3	65.7
Shefa	0.1	87.3
Tafea	0.3	47.5

Percent of population using sewer connections and onsite sanitation, by province.

Management of excreta from household sanitation facilities

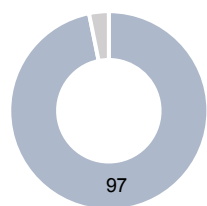


Percent of population by management of excreta from household sanitation facilities.

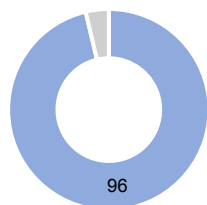
*Additional information required to determine whether faecal sludge and wastewater is safely treated.

Safely managed sanitation services represents an ambitious new level of service during the SDGs and is the indicator for target 6.2. Safely managed sanitation services are improved facilities that are not shared with other households and where excreta are safely disposed of in situ or transported and treated offsite. The MICS survey collected information on the management of excreta from onsite facilities. For households where excreta are transported offsite (sewer connection, removal for treatment), further information is needed on the transport and treatment of excreta to calculate the proportion that are safely managed.

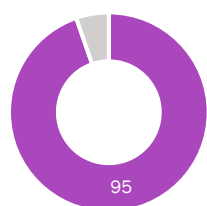
Menstrual Hygiene Management



Women with a private place to wash & change at home



Women with appropriate materials



Women with appropriate materials & a private place to wash & change at home

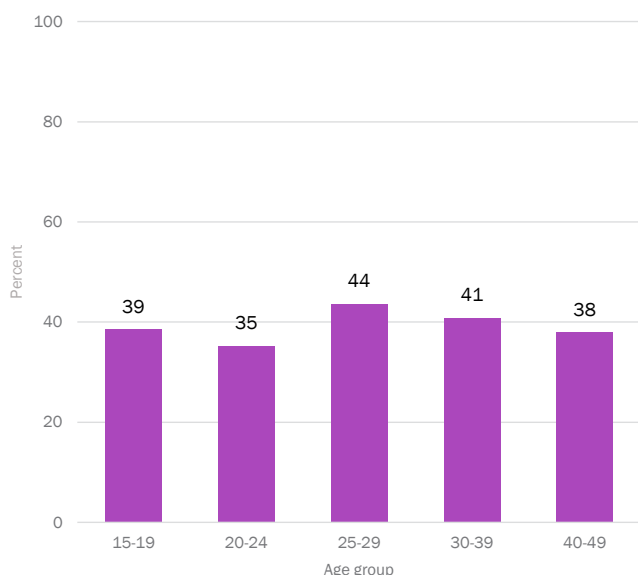
Denominator for all 3 indicators: women age 15-49 who reported menstruating in the last 12 months.

Inequities in Access to Appropriate Materials & Private Place to Wash & Change at Home



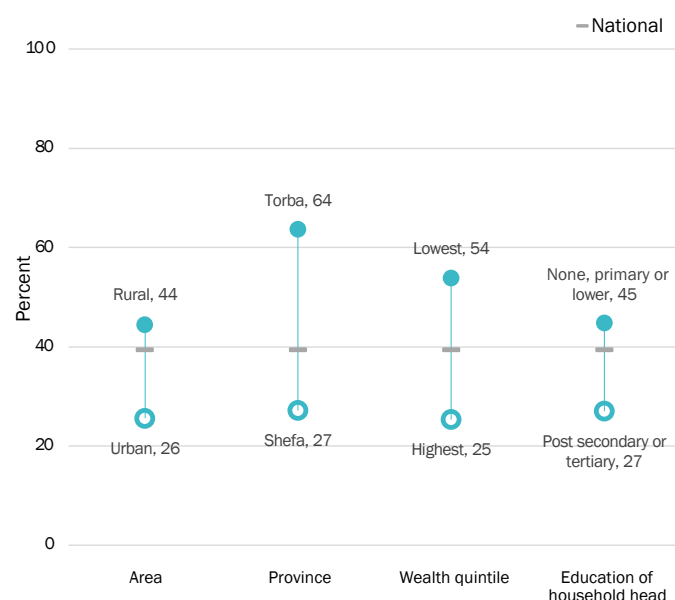
Percent of women age 15-49 using appropriate menstrual hygiene materials with a private place to wash and change while at home, among women reporting menstruating in the last 12 months.

Exclusion from Activities during Menstruation



Percent of women who did not participate in social activities, school or work due to their last menstruation in the last 12 months, by age, among women reporting menstruating in the last 12 months.

Exclusion from Activities during Menstruation by Various Characteristics



Percent of women who did not participate in social activities, school or work due to their last menstruation in the last 12 months, by residence, wealth quintile, education and province, among women reporting menstruating in the last 12 months.

Key Messages

- Most people in Vanuatu (83 percent) have access to basic drinking water services, and nearly three-quarters (74 percent) have improved water sources located on their premises.
- The drinking water is highly contaminated with E-Coli, both at the source (85 percent) and at the household level (81 percent).
- 69 percent of the population have access to improved sanitation facilities, 27 percent use unimproved sanitation facilities, and 4 percent still practice open defecation.
- A third of rural (32 percent) and urban dwellers (34 percent) have unimproved sanitation services and use shared sanitation facilities.
- Only 35 percent of the population have access to handwashing facility with water and soap, and this number drops to 26 percent in rural areas.
- Urban areas (64 percent) and the wealthiest quintile (79 percent) have the highest rates of child's faeces disposal in the garbage.
- 40 percent of women did not participate in social activities during menstruation, but 96 percent of women used proper menstrual materials.
- When comparing provinces, Penama and Torba stand out with the highest percentages of population relying on unimproved water sources, at 46 percent and 39 percent, respectively. This trend is largely attributed to insufficient water availability at these sources, with 95 percent of household members in Torba and 90 percent in Malampa reporting this issue.
- While boiling is a common method of water treatment among households in all provinces, it is concerning that over 80 percent of household members in Torba and Sanma do not treat their water before consumption.
- Regarding sanitation, rural areas in Tafea and Malampa exhibit the highest rates of open defecation, standing at 11 percent and 6 percent, respectively. While the lowest wealth quintile shows an open defecation level of 7 percent.



Gender Equality

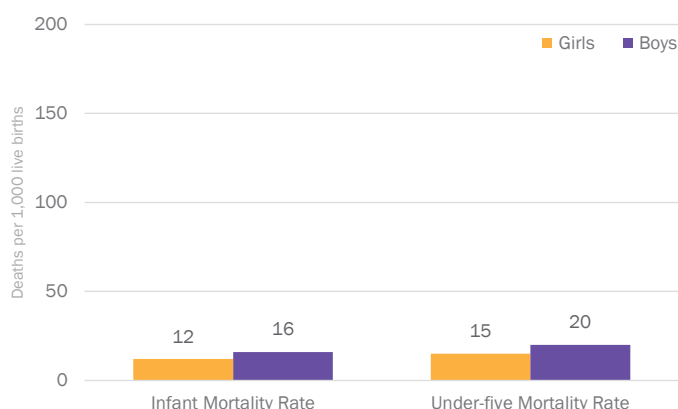


Gender equality means that girls and boys, women and men, enjoy the same rights, resources, opportunities and protections. Investments in gender equality contribute to lifelong positive outcomes for children and their communities and have considerable inter-generational payoffs because children's rights and well-being often depend on women's rights and well-being. This Snapshot shows key dimensions of gender equality during the lifecycle. It is organized around: 1) the first decade of life (0-9 years of age) when gender disparities are often small, particularly in early childhood; 2) the second decade of childhood (10-19 years of age) when gender disparities become more pronounced with the onset of puberty and the consolidation of gender norms; and 3) adulthood, when gender disparities impacts both the wellbeing of women and girls and boys.

Every Girl & Boy Survives & Thrives: The First Decade of Life

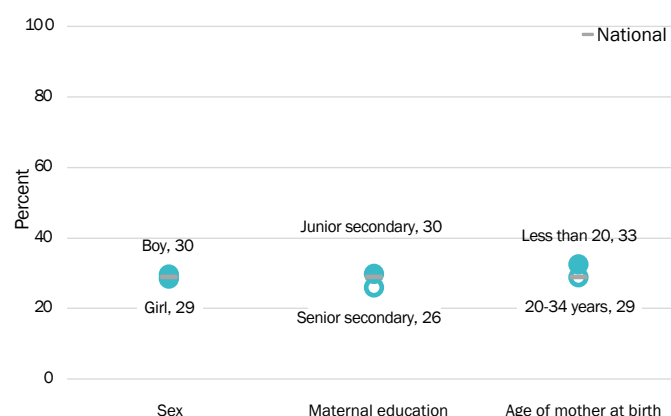
Nutrition and a supportive environment in early childhood are among the key determinants of the health and survival of children and their physical and cognitive development. Generally, girls tend to have better biological endowments than boys for survival to age five, and thus higher survival chances under natural circumstances. However, gender discrimination against girls can affect survival, resulting in higher than expected female mortality. Similarly, stunting rates are typically lower among girls than boys, potentially due to the higher risk for preterm birth among boys, which is inextricably linked with lower birth weight. However, children with mothers who gave birth at a young age or who have no education may be more likely to be malnourished. Children with restricted cognitive development during early life are at risk for later neuropsychological problems, poor school achievement, early school drop-out, low-skilled employment, and poor care of their own children. Stimulation and interaction with parents and caregivers can jumpstart brain development and promote well-being in early childhood. This is also the period of development when gender socialization, or the process of learning cultural roles according to one's sex, manifests. Caregivers, particularly fathers, may respond to, and interact with, sons and daughters differently.

Mortality Rates among Children Under-5, SDG 3.2.1 Sex Disaggregate



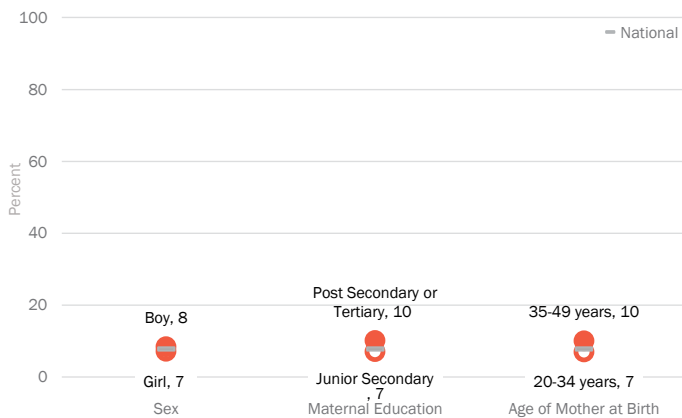
Infant mortality: probability of dying between birth and the first birthday
Under-five mortality: the probability of dying between birth and the fifth birthday.

Malnutrition: Stunting (Moderate & Severe) among Children Under-5, SDG 2.2.1



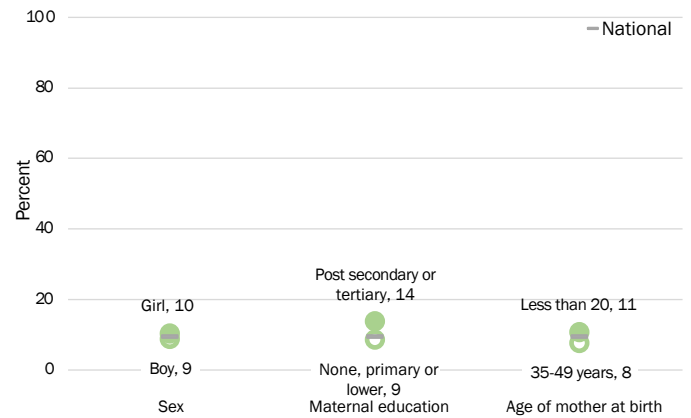
Stunting refers to a child too short for his or her age.

Malnutrition: Wasting (Moderate & Severe) among Children Under-5, SDG 2.2.2



Wasting refers to a child who is too thin for his or her height.

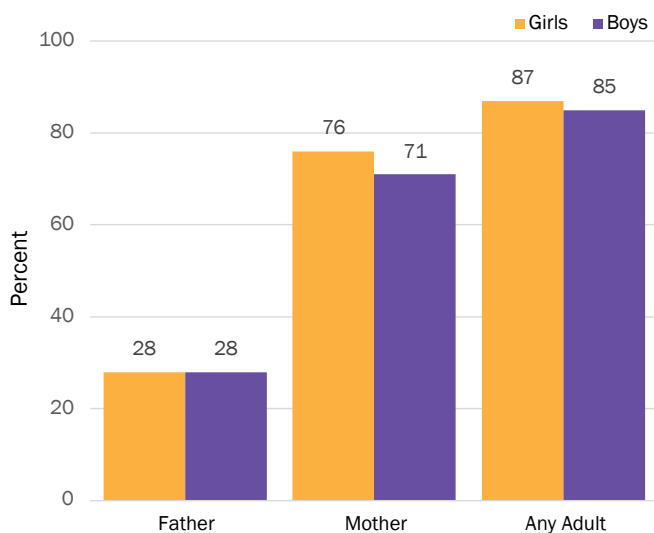
Malnutrition: Overweight (Moderate & Severe) among Children Under-5, SDG 2.2.2



Overweight refers to a child who is too heavy for his or her height.

Every Girl & Boy Survives & Thrives: The First Decade of Life

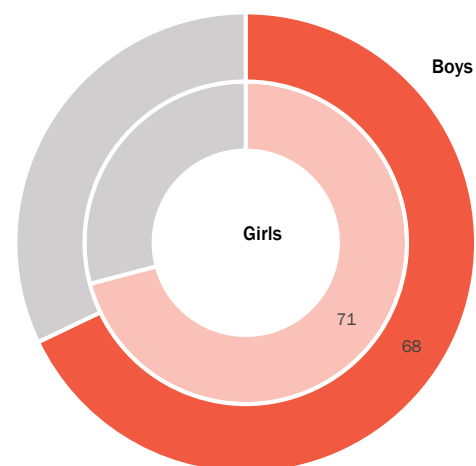
Early Stimulation & Responsive Care by Adults



Percentage of children age 2-4 years with whom adult household members engaged in activities that promote learning and school readiness during the last three days, by person interacting with child and sex of child.

Note: Activities include: reading books to the child; telling stories to the child; singing songs to the child; taking the child outside the home; playing with the child; and naming, counting or drawing things with the child.

Early Childhood Development Index, SDG 4.2.1

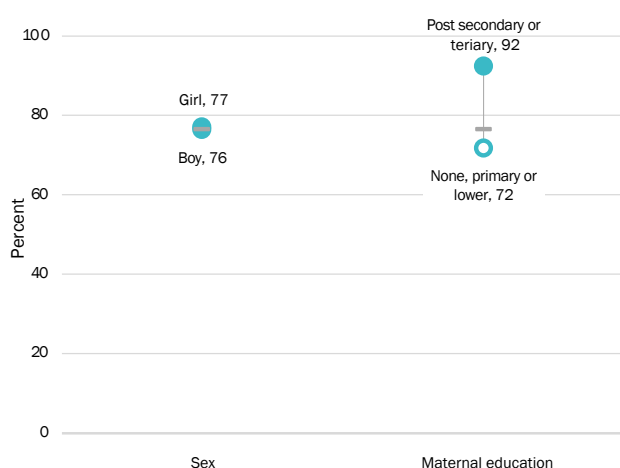


Percentage of children aged 2-4 years who have achieved the minimum number of milestones expected for their age group, by sex.

Every Girl & Boy Is Protected From Violence & Exploitation: The First Decade of Life

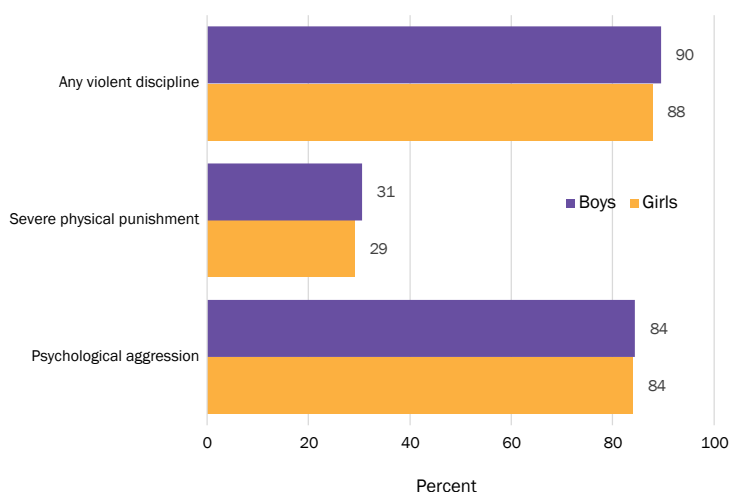
Registering children at birth is the first step in securing their recognition before the law, safeguarding their rights, and ensuring that any violation of these rights do not go unnoticed. While vitally important for both girls and boys, the implications of low birth registration rates for girls are significant, rendering them more vulnerable to certain forms of exploitation they are at greater risk of, including child marriage and international trafficking. Although average birth registration rates are similar for girls and boys, children with mothers who have no education may be less likely to have their births registered. While girls and boys face similar risks of experiencing violent discipline - which includes physical punishment and psychological aggression - by caregivers in the home, gender inequality and domestic violence are among the factors associated with an elevated risk of violence against both girls and boys.

Birth Registration, SDG 16.9.1 Sex Disaggregate



Percentage of children under age 5 whose births are registered, by sex and maternal education level.

Violent Discipline, SDG 16.2.1 Sex Disaggregate



Percentage of children age 1-14 years who experienced violent discipline in the past month, by sex.

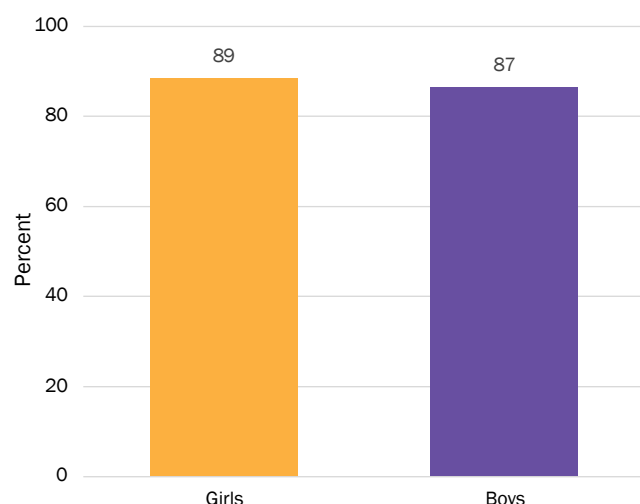
Note: The age group 1-14 spans the first and second decades of life.

Every Girl & Boy Learns: The First Decade of Life

Investment in good quality early childhood education services prior to entering school improves learning outcomes for children. It also enhances the efficiency of the school system by reducing repetition and drop-out and improving achievement, especially among girls and marginalized groups. Primary education provides the foundation for a lifetime of learning. Considerable progress has been made in achieving universal education and closing the gender gap but gender disparities to the disadvantage of girls still exist in some countries. Further, girls still comprise the majority of the world's out-of-school population.

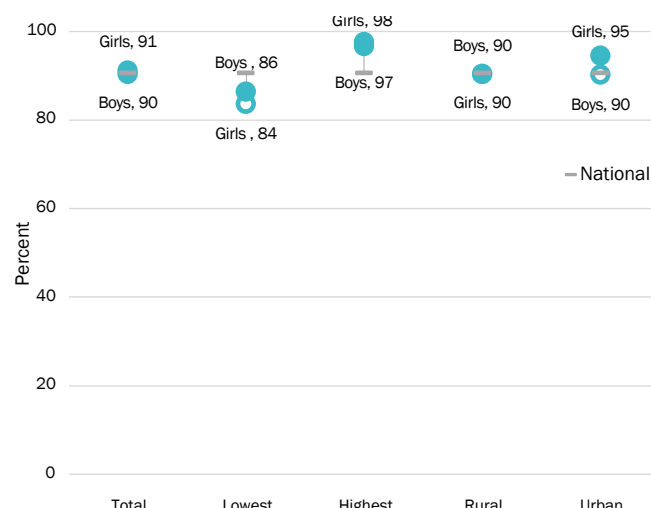
Note: Because children of primary school age range from 6-14 years, these indicators include some children in their second decade of life.

Participation Rate in Organized Learning, SDG 4.2.2



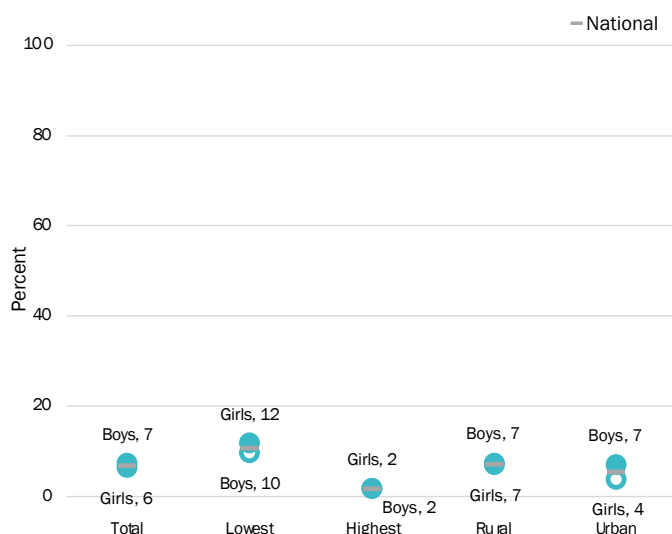
Percentage of children age one year younger than the official primary school entry age at the beginning of the school year who are attending an early childhood education programme or primary school (adjusted net attendance rate), by sex.

Primary School Attendance



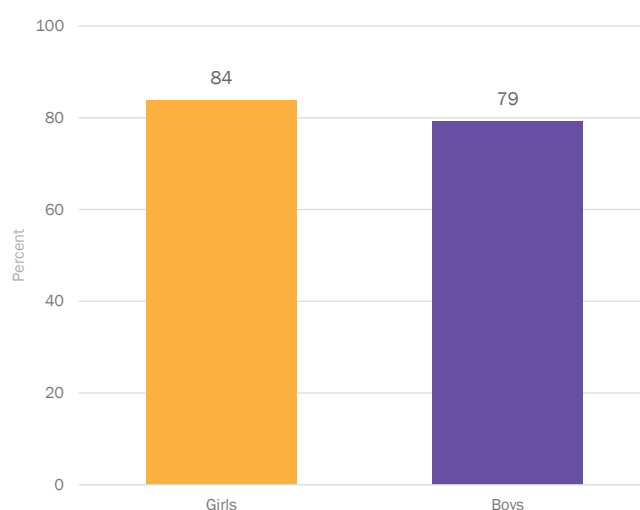
Percentage of children of primary school age attending primary, lower or upper secondary school (adjusted net attendance rate), by wealth quintile and urban/rural residence.

Children of Primary School Age Out of School



Percentage of children of primary school age who are not attending any level of education, by wealth quintile and area.

Primary Completion, SDG 4.1.2



Percentage of children age 3 to 5 years above the intended age for the last grade of primary school who have completed primary education, by sex.

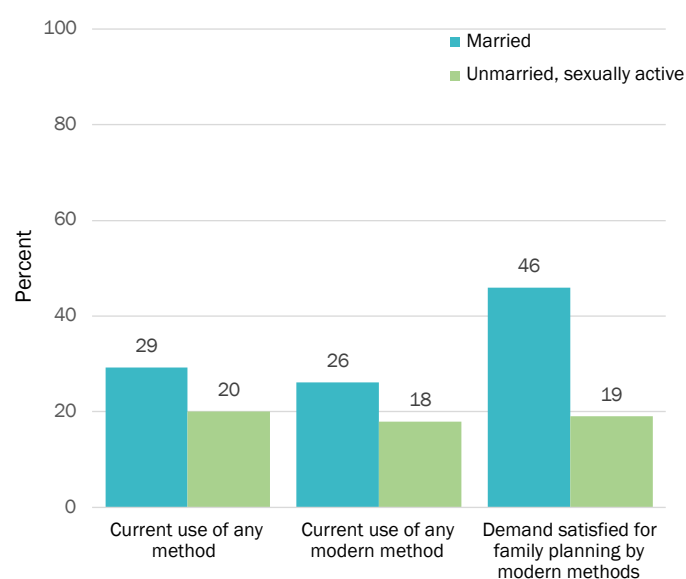
Key Messages

- In the first decade of life, differences between boys and girls seem quite marginal. Boys are more likely to participate in early organised learning, but only by 2 percentage points. Girls score slightly higher on the Early Childhood Development Index. In addition, girls have a five percentage point higher primary school completion rate. In terms of not attending any level of education, the differences between boys and girls are tiny.
- Both boys and girls have their births registered to the same extent.
- Both boys and girls experience similarly high levels of physical discipline.
- Infant and under five mortality rates for young boys are slightly higher than for young girls – following a pattern seen throughout the world.

Every Adolescent Girl & Boy Survives & Thrives: The Second Decade of Life

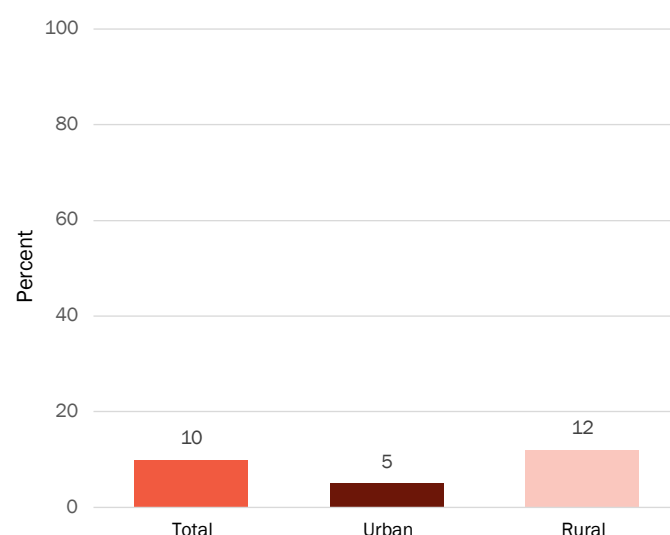
While adolescence carries new health risks for both girls and boys, girls often face gender-specific vulnerabilities, with lifelong consequences. Complications related to pregnancy and childbirth are among the leading causes of death worldwide for adolescent girls age 15 to 19. Preventing adolescent pregnancy not only improves the health of adolescent girls, but also provides them with opportunities to continue their education, preparing them for jobs and livelihoods, increasing their self-esteem and giving them more say in decisions that affect their lives. Yet, too often, adolescent girls lack access to appropriate sexual and reproductive health services, including modern methods of contraception. Additionally, despite having a higher risk of contracting HIV due to both greater physiological vulnerabilities and gender inequalities, adolescent girls are often less knowledgeable than adolescent boys about how HIV is transmitted. However, gender norms adversely impact adolescent boys as well. For example, norms around masculinity that encourage risk taking may heighten adolescent boys' use of alcohol and tobacco, increasing their likelihood of developing non-communicable diseases later in life.

Contraceptive Use & Demand Satisfied



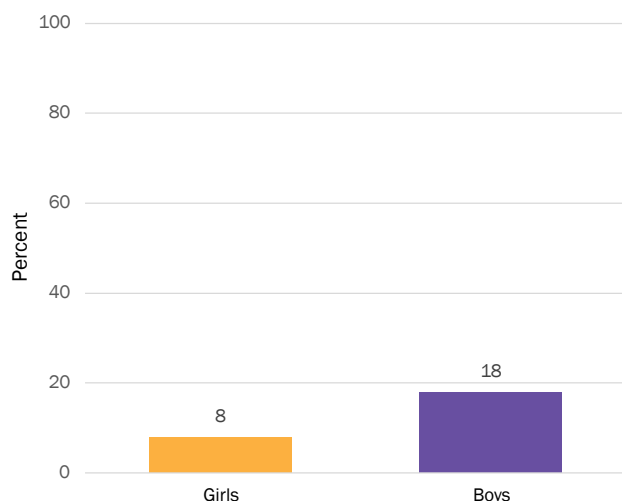
Contraceptive use and demand for family planning satisfied by modern methods among adolescent girls age 15-19, by marital status.

Early Childbearing - by Age 18



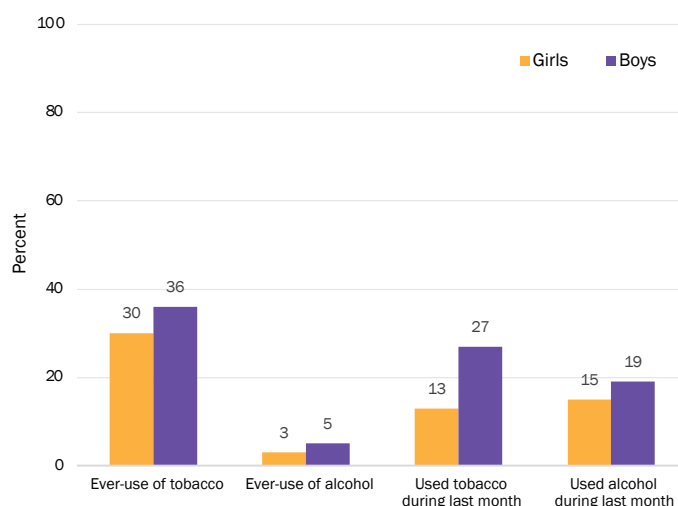
Percentage of women age 20-24 years who had a live birth by age 18, by urban/rural residence.

Comprehensive Knowledge of HIV



Percent of girls and boys age 15-19 who know of the two ways of HIV prevention (having only one faithful uninfected partner and using a condom every time), who know that a healthy looking person can be HIV-positive, and who reject the two most common misconceptions, and any other local misconception.

Tobacco* & Alcohol Use



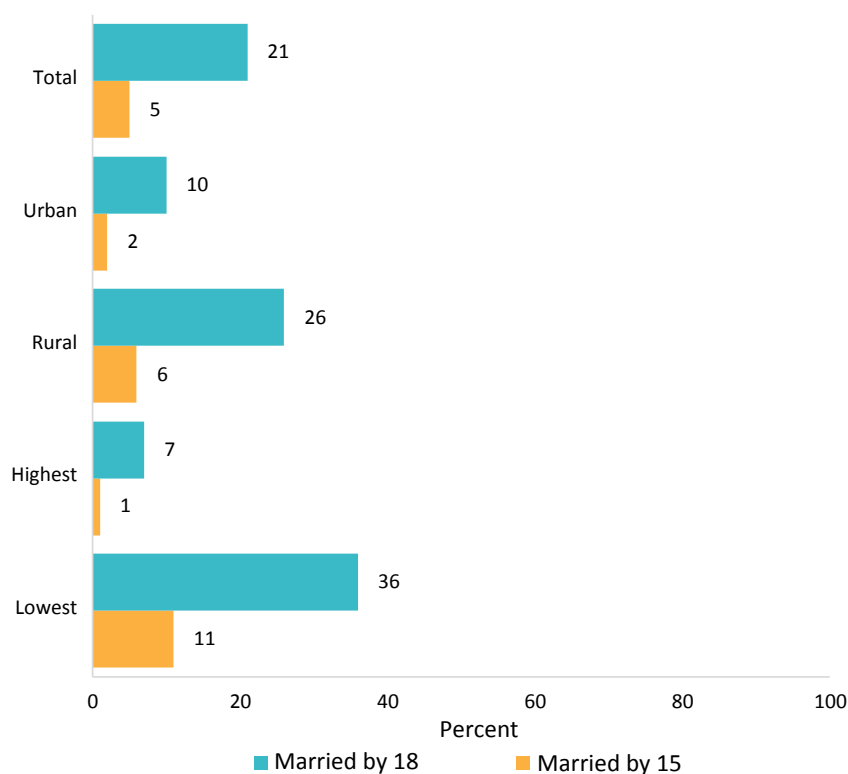
Tobacco and alcohol use among adolescents age 15-19, by sex.

*Includes an age and sex disaggregate of SDG 3.a.1: use of tobacco.

Every Adolescent Girl & Boy is Protected from Violence & Exploitation: The Second Decade of Life

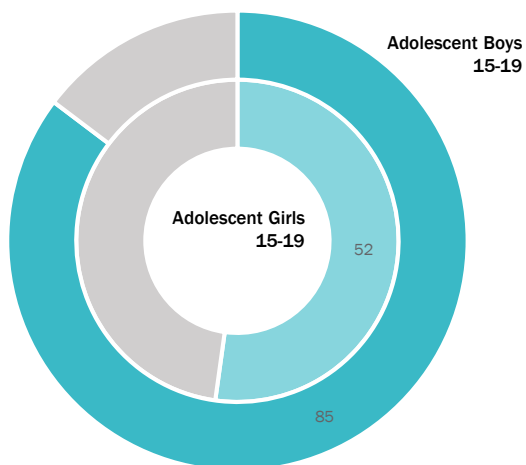
Adolescence presents unique vulnerabilities to violence and exploitation for girls. In many countries, marriage before the age of 18 is a reality for girls due to the interaction of several factors that place a girl at risk, including poverty, social norms, customary or religious laws that condone the practice, an inadequate legislative framework and the state of a country's civil registration system. Child marriage often compromises a girl's development by resulting in early pregnancy and social isolation, interrupting her schooling, and limiting her opportunities for career and vocational advancement. It also often involves a substantial age difference between the girl and her partner, thus further disempowering her and putting her at greater risk of partner violence, sexually transmitted diseases and lack of agency. Attitudes about wife beating serve as a marker for the social acceptability of intimate partner violence. Acceptance of wife beating among adolescent girls and boys suggests that it can be difficult for married girls who experience violence to seek assistance and for unmarried girls to identify and negotiate healthy and equitable relationships. Female genital multi is a human rights issue that also affects girls and women. Adolescence, in particular, is a vulnerable period for girls who have undergone FGM because they may experience heightened consequences of the procedure as they become sexually active and begin childbearing. Gender-based discrimination may be one of the most ubiquitous forms of discrimination adolescent girls face, and it has long-lasting and far-reaching effects on their personal trajectories as well as on all aspects of social and economic development. While in most regions, girls and boys are equally likely to be involved in child labour, gender is a determinant of the types of activities boys and girls engage in, with girls more likely to be involved in domestic work.

Child Marriage, SDG 5.3.1



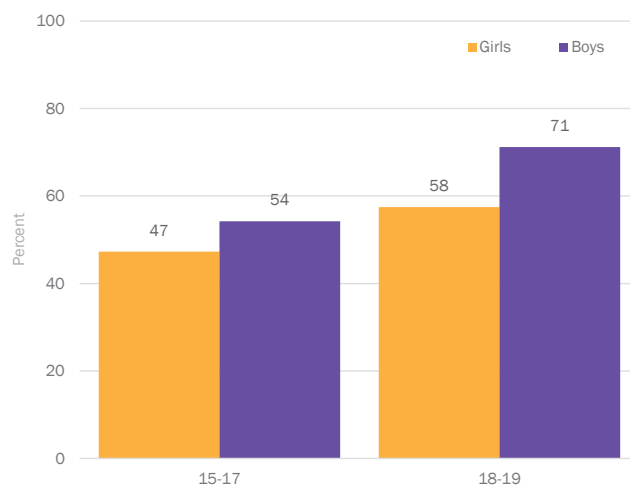
Percentage of women age 20-24 years who were first married or in union before age 15 and before age 18, by residence and wealth quintile.

Feelings of Safety, SDG 16.1.4 Age & Sex Disaggregate



Percentage of adolescents age 15-19 who feel safe walking alone in their neighbourhood after dark, by sex.

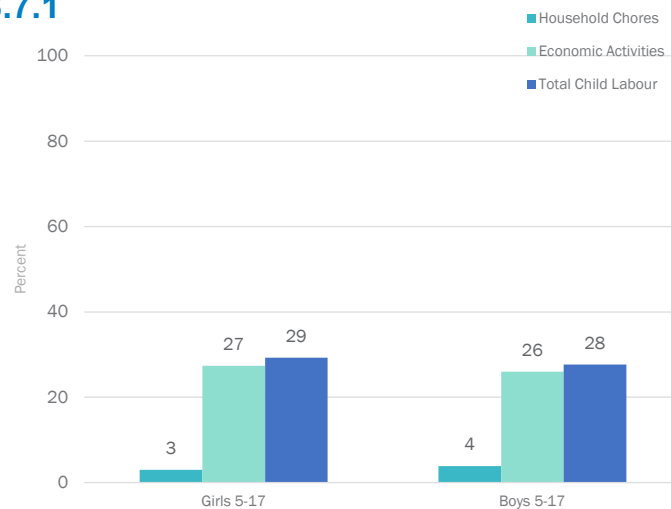
Attitudes toward Domestic Violence



Percentage of adolescents age 15-19 years who justify wife beating for any of the following reasons: she goes out without telling him; she neglects the children; she argues with him; she refuses sex with him; she burns the food, by sex and age group.

Every Adolescent Girl & Boy is Protected from Violence & Exploitation: The Second Decade of Life

Child Labour, SDG 8.7.1



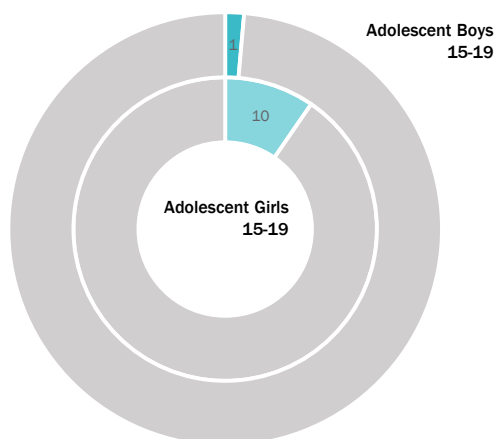
Percentage of children age 5-17 years engaged in child labour, by sex, age group and type of activity.

* Note: Indicator includes children in the first & second decade of life.

Every Adolescent Girl & Boy has an Equitable Chance in Life: The Second Decade of Life

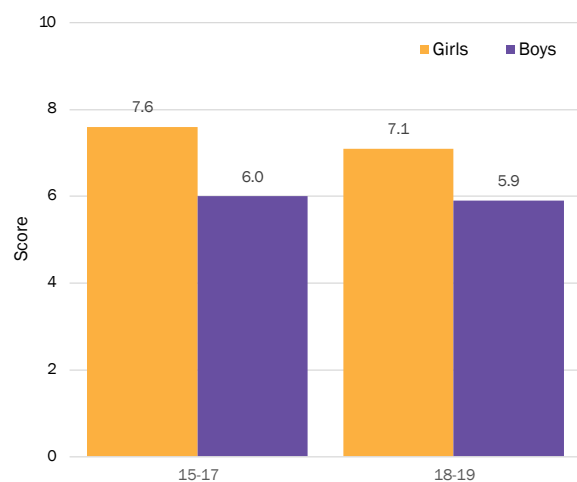
To become empowered, adolescent girls and boys need to be engaged as civic participants in the decisions affecting their lives and communities. People's sense of security and freedom from the fear of crime influences how they move about those communities, access services and economic opportunities and participate in public life. Adolescent girls and boys are likely to have different perceptions of personal safety due to different gender-based vulnerabilities to sexual violence and other crimes. Life satisfaction measures an individual's perceived level of well-being or how an individual feels about their life as a whole. Measuring adolescent girls and boy's satisfaction with their lives can provide important insights into their mental health during a stage of life when gender norms consolidate and girls and boys experience different risk factors for mental health disorders.

Discrimination & Harassment



Percentage of adolescent girls and boys age 15-19 years who have ever felt discriminated or harassed based on their gender.

Life Satisfaction

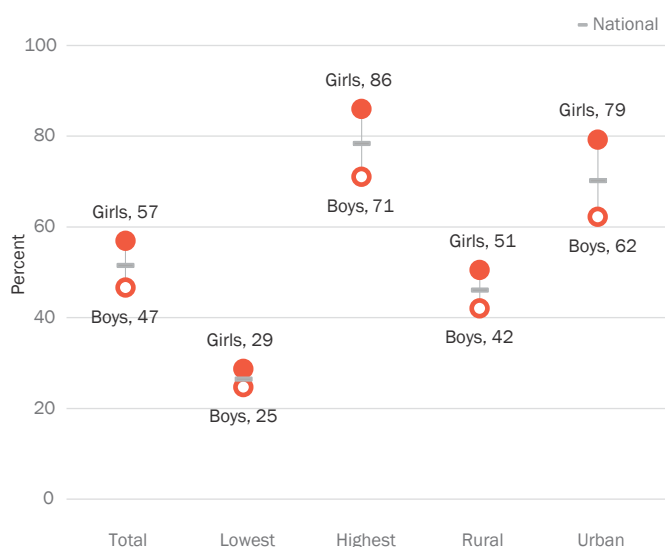


Among adolescents age 15-19, average life satisfaction score on a scale of 0 to 10, by sex and age group. A score of ten indicates a high level of happiness and zero indicates extreme unhappiness.

Every Adolescent Girl & Boy Learns: The Second Decade of Life

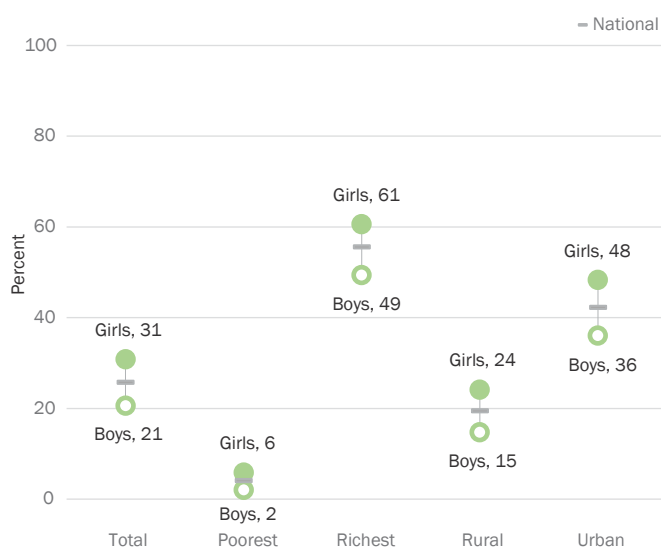
Globally, participation in secondary education is expanding, progress lags behind primary education. Gender disparities disadvantaging girls are also wider and occur in more countries at the secondary level than at the primary level. Yet, advancing girls' secondary education is one of the most transformative development strategies countries can invest in. Completion of secondary education brings significant positive benefits to girls and societies – from increased lifetime earnings and national growth rates, to reductions in child marriage, stunting, and child and maternal mortality.

Junior Secondary Attendance Net Attendance Rate



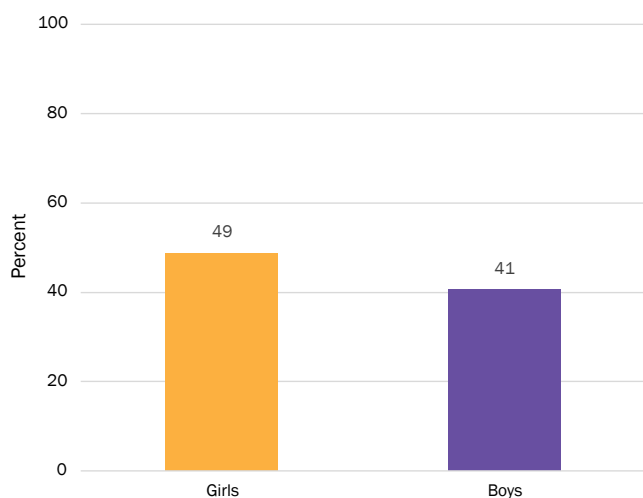
Percentage of children of junior secondary school age attending junior secondary school or higher (adjusted net attendance rate), by sex, wealth quintile and area.

Senior Secondary Attendance Net Attendance Rate



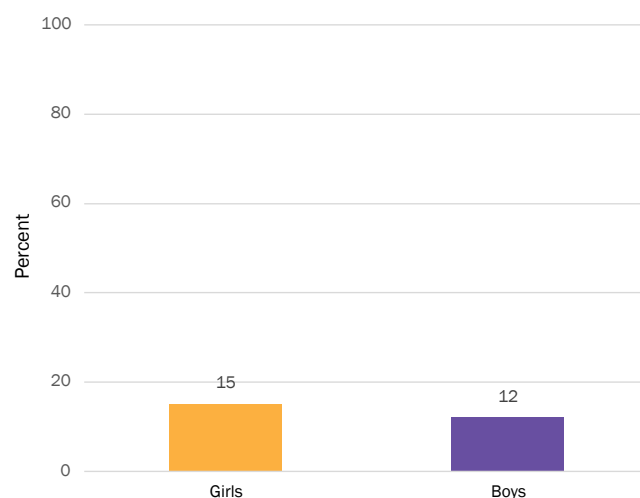
Percentage of children of senior secondary school age attending senior secondary school or higher (adjusted net attendance rate), by sex, wealth quintile and area.

Junior Secondary Completion, SDG 4.1.2



Percentage of children who age 3 to 5 years above the intended age for the last grade of junior secondary school who have completed lower secondary education, by sex.

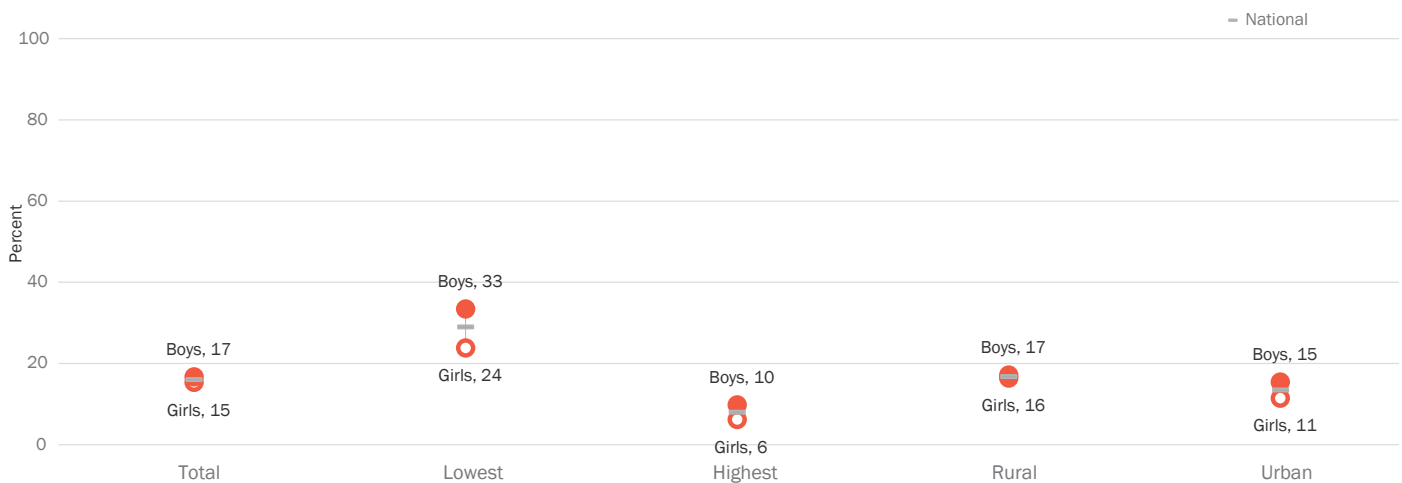
Senior Secondary Completion, SDG 4.1.2



Percentage of children or youth who age 3 to 5 years above the intended age for the last grade of senior secondary school who have completed upper secondary education, by sex.

Every Adolescent Girl & Boy Learns: The Second Decade of Life

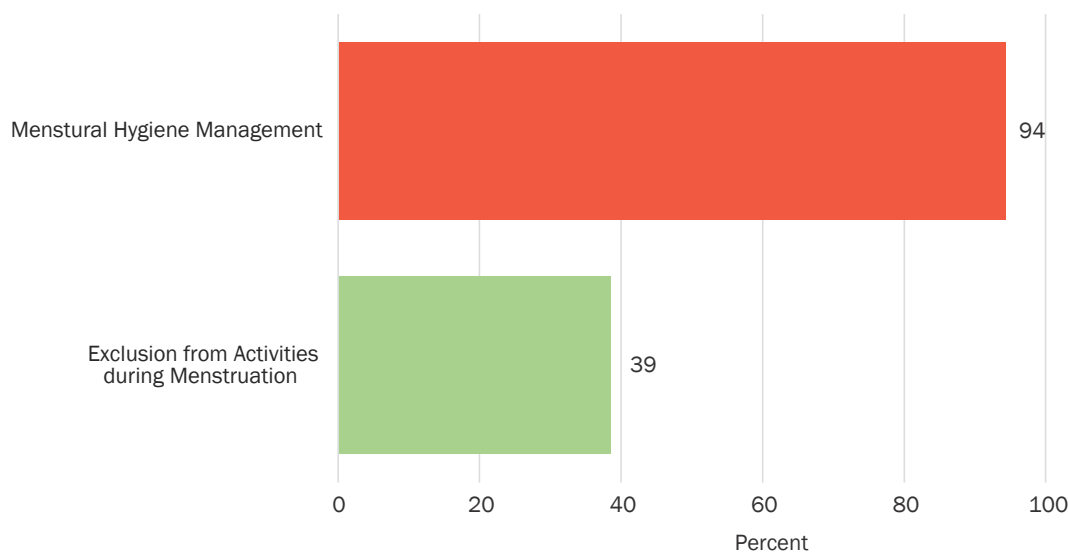
Children of Junior Secondary School Age Out of School



Percentage of children of junior secondary school age who are not attending any level of education, by wealth quintile and area.

Every Adolescent Girl & Boy Lives in a Safe & Clean Environment: The Second Decade of Life

Menstrual Hygiene Management



The ability of adolescent girls to safely manage their monthly menstrual cycle in privacy and with dignity is fundamental to their health, psychosocial well-being and mobility. Girls in low-resource and emergency contexts without access to adequate menstrual hygiene management facilities and supplies experience stigma and social exclusion while also forgoing important educational, social and economic opportunities.

Menstrual Hygiene Management: Among adolescent girls age 15-19 who reported menstruating in the last 12 months, percentage using appropriate menstrual hygiene materials with a private place to wash and change while at home.

Exclusion from Activities during Menstruation: Among adolescent girls age 15-19 who reported menstruating in the last 12 months, percentage of women who did not participate in social activities, school or work due to their last menstruation in the last 12 months.

Key Messages

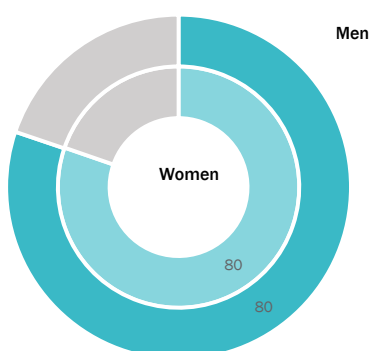
- In the second decade of life, educational attainment differences between girls and boys becomes more prominent. Girls do increasingly better than boys as they move through the educational system.
- Life satisfaction for young women is higher than for young men.
- Among adolescent girls age 15-19 who reported menstruating in the last year, the percentage using appropriate menstrual hygiene materials with a private place to wash and change while at home is high at 94 percent. However, two out of five (39 percent) young women reported not participating in social activities, school or work due to their last menstruation during the last 12 months.
- Adolescent girls have less knowledge about HIV, which could put them more at risk. However, they are less likely to smoke cigarettes or drink alcohol than adolescent boys.
- Perhaps surprisingly, both young men and women have quite a high acceptance that wife beating is justified in some circumstances. The rates of agreement are higher for young men, but 58 percent of young women age between 18 and 19 agreed that wife beating was acceptable in some instances.
- The extent of child labour is similar for both girls and boys.

Gender Equality in Adulthood

To survive and thrive, all children require care and support from women and men. Care and support can be substantively improved by fostering gender equality, an important goal in its own right, and by reducing the gender-related barriers. Gender-related barriers include women's and girls' disproportionate lack of information, knowledge and technology, resources, and safety and mobility, as well as the gender division of labour and gender norms. For example, a mother's lack of mobility, due to prohibitive norms or lack of transportation, may impede birth registration, nutrition, and other child outcomes. The internalization of gender norms around masculine and feminine expectations and behaviours may influence women's and men's attitudes toward intimate partner violence and physical punishment of children as well as self-perceptions of well-being, including life satisfaction and expectations for the future.

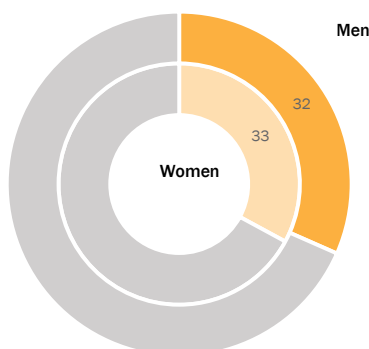
Access to Knowledge, Information & Technology

Literacy



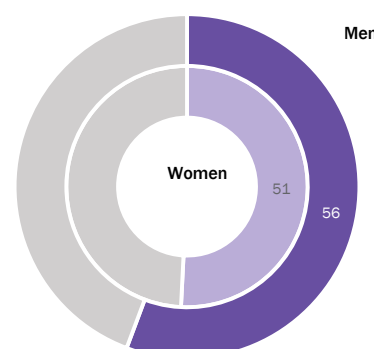
Percentage of adults age 15-49 who are literate, by sex.

Media Access



Percentage of adults age 15-49 who read a newspaper, listen to the radio, or watch television at least once a week.

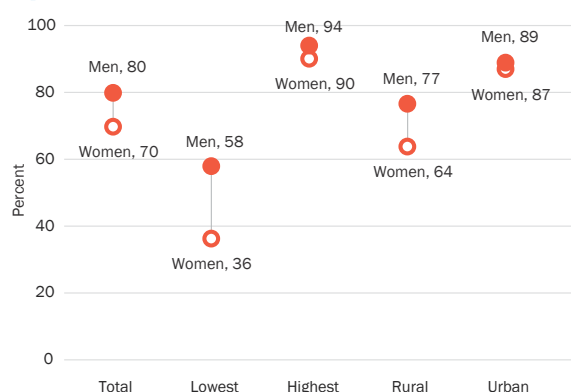
Internet Use: SDG17.8.1



Percentage of adults age 15-49 using the internet at least once in the past 3 months, by sex.

Access to Resources

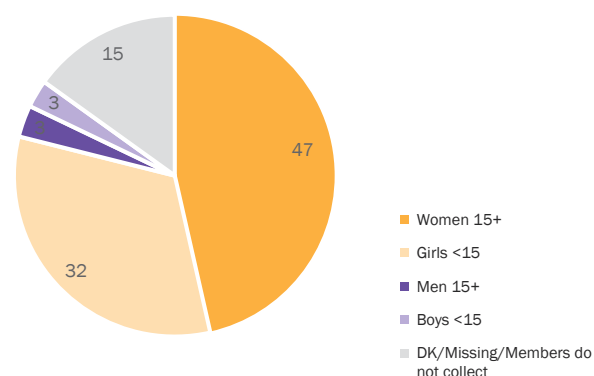
Mobile Phone Ownership, SDG 5.b.1



Percentage of adults age 15-49 who own a mobile phone, by sex, wealth quintile and area.

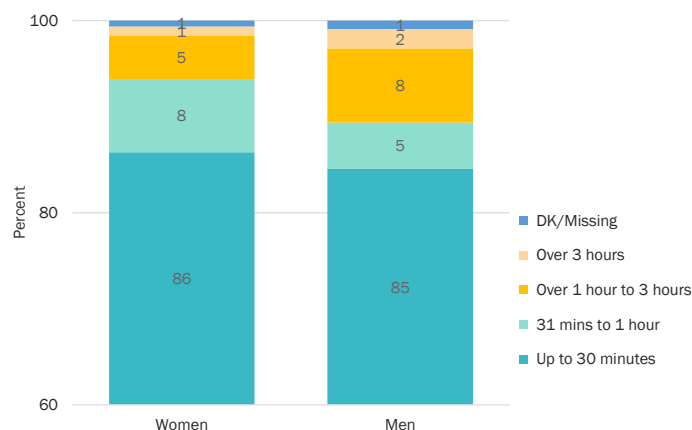
Time on Household Chores: Water Collection

Who collects water?



Percent distribution of household members without drinking water on premises by person usually collecting drinking water used in the household.

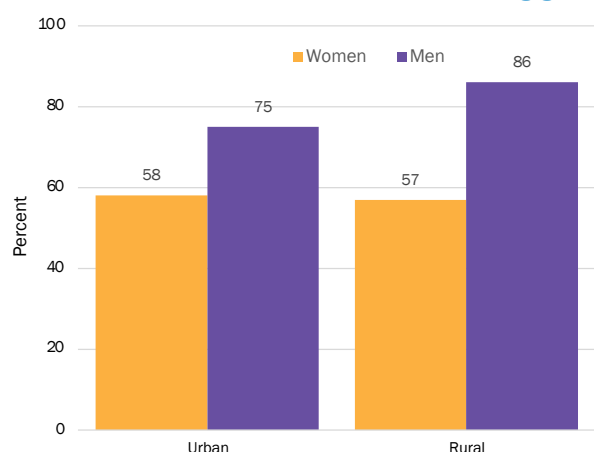
Time spent on water collection



Percent distribution of average amount of time spent collecting water per day by sex of person primarily responsible for water collection in households without drinking water on premises.

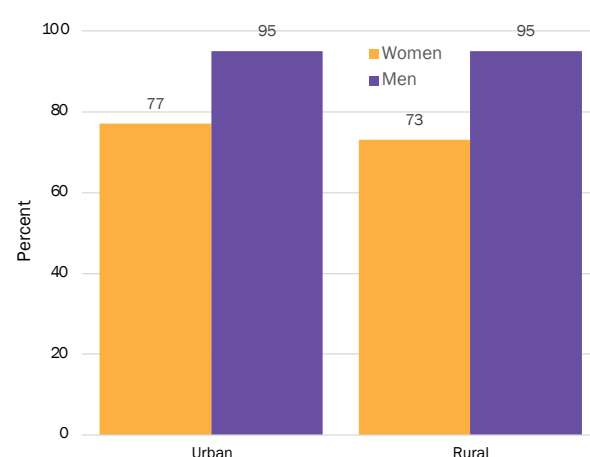
Safety & Security

Feeling safe while walking alone after dark, SDG 16.1.4 sex disaggregate



Percentage of adults who feel safe walking alone in their neighbourhood after dark, by sex and area.

Feeling safe while being at home alone after dark



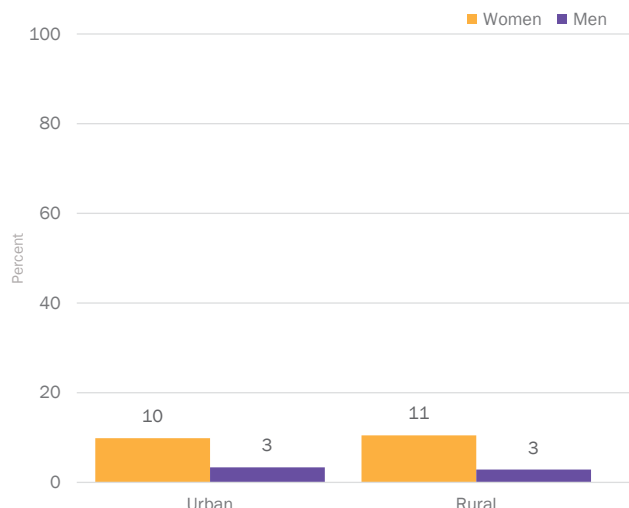
Percentage of adults (age 15-49) who feel safe being home alone after dark, by sex and area.

Victimisation



Percentage of adults age 15-49 who experienced physical violence of robbery or assault in the last year, by sex, wealth quintile and area.

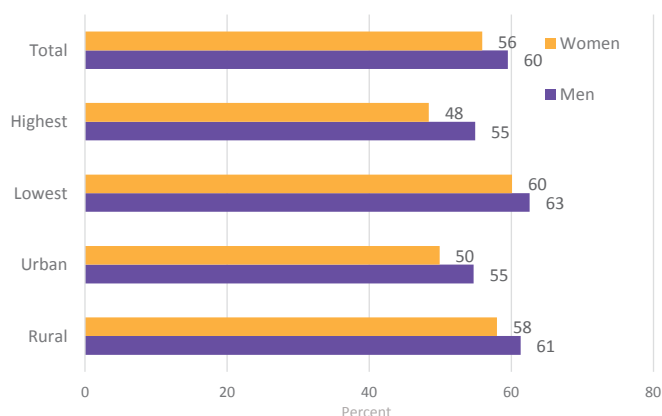
Discrimination & harassment



Percentage of adults age 15-49 who have ever personally felt discriminated or harassed based on their gender, by sex and area.

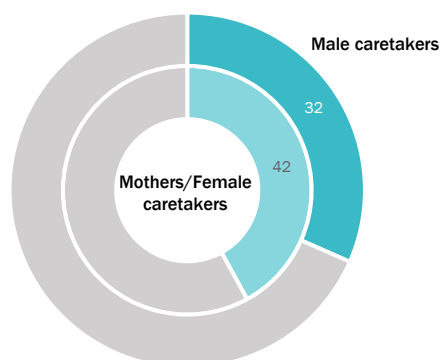
Feminine & masculine attitudes & expectations

Attitudes toward domestic violence



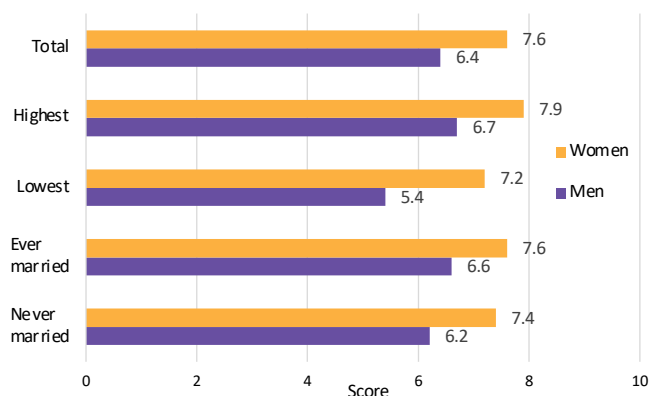
Percentage of adults age 15-49 who justify wife beating for any of the following reasons: she goes out without telling him; she neglects the children; she argues with him; she refuses sex with him; she burns the food, by sex, wealth quintile and area.

Attitudes toward physical punishment



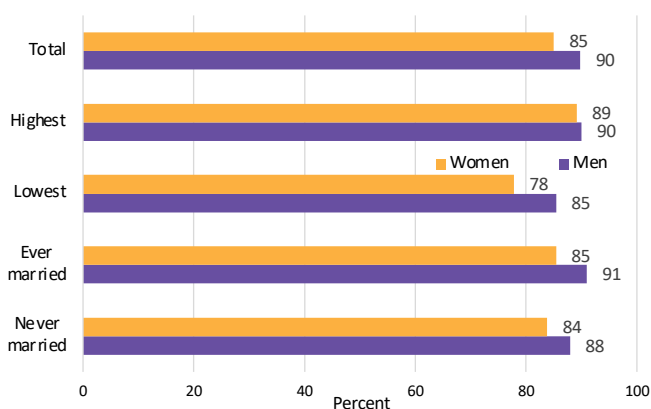
Percentage of mothers/caretakers who believe that physical punishment is needed to bring up, raise, or educate a child properly, by sex of caretaker.

Life satisfaction



Among adults age 15-49, average life satisfaction score on a scale of 0 to 10, by sex, wealth quintile and marital status. Higher scores indicate higher satisfaction levels.

Perceptions of a better life



Percentage of adults age 15-49 who expect that their lives will get better in one year, by sex, wealth quintile and marital status.

Key Messages

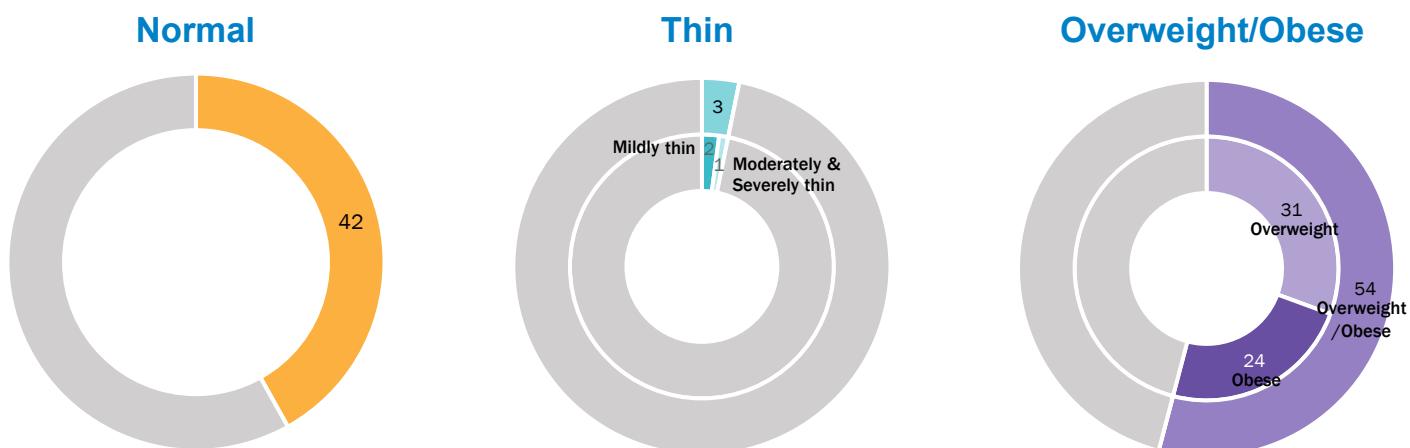
- Once into adulthood, one stark gender difference relates to feelings of safety. Women feel less safe than men both walking and at home at night. This is particularly noticeable for rural women with only 73 percent feeling safe at home alone at night compared to 95 percent of men.
- Women bear by far the bulk of water collection activities, with 86 percent of women spending up to 30 minutes a day collecting water.
- Women are ten times more likely than men to feel discriminated against based on their gender.
- In terms of media and communications, here the gender differences are small. Media access and internet use is almost identical for men and women. Mobile phone ownership is at a high level for men and women, except for women in households in the lowest wealth quintile or women living in rural areas.
- The literacy rate is the same for both adult men and women.
- Attitudes towards wife beating are high for both adult men and women. The attitude that children occasionally need physical punishment is expressed more by female caregivers than male.
- Life satisfaction scores are slightly higher for women, but men are a little more optimistic that their lives will improve in years time.



Women's Nutrition & Dietary Diversity



Women's Body Mass Index*



Percentage of women age 15-49 years with specific BMI levels.

* **The Body Mass Index (BMI)** is expressed as the ratio of weight in kilograms to the square of height in meters (kg/m^2);

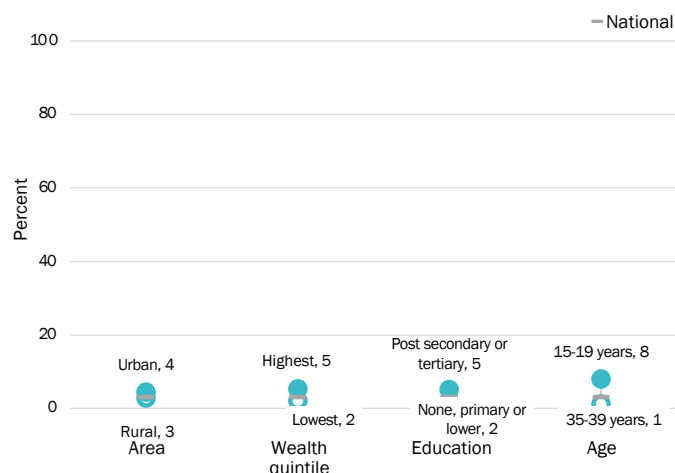
Normal BMI levels: in range between 18.5-24.9; **Thin BMI levels:** below 18.5, women with **mildly thin** BMI levels in range between 17.0-18.4 and **moderately and severely thin** women with BMI levels below 17 (≥ 25.0 (Total overweight or obese);

Overweight/obese BMI levels: above 25.0, for **overweight** women in range between 25.0-29.9 and for **obese** for women, BMI above 30.

Provincial Data on Women's Body Mass Index

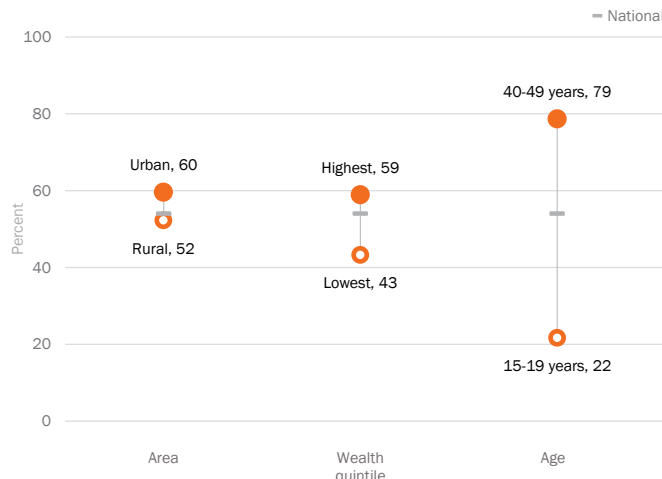
Province	Normal	Thin		Overweight/Obese	
	% normal	% mildly thin	% moderately and severely thin	% overweight	% obese
National	42.4	2.1	1.1	30.6	23.5
Torba	53.9	2.1	1.2	32.3	10.5
Sanma	33.9	1.7	0.6	36.0	27.0
Penama	50.3	2.1	0.5	28.8	17.5
Malampa	48.4	2.1	1.7	30.1	17.7
Shefa	36.3	2.7	1.6	29.8	29.5
Tafea	58.2	0.9	0.3	27.1	13.6

Women's Thin BMI Levels



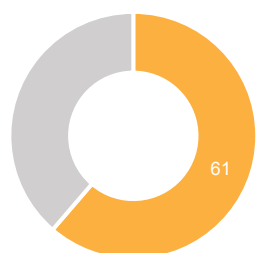
Percent of women age 15-49 with thin BMI levels, by background characteristics.

Women's Overweight/Obese BMI Levels

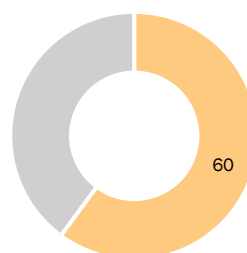


Percent of women age 15-49 with overweight/obese BMI levels, by background characteristics.

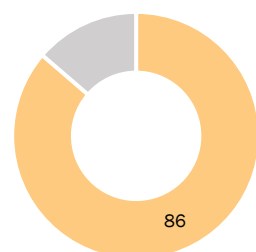
Minimum Dietary Diversity for Women



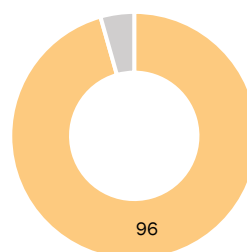
Percent achieving **Minimum Dietary Diversity for Women** consuming 5 or more food groups yesterday.



Percent of women consuming **pulses, nuts and seeds** within 24 hours.



Percent of women consuming **animal-source foods** within 24 hours.



Percent of women consuming **fruits and vegetables** within 24 hours.

Gaps in Minimum Dietary Diversity for Women



Percent of women age 15-49 who achieved minimum dietary diversity, by background characteristics.

Provincial Data on Minimum Dietary Diversity for Women

Province	Minimum Dietary Diversity for Women
National	61.3
Torba	37.9
Sanma	62.9
Penama	41.3
Malampa	79.6
Shefa	65.8
Tafea	50.4

Percent of women age 15-49 who achieved minimum dietary diversity, by province.

Key Messages

- More than half (54 percent) of women in Vanuatu are overweight or obese (BMI ≥ 25.0). Of these, 24 percent are obese (BMI ≥ 30.0).
- About two-thirds of women achieved minimum dietary diversity by consuming at least five different food groups 24 hours prior to the survey. The percentage was higher for women living in wealthier households (70 percent) and those having post secondary education or tertiary education (74 percent).



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